Answer Key & Rationales · Form 7

Quick key. **P** = pretest (unscored; do not count). Bank 2026.10.22-0834.

#	Key	Area	Sub-area	P
1	**B**	II	II.A	
2	**A**	III	III.B	**P**
3	**C**	III	III.D	
4	**B**	III	III.D	
5	**C**	III	III.A	
6	**C**	II	II.B	
7	**B**	II	II.A	
8	**A**	II	II.B	
9	**C**	I	I.C	
10	**C**	II	II.B	**P**
11	**B**	III	III.B	
12	**A**	I	I.B	
13	**B**	III	III.A	
14	**B**	I	I.B	
15	**D**	II	II.C	
16	**A**	I	I.A	**P**
17	**A**	II	II.C	
18	**A**	II	II.C	
19	**B**	I	I.C	
20	**B**	I	I.C	
21	**C**	III	III.B	
22	**B**	II	II.B	
23	**D**	III	III.B	
24	**D**	II	II.A	
25	**C**	I	I.B	
26	**A**	I	I.B	
27	**A**	I	I.A	
28	**A**	I	I.A	
29	**B**	III	III.B	
30	**C**	III	III.B	**P**
31	**A**	II	II.A	
32	**C**	I	I.A	
33	**B**	III	III.C	
34	**A**	I	I.C	
35	**B**	I	I.B	
36	**A**	III	III.B	
37	**A**	II	II.A	
38	**B**	III	III.B	**P**
39	**A**	II	II.A	
40	**B**	II	II.A	
41	**C**	II	II.A	
42	**A**	III	III.C	
43	**C**	I	I.C	
44	**C**	III	III.B	
45	**B**	III	III.B	
46	**B**	II	II.B	
47	**A**	II	II.B	
48	**C**	I	I.B	
49	**A**	II	II.A	
50	**B**	I	I.B	
51	**B**	I	I.A	
52	**B**	I	I.A	
53	**A**	I	I.B	
54	**C**	I	I.A	
55	**C**	III	III.B	
56	**B**	I	I.B	
57	**A**	I	I.B	**P**
58	**C**	III	III.B	
59	**C**	III	III.A	
60	**A**	II	II.B	
61	**A**	III	III.B	
62	**A**	II	II.B	
63	**D**	II	II.B	
64	**B**	II	II.A	
65	**C**	I	I.C	
66	**B**	I	I.A	
67	**C**	III	III.D	
68	**C**	I	I.B	
69	**C**	III	III.B	
70	**B**	II	II.A	
71	**C**	III	III.B	**P**
72	**C**	III	III.A	
73	**A**	II	II.B	
74	**C**	III	III.B	
75	**B**	II	II.A	
76	**C**	I	I.B	
77	**B**	II	II.A	
78	**A**	I	I.B	
79	**C**	I	I.A	
80	**C**	II	II.B	**P**
81	**B**	I	I.C	
82	**B**	III	III.D	
83	**C**	I	I.B	**P**
84	**A**	III	III.C	
85	**A**	I	I.A	
86	**C**	III	III.B	
87	**A**	I	I.B	
88	**B**	III	III.B	
89	**D**	I	I.B	
90	**B**	II	II.B	
91	**C**	I	I.C	
92	**A**	III	III.B	
93	**A**	I	I.B	
94	**C**	II	II.A	
95	**A**	I	I.B	
96	**B**	III	III.A	
97	**B**	III	III.B	
98	**C**	I	I.C	
99	**A**	III	III.B	
100	**C**	III	III.B	
101	**A**	III	III.C	
102	**C**	I	I.C	
103	**C**	II	II.B	
104	**C**	II	II.B	**P**
105	**B**	III	III.B	
106	**C**	II	II.A	
107	**B**	II	II.B	**P**
108	**A**	II	II.A	
109	**D**	II	II.A	
110	**B**	III	III.B	
111	**A**	III	III.A	
112	**C**	I	I.B	
113	**C**	I	I.A	
114	**B**	I	I.C	
115	**C**	I	I.A	
116	**C**	II	II.C	
117	**B**	III	III.C	
118	**D**	II	II.B	
119	**A**	II	II.A	
120	**D**	I	I.A	
121	**C**	I	I.B	**P**
122	**D**	II	II.C	

Scoring worksheet

Count correct answers on **scored items only**. Item numbers for each sub-area are listed; pretest items are left out.

Area / sub-area	Scored item numbers	Items	Correct
I. Values and Ethics		**40**	**____ / 40**
I.A Ethical Principles and Responsibilities	27, 28, 32, 51, 52, 54, 66, 79, 85, 113, 115, 120	12	____
I.B Ethical Service Delivery	12, 14, 25, 26, 35, 48, 50, 53, 56, 68, 76, 78, 87, 89, 93, 95, 112	17	____
I.C Diversity and Social Justice	9, 19, 20, 34, 43, 65, 81, 91, 98, 102, 114	11	____
II. Assessment and Planning		**35**	**____ / 35**
II.A Assessment Concepts	1, 7, 24, 31, 37, 39, 40, 41, 49, 64, 70, 75, 77, 94, 106, 108, 109, 119	18	____
II.B Assessment Methods and Techniques	6, 8, 22, 46, 47, 60, 62, 63, 73, 90, 103, 118	12	____
II.C Assessment Practices	15, 17, 18, 116, 122	5	____
III. Intervention and Practice		**35**	**____ / 35**
III.A Practice Concepts	5, 13, 59, 72, 96, 111	6	____
III.B Intervention Methods and Techniques	11, 21, 23, 29, 36, 44, 45, 55, 58, 61, 69, 74, 86, 88, 92, 97, 99, 100, 105, 110	20	____
III.C Practice Evaluation and Research	33, 42, 84, 101, 117	5	____
III.D Supervision and Administration	3, 4, 67, 82	4	____

Pretest items (skip when scoring): 2, 10, 16, 30, 38, 57, 71, 80, 83, 104, 107, 121

Total correct (scored items)	_____ / 110
Practice pass line	77 / 110 (70%)
Correct needed to reach the line	77 − your total = _____

The real exam sets its passing score separately for each form (it is reported as a scaled score). 77 of 110 is a deliberately cautious practice target that builds a margin; it is not a prediction of your real result.

Score conversion (raw out of 110 → percent)

The boxed row is the practice pass line: **77 / 110 = 70%**. Any score at or above it meets the line.

Raw	%	Raw	%	Raw	%	Raw	%	Raw	%	Raw	%
110	100%	91	83%	72	65%	53	48%	34	31%	15	14%
109	99%	90	82%	71	65%	52	47%	33	30%	14	13%
108	98%	89	81%	70	64%	51	46%	32	29%	13	12%
107	97%	88	80%	69	63%	50	45%	31	28%	12	11%
106	96%	87	79%	68	62%	49	45%	30	27%	11	10%
105	95%	86	78%	67	61%	48	44%	29	26%	10	9%
104	95%	85	77%	66	60%	47	43%	28	25%	9	8%
103	94%	84	76%	65	59%	46	42%	27	25%	8	7%
102	93%	83	75%	64	58%	45	41%	26	24%	7	6%
101	92%	82	75%	63	57%	44	40%	25	23%	6	5%
100	91%	81	74%	62	56%	43	39%	24	22%	5	5%
99	90%	80	73%	61	55%	42	38%	23	21%	4	4%
98	89%	79	72%	60	55%	41	37%	22	20%	3	3%
97	88%	78	71%	59	54%	40	36%	21	19%	2	2%
96	87%	**77**	**70%**	58	53%	39	35%	20	18%	1	1%
95	86%	76	69%	57	52%	38	35%	19	17%	0	0%
94	85%	75	68%	56	51%	37	34%	18	16%		
93	85%	74	67%	55	50%	36	33%	17	15%		
92	84%	73	66%	54	49%	35	32%	16	15%		

Per-area practice guide: aim for at least 70% in each area as well as overall (Area I 28 of 40; Areas II and III 25 of 35 each).

Rationales for every item

Section 1

1. ASM-0108 (rev 1) · Best answer: B

A hospital social worker meets with an 81-year-old patient who lives alone and was admitted after a fall. The team recommends assisted living, but the patient insists on going home. A screening last week showed some memory problems, and the patient cannot explain how they would manage their medications. The patient's adult child asks the social worker to arrange placement. What should the social worker do **FIRST**?

A. Arrange home services so the patient can return home
B. Assess the patient's ability to make this specific decision
C. Begin the assisted living placement the adult child requested

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). If the patient has capacity, supporting a return home with services respects self-determination even when the choice is risky. Acting on it before the doubts about understanding and medication management are assessed could leave the patient unsafe.

B **Correct.** Adults are presumed to have decision-making capacity, but when there is concrete reason for doubt, capacity for the specific decision is assessed before acting either way (Appelbaum, 2007). Here, memory problems and the patient's inability to explain medication management raise that doubt. Capacity covers the abilities to understand, appreciate, reason and express a choice. It beats A because arranging a discharge home honors the stated wish before confirming the patient grasps the risks; if capacity is present, A becomes the right next step (NASW Code 1.02, 1.14).

C **Incorrect** (trap: Overrides the client's choice). Placing the patient because a family member asks overrides a choice that may be the patient's right to make. A family's wishes do not replace the patient's decision unless the patient lacks capacity and a legally authorized person decides.

Takeaway: Presume capacity, but when there is reason for doubt, assess decision-specific capacity before honoring or overriding a risky choice.

Sources: Appelbaum, P. S. (2007). Assessment of patients' competence to consent to treatment. New England Journal of Medicine, 357(18), 1834-1840, whole article; NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.14

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.23 Client capacity, self-determination, self-monitoring (consent age, emancipation, permanency) · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

2. INT-1138 (rev 2) · Best answer: A

PRETEST — unscored on this form; do not count it

A client with a lifelong pattern of giving in to others starts assertiveness training at a housing agency. In 3 days, the client meets with a landlord who is holding back a security deposit the client needs for a new apartment. Only one session fits before the meeting, and the client asks to use it to prepare. Which practice situation is **BEST** for that session?

A. The upcoming talk to get the money returned
B. A low-stakes request, such as returning a store item
C. A long-standing conflict with a relative

Why each option is right or wrong

A **Correct.** Assertiveness training rehearses the real situations a client faces (Lange & Jakubowski, 1976), and work starts where the client is: with the problem the client names as most pressing (Hepworth et al., 11th ed.). Only one session remains before a meeting that decides whether the client can afford the new apartment, so rehearsing that conversation gives the session its greatest value. It beats B because a graded start with easy situations is the usual approach when several sessions are available, but here it would spend the only session before the meeting on practice the client did not ask for.

B **Incorrect** (trap: Right action, wrong time). Starting with low-risk situations and building up is a common, sound way to teach assertiveness when several sessions are available, and it can resume after the meeting. With one session left and the client asking to prepare, spending it on easy situations would leave the client unrehearsed for the conversation that matters most.

C **Incorrect** (trap: Good but not best). A long-standing family conflict may matter to the client, but it carries heavy emotion, has no deadline, and is not what the client asked to work on. It fits later in training, once basic skills are stronger.

Takeaway: Rehearse the real situation a client faces soon; graded practice from easy situations fits when time allows several sessions.

Sources: Lange, A. J., & Jakubowski, P. (1976). Responsible Assertive Behavior: Cognitive/Behavioral Procedures for Trainers. Research Press, behavior rehearsal; Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, engagement: starting where the client is

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.25 Teaching assertiveness skills · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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3. INT-0117 (rev 2) · Best answer: C

A social worker practicing under clinical supervision for licensure presents a case at a monthly peer consultation group. The group recommends a different treatment approach from the one the clinical supervisor directed. What should the social worker do **FIRST**?

A. Switch to the approach the consultation group recommended
B. Continue the supervisor's plan without raising the group's view
C. Discuss the group's recommendation with the supervisor

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Peer consultation offers advice, not direction, and carries no authority over a supervised case. Changing the treatment without the supervisor bypasses the person who shares responsibility and liability for the work.

B **Incorrect** (trap: Good but not best). Following the supervisor's plan respects the supervisor's authority, but staying silent keeps potentially useful clinical input from the person accountable for the case. Supervisees are expected to raise questions openly.

C **Correct.** Supervision carries authority and shared accountability: the supervisor directs the supervisee's work and can be held directly and vicariously liable for it (NASW & ASWB, 2013, Standard 3). Consultation is advisory; the consultant has no authority over the case, and the person consulting stays responsible for decisions (Caplan, 1970). The peer group's idea may be valuable, so the social worker brings it to the supervisor, where the decision belongs. It beats B because silently following the plan withholds clinically useful input from the person accountable for the case.

Takeaway: Consultation advises; supervision directs and shares accountability. A supervisee brings outside consultation to the supervisor rather than acting on it alone.

Sources: NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Standard 3 (Legal and Regulatory Issues): Liability; Caplan, G. (1970). The Theory and Practice of Mental Health Consultation. Basic Books, definition of consultation: consultee retains responsibility

Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.2 Supervision and consultation models: individual, peer, group · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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4. INT-1683 (rev 1) · Best answer: B

In supervision, an associate describes rushing to end sessions whenever clients cry. The supervisor asks what the associate notices in themself at those moments and how those reactions shape the work, keeping the focus on the associate's practice rather than personal history. In Bernard's discrimination model, which supervisor role does this **MOST** likely show?

A. Teacher role

B. Counselor role

C. Consultant role

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). In the teacher role, the supervisor instructs, models or gives direct feedback on skills, such as demonstrating how to stay with a crying client. The supervisor here invites reflection on internal reactions instead of instructing.

B **Correct.** Bernard's discrimination model has the supervisor shift among three roles, teacher, counselor and consultant, depending on what the supervisee needs. In the counselor role, the supervisor helps the supervisee examine internal reactions that affect the work, while keeping supervision distinct from therapy. Asking what the associate notices internally when clients cry fits this role. It beats C because the consultant role is a collegial exchange in which the supervisee leads and the supervisor offers options, not an exploration of the supervisee's reactions (Bernard & Goodyear, 2019).

C **Incorrect** (trap: Wrong model or concept). The consultant role treats the supervisee as a colleague who sets the agenda while the supervisor offers resources or alternatives. It suits experienced workers solving a case problem, not the examination of a personal reaction.

Takeaway: In Bernard's discrimination model, the counselor role addresses the supervisee's own reactions that affect practice, without becoming therapy.

Sources: Bernard, J. M., & Goodyear, R. K. (2019). Fundamentals of Clinical Supervision (6th ed.). Pearson, Discrimination model: teacher, counselor and consultant roles

Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.2 Supervision and consultation models: individual, peer, group · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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5. INT-1500 (rev 1) · Best answer: C

A hospital patient recovering from a leg amputation says, 'Everyone keeps telling me how lucky I am to be alive.' The social worker responds, 'Part of you may be grieving a loss that nobody around you has made room for yet.' Which skill does the social worker's response demonstrate?

A. Reciprocal empathy
B. Interpretation
C. Additive empathy
D. Paraphrasing

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Reciprocal, or interchangeable, empathy reflects the feeling and content the client actually expressed, at the same depth. A reciprocal reply here would name frustration with what others keep saying. This response adds a feeling the patient did not state.

B **Incorrect** (trap: Wrong model or concept). Interpretation offers a new explanation, often linking present reactions to patterns or past experience. This response stays with a present, implied feeling and offers no causal frame, so it is closer to additive empathy than to interpretation.

C **Correct.** Additive empathy goes a step past what the client said, naming feelings or meanings that are implied but unspoken. The patient voiced irritation at other people's comments; the social worker names an unstated grief and the sense that no one has allowed it. Going beneath the surface in this way is additive. It beats A because reciprocal empathy would mirror only the expressed feeling, irritation with others, without adding the deeper meaning (Hepworth et al.).

D **Incorrect** (trap: Wrong model or concept). Paraphrasing restates the content of what the client said, such as others calling the patient lucky. The response here centers on an unspoken feeling rather than restating content.

Takeaway: Reciprocal empathy mirrors the feeling a client expressed; additive empathy names implied, unspoken feelings and fits once rapport exists.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Additive empathy, interpretation, and confrontation

Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.2 Building and sustaining the helping relationship (empathy, rapport, acceptance) · **Qualifier** none · **Cognitive** recall · **Difficulty** 3 of 3

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6. ASM-0113 (rev 3) · Best answer: C

On an inpatient psychiatric unit, a social worker gathers history for a 22-year-old patient. The patient has heard voices and believed neighbors were spying for the past 4 months. A parent reports that for about 5 months before the voices began, the patient withdrew from friends, stopped attending classes and talked about strangers sending hidden signals. There is no substance use, mood episode or medical cause. What is the **MOST** likely diagnosis?

A. Brief psychotic disorder
B. Schizophreniform disorder
C. Schizophrenia
D. Delusional disorder

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Brief psychotic disorder lasts at least 1 day but less than 1 month, with full return to prior functioning. Two months of active psychosis after a long decline rules it out.

B **Incorrect** (trap: Near-miss diagnosis). Schizophreniform disorder covers a total illness of 1 month to under 6 months. It fits only if the 4 active months are counted alone, but the prodromal period counts toward duration, bringing the total to about 9 months.

C **Correct.** Schizophrenia requires continuous signs of the disturbance for at least 6 months, including at least 1 month of active symptoms, and the 6 months can include prodromal symptoms such as social withdrawal and odd, milder beliefs (DSM-5-TR). Here, about 5 months of prodrome plus 4 months of hallucinations and delusions totals about 9 months, with a clear drop in functioning. It beats B because schizophreniform disorder applies only when the whole episode, prodrome included, lasts less than 6 months; counting only the active phase is the trap.

D **Incorrect** (trap: Near-miss diagnosis). Delusional disorder involves delusions for at least 1 month without prominent hallucinations or the broader decline seen here. Hearing voices, withdrawal and dropping out of classes point elsewhere in the schizophrenia spectrum.

Takeaway: Schizophrenia's 6-month duration counts prodromal and residual periods, not just active psychosis; under 6 months total suggests schizophreniform disorder.

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizophrenia; DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizophreniform Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

7. ASM-1564 (rev 1) · Best answer: B

A social worker is writing a biopsychosocial formulation for a 71-year-old with depression. The client's parent had depression, and symptoms began after a hip fracture last spring. Since then the client rarely leaves the house, and mood dips further with each week spent alone indoors. A neighbor visits daily. In a 4P formulation, how is the client's staying home classified?

A. Precipitating factor
B. Perpetuating factor
C. Predisposing factor
D. Protective factor

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Precipitating factors are events just before onset that trigger the problem, here the hip fracture. Staying home came afterward and keeps the depression going, so it belongs elsewhere.

B **Correct.** In a 4P formulation, perpetuating factors are those that keep current difficulties going, such as ongoing behaviors and thinking patterns (Macneil et al., 2012). The client's withdrawal deepens low mood week by week, so it sustains the depression rather than starting it. Sorting biological, psychological and social data this way links the assessment to treatment targets. It beats A because the precipitating factor is the event that set off the episode, the hip fracture, not the pattern that followed.

C **Incorrect** (trap: Wrong model or concept). Predisposing factors are background vulnerabilities, such as the parent's history of depression in this case. They raise risk before any trigger occurs.

D **Incorrect** (trap: Wrong model or concept). Protective factors are strengths that buffer the problem, such as the neighbor's daily visits. Withdrawal works against recovery, so it is not protective.

Takeaway: A 4P formulation sorts biopsychosocial data into predisposing, precipitating, perpetuating and protective factors that point to treatment targets.

Sources: Macneil, C. A., Hasty, M. K., Conus, P., & Berk, M. (2012). Is diagnosis enough to guide interventions in mental health? Using case formulation in clinical practice. BMC Medicine, 10, 111, case formulation: predisposing, precipitating, perpetuating, protective factors

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.7 Biopsychosocial assessment framework and its components · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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8. ASM-1027 (rev 1) · Best answer: A

In private practice, a client seeks help after receiving an eviction notice. The client feels worried and sad, keeps working full time, sleeps well and is actively searching for housing. The client says the worry 'makes sense given the notice.' After a full assessment, the social worker must record the findings, and the client's insurance requires a diagnosis for payment. How should the social worker record the assessment?

A. Record a Z-code condition and no mental disorder
B. Record adjustment disorder because the stressor is identifiable
C. Record unspecified anxiety disorder pending further sessions

Why each option is right or wrong

A **Correct.** A diagnosis must reflect the assessment, and records must be accurate (NASW Code 3.04(a), 4.04). DSM-5-TR lists housing problems among other conditions that may be a focus of clinical attention (Z codes), which name the clinical focus without labeling a mental disorder. The client works, sleeps well and sees the worry as fitting the eviction, so no disorder's criteria are met. It beats B because adjustment disorder needs distress out of proportion to the stressor or real impairment, not just an identifiable stressor; using it to secure payment would misstate the record.

B **Incorrect** (trap: Near-miss diagnosis). An identifiable stressor is necessary but not enough. Adjustment disorder also requires distress out of proportion to the stressor or significant impairment, so recording it here would be inaccurate, whatever the payment pressure.

C **Incorrect** (trap: Misreads law or ethics). Unspecified anxiety disorder still requires anxiety symptoms that cause clinically significant distress or impairment. It is not a placeholder for proportionate worry, and using it this way would make the record inaccurate.

Takeaway: Record only what the assessment supports; use Z codes for a clinical focus when no disorder's criteria are met, even under payment pressure.

Sources: DSM-5-TR (APA, 2022), Other Conditions That May Be a Focus of Clinical Attention: Housing Problems; NASW Code of Ethics (2021), 3.04(a); NASW Code of Ethics (2021), 4.04
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** none · **Cognitive** application · **Difficulty** 2 of 3

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9. ETH-1678 (rev 1) · Best answer: C

At the social worker's invitation, a 15-year-old Black adoptee in a family session describes being followed through a store by a security guard. The white adoptive parent responds, 'You're overthinking it. In this family, we don't see color.' The teen looks away and stops talking. What should the social worker do **NEXT**?

A. Meet with the parent alone later to address the color-blind view
B. Help the teen plan ways to respond calmly to store security
C. Slow the exchange and help the parent hear the teen's experience

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Coaching the parent privately about color-blind parenting may be valuable, and it can follow. Waiting until then leaves the teen's account dismissed in front of the worker, which can confirm the teen's choice to go silent.

B **Incorrect** (trap: Good but not best). Planning for encounters with security may matter later, if the teen wants it. Offered now, it skips past the parent's dismissal and implies the teen must manage the problem alone, which can feel like a second invalidation.

C **Correct.** A color-blind reply to a report of racial profiling is a microinvalidation: it dismisses the person's experience of race, and from a parent it can teach a child to stop sharing (Sue et al., 2007). The teen's withdrawal shows the harm is happening now, so the worker steps in, slowing things down and helping the parent listen without

correcting (NASW Code 1.05(b)). It beats A because a later private meeting may help the parent, but the teen would leave this session having learned that raising race shuts the family down.

Takeaway: When a family member dismisses a client's experience of racism in session, intervene in the moment so the client's account is heard.

Sources: Sue, D. W., Capodilupo, C. M., Torino, G. C., Bucceri, J. M., Holder, A. M. B., Nadal, K. L., & Esquilin, M. (2007). Racial microaggressions in everyday life: Implications for clinical practice. American Psychologist, 62(4), 271-286, microinvalidation: color blindness; NASW Code of Ethics (2021), 1.05(b)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.6 Privilege, implicit bias, microaggressions and their client impact · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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10. ASM-1568 (rev 1) · Best answer: C

PRETEST — unscored on this form; do not count it

On an inpatient psychiatric unit, a client tells the social worker during an assessment interview, 'Sometimes I just lose it, and then it's over.' The client offers nothing more and looks at the social worker, waiting. What should the social worker do **FIRST**?

A. Reflect the client's frustration about losing control
B. Ask whether the client hurts anyone when this happens
C. Ask the client to describe a recent time they lost it
D. Suggest the anger management group on the unit

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Reflection builds rapport, but reflecting frustration about losing control assigns a feeling and meaning the client has not stated. Accurate reflection depends on first understanding what the client meant.

B **Incorrect** (trap: Right action, wrong time). Specific risk questions matter on an inpatient unit and may come next. Asked first, this closed question assumes the phrase means aggression and could steer the client toward or away from disclosure.

C **Correct.** Clarifying, or seeking concreteness, turns vague words into specific behavior and meaning (Hepworth et al.). 'Lose it' could mean anger, panic, dissociation, crying or self-harm, and 'then it's over' is just as unclear. Asking for a recent example lets the client define the experience before the social worker responds to an assumption. It beats B because a closed question about hurting others presumes the meaning is aggression; if the example points to danger, specific risk questions follow.

D **Incorrect** (trap: Skips assessment). An anger group assumes 'losing it' means anger. The phrase could describe panic, dissociation or self-harm, so recommending a service before clarifying risks a poor match and may miss a safety concern.

Takeaway: When a client uses vague or loaded phrases, ask for a concrete example before reflecting, assuming a meaning or recommending services.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Verbal following, exploring, and focusing skills: seeking concreteness
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.2 Interviewing skills: clarifying, validating, confronting, reflecting, interpreters · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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11. INT-1131 (rev 1) · Best answer: B

A 78-year-old hospital patient recovering from a fall has agreed to talk with the social worker about safety at home. When the social worker mentions grab bars and a shower chair, the patient snaps, 'You sound just like my kids. Everyone is telling me what to do.' Which response is **MOST** appropriate for a social worker using motivational interviewing?

A. Explain the fall risks again in simpler, clearer terms
B. Emphasize that changes at home are the patient's decision
C. Ask what it would take for the patient to accept the changes

Why each option is right or wrong

A **Incorrect** (trap: Too directive). Repeating the risks is the righting reflex: the urge to persuade. When a patient already feels lectured, more information tends to strengthen the pushback. Information works better later, offered with permission.

B **Correct.** MI separates sustain talk about a behavior from discord, which is friction in the relationship itself. The patient's complaint about being told what to do signals discord, and MI responds by changing direction, often by emphasizing personal choice (TIP 35, Ch. 3). Affirming that decisions about the home belong to the patient eases the pressure and keeps the conversation open. It beats C because asking what it would take still pushes toward the worker's goal while the patient feels pushed.

C **Incorrect** (trap: Misuses the technique). Evoking the patient's own reasons is a core MI skill, but this question presses toward acceptance while the relationship is strained. Discord calls for easing pressure first; evoking can resume once the patient feels respected.

Takeaway: When a client signals discord with the worker, ease pressure by emphasizing autonomy before trying to evoke change.

Sources: SAMHSA TIP 35 (2019), Ch. 3: Motivational Interviewing as a Counseling Style (discord; emphasizing autonomy)
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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12. ETH-1557 (rev 1) · Best answer: A

A private practice client owes for 6 sessions despite a written fee agreement and two talks about the balance. The social worker had planned to end services for nonpayment at the end of this month. Today the client reports thoughts of suicide most days since losing a job last week. How should the social worker handle the planned termination?

A. Postpone ending services while the risk is addressed
B. End services as planned and provide crisis line numbers
C. Refer the client to a sliding-scale clinic's waitlist

Why each option is right or wrong

A **Correct.** In fee-for-service settings, the Code allows ending services for nonpayment only when the financial terms were clear, the consequences were discussed, and the client poses no imminent danger to self or others (NASW Code 1.17(c)). This client's new, frequent suicidal thoughts mean the worker cannot conclude that no danger exists, so termination waits while risk is assessed and treated. It beats C because a waitlist leaves a client at heightened risk with no current provider; a transfer can follow once the client is stable and a new provider accepts the case (1.17(b)).

B **Incorrect** (trap: Ignores risk). The fee terms were clear, but ending care for a client who just disclosed frequent suicidal thoughts ignores the Code's danger condition. Crisis numbers do not replace an ongoing treatment relationship at a time of heightened risk.

C **Incorrect** (trap: Hands off too soon). A low-fee referral is a sensible step once the client is stable. Placing a client with new suicidal thoughts on a waitlist leaves them without care when risk is highest.

Takeaway: Ending services for nonpayment requires clear terms, discussed consequences and no imminent danger; new suicide risk postpones termination.

Sources: NASW Code of Ethics (2021), 1.17(c); NASW Code of Ethics (2021), 1.17(b)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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13. INT-1562 (rev 2) · Best answer: B

An adult with schizophrenia has been clinically ready to leave a state psychiatric hospital for 5 months and wants to live in the community. The treatment team agrees. The only barrier is a waitlist for state-funded supported housing. A nursing facility has an open bed, and hospital administrators suggest transferring the client there. What is the social worker's **BEST** action?

A. Support the nursing facility transfer as a less restrictive setting
B. Press the state for supported housing under disability rights law
C. Keep the client in the state hospital until housing opens
D. Discharge the client to a shelter with outpatient follow-up

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). A nursing facility may look less restrictive than a state hospital, but it is still an institution, and the client wants community living. The integration mandate calls for the most integrated setting appropriate to the client, not a different institution.

B **Correct.** Under the Americans with Disabilities Act (ADA), as interpreted in Olmstead v. L.C. (1999), unjustified institutional isolation of people with disabilities is discrimination. Public entities must serve people in the most integrated setting appropriate to their needs (28 CFR 35.130(d)) when professionals judge community care appropriate, the person does not oppose it, and it can be reasonably accommodated. The first two conditions are clearly met and a state program exists, so the social worker presses the state to provide supported housing. It beats A because a nursing facility swaps one institution for another.

C **Incorrect** (trap: Good but not best). Staying put avoids an unsafe discharge, but months of unnecessary hospitalization is itself the problem. Waiting without pressing for housing leaves the client institutionalized indefinitely.

D **Incorrect** (trap: Ignores risk). A shelter ends the hospital stay but puts the client's stability at risk. The most integrated appropriate setting means community living with the supports the client needs, not the least supported option.

Takeaway: Policies such as the ADA integration mandate support advocating for community-based services over unnecessary institutional care.

Sources: Olmstead v. L.C. ex rel. Zimring (1999), 527 U.S. 581; ADA Title II regulations, 28 CFR 35.130(d)
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.7 Effects of policy, regulation, legislation on practice and services · **Qualifier** BEST · **Cognitive** application · **Difficulty** 3 of 3

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14. ETH-1534 (rev 1) · Best answer: B

A surrogate makes a choice for a patient who can no longer decide. The surrogate picks what the patient would have wanted, based on the patient's values and past words. Which standard is this?

A. Best interests standard
B. Substituted judgment standard
C. Reasonable person standard

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). The best interests standard asks what would most benefit the patient, weighing benefits and burdens. It is used when the patient's wishes and values are unknown, such as for a person who never had decision-making capacity.

B **Correct.** Under substituted judgment, the surrogate decides as the patient would have decided, drawing on the patient's known values, statements and preferences (Beauchamp & Childress, 2019). It extends the patient's autonomy after capacity is lost, and it is what social workers help surrogates focus on when they support decisions consistent with the client's wishes (NASW Code 1.03(c)). It beats A because the best interests standard applies when the patient's wishes are unknown, so the surrogate weighs benefits and burdens instead.

C **Incorrect** (trap: Wrong model or concept). The reasonable person standard concerns how much a clinician must disclose for informed consent: what a reasonable patient would want to know. It does not guide a surrogate's choice for an incapacitated patient.

Takeaway: Substituted judgment: decide as the patient would have. Best interests: weigh benefits and burdens when the patient's wishes are unknown.

Sources: Beauchamp, T. L., & Childress, J. F. (2019). Principles of Biomedical Ethics (8th ed.). Oxford University Press, Ch. 5: standards of surrogate decision making; NASW Code of Ethics (2021), 1.03(c)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.6 Law and ethics around death and dying · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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15. ASM-1603 (rev 1) · Best answer: D

On a busy afternoon, a social worker is the only staff member covering walk-in triage at a community mental health center. Four people have signed in at about the same time and are waiting in the lobby. Which person should the social worker see **FIRST**?

A. A client with long-standing, unchanged passive death wishes, here for routine care
B. A client who ran out of an antidepressant 2 days ago and feels fine
C. A parent who needs a letter for a housing appointment tomorrow morning
D. An older adult whose confusion and fever began this morning

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Passive death wishes deserve attention, and this client should have a risk check today. Because the thoughts are long-standing and unchanged and the visit is routine, they are less urgent than a sudden medical change. New intent or a plan would move this client to the front.

B **Incorrect** (trap: Right action, wrong time). Two days without an antidepressant can bring discontinuation symptoms and calls for a prompt contact with the prescriber. The client feels fine, so the need is urgent but not emergent and can follow the medical concern.

C **Incorrect** (trap: Good but not best). A housing letter matters to the parent's stability and should be done today, but a deadline tomorrow morning leaves time. Administrative needs come after possible medical emergencies in any triage order.

D **Correct.** Triage puts possible medical emergencies ahead of stable concerns. Confusion that began within hours, together with fever, suggests delirium, which develops quickly, usually reflects an underlying medical condition and can be life-threatening without prompt care (DSM-5-TR; Kaplan & Sadock's Synopsis). Seeing this person first lets the social worker confirm the picture and arrange immediate medical evaluation. It beats A because that client's passive death wishes are long-standing and unchanged and the visit is routine, so a risk check right after the emergency is handled is appropriate.

Takeaway: In triage, sudden changes in mental status or physical condition come before stable, chronic or administrative needs, even serious-sounding ones.

Sources: DSM-5-TR (APA, 2022), Neurocognitive Disorders: Delirium; Boland, R., Verduin, M. L., & Ruiz, P. (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.). Wolters Kluwer, delirium
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.2 Goal setting, treatment planning, triage, and service plans · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

16. ETH-1011 (rev 2) · Best answer: A

PRETEST — unscored on this form; do not count it

A social worker at an opioid treatment program gets a call from an emergency department physician. A program client was brought in unresponsive, and the physician asks for the client's current medications to guide treatment. The client has signed no consent for the hospital. What is the **BEST** response?

A. Provide the information needed for emergency care
B. Decline until the client can sign a written consent
C. Confirm that the person is enrolled, but share nothing more
D. Ask the client's emergency contact to give consent

Why each option is right or wrong

A **Correct.** 42 CFR Part 2 lets a program disclose patient information without consent to medical personnel as needed to meet a bona fide medical emergency in which the patient cannot consent (42 CFR 2.51). An unresponsive client in an emergency department meets that standard, and the physician needs current medications to treat safely. The program then documents the disclosure. A beats B because waiting for consent from someone who cannot give it risks the client's life. The Code also permits disclosure to prevent serious, imminent harm (NASW Code 1.07(c)).

B **Incorrect** (trap: Misreads law or ethics). Part 2 usually requires written consent, which makes this tempting. The medical emergency exception exists for exactly this case: the client cannot consent, and treatment depends on the information.

C **Incorrect** (trap: Good but not best). Confirming enrollment discloses protected information without helping the physician treat. When disclosure is justified, it should include what the emergency requires, such as current medications.

D **Incorrect** (trap: Ignores risk). An emergency contact usually has no legal authority to consent for an adult, and searching for one delays care. The emergency exception allows direct disclosure to the treating physician.

Takeaway: In a bona fide medical emergency, a substance use program may share needed information with treating medical staff without consent, then document it.

Sources: 42 CFR Part 2, 42 CFR 2.51; NASW Code of Ethics (2021), 1.07(c)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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17. ASM-1683 (rev 1) · Best answer: A

A parent calls an outpatient clinic about a 16-year-old on its 3-week waitlist. The teen has posted goodbye messages online and given away a prized gaming system. When the parent asked directly, the teen said they have been saving pills to take this weekend. The teen is at home now. What should the intake social worker do **FIRST**?

A. Have the parent call emergency services now and stay with the teen
B. Book the teen into the clinic's first urgent opening tomorrow morning
C. Complete a full suicide risk screening with the parent over the phone

Why each option is right or wrong

A **Correct.** Triage sorts by urgency, and a stated plan, gathered means, a time frame and goodbye behavior make this emergent rather than urgent or routine. The parent has already asked directly and the teen confirmed saving pills, so the key risk facts are known. Having the parent call emergency services now, while staying with the teen, matches the response to the danger; crisis systems exist to respond in person to this level of risk (SAMHSA, 2020). It beats C because a longer phone screening would delay help the known facts already call for.

B **Incorrect** (trap: Ignores risk). An urgent next-day appointment fits risk without a plan or means. Here the teen has a plan, saved pills and a weekend time frame, so waiting until tomorrow leaves the danger unaddressed tonight.

C **Incorrect** (trap: Right action, wrong time). Structured screening is the right move when risk is unclear. The parent's direct question has already surfaced a plan, means and timing, so more screening by phone now delays emergency care.

Takeaway: Triage a suicide plan with means and a time frame as emergent: arrange immediate emergency help rather than an expedited appointment.

Sources: SAMHSA (2020) National Guidelines for Behavioral Health Crisis Care: Best Practice Toolkit, Core elements: someone to respond (mobile crisis)
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.2 Goal setting, treatment planning, triage, and service plans · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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18. ASM-1607 (rev 2) · Best answer: A

A 16-year-old and a parent come to a family service agency for help. The parent wants the plan to focus on raising the teen's grades. The teen wants the parent to 'stop yelling.' Both say that evenings at home have become tense and that they want more peace. What is the **MOST** appropriate way to begin setting goals?

A. Start with the goal they share: calmer evenings
B. Focus on grades first, since the parent consents to care
C. Start with the teen's goal of less yelling in the evenings

Why each option is right or wrong

A **Correct.** Goals are set collaboratively, and in family work a goal that every member endorses builds the joint commitment that later goals need (Hepworth et al.; NASW Code 1.02). Both people already agree that evenings are tense and want calm, so that shared goal unites them, and work on yelling and on homework routines can fit inside it. It beats C because opening with only the teen's goal aligns the social worker with one side and risks losing the parent, whose cooperation the plan needs.

B **Incorrect** (trap: Misreads law or ethics). A parent's consent to treatment does not make the parent's goal the plan's only focus. Opening with grades sidelines the teen, who must take part for anything to change, and signals that the social worker has taken the parent's side.

C **Incorrect** (trap: Boundary slip or collusion). The teen's concern matters, but leading with it casts the parent as the problem and invites the parent to disengage. Both concerns can be addressed within the goal the two already share.

Takeaway: In family goal setting, begin with a goal all members share to build joint commitment before addressing each person's own goal.

Sources: Hepworth, D. H., et al. Direct Social Work Practice: Theory and Skills (11th ed.). Cengage, negotiating goals with families; NASW Code of Ethics (2021), 1.02
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.2 Goal setting, treatment planning, triage, and service plans · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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19. ETH-1592 (rev 1) · Best answer: B

A white supervisee describes a Black client who wants help finding a better-paying job. The supervisee keeps steering sessions toward anger management, saying, 'I know what this client really needs.' The client has missed the last two sessions. What is the **MOST** appropriate response by the supervising social worker?

A. Reassign the client to a social worker with more experience
B. Help the supervisee reflect on privilege and power in this pattern
C. Direct the supervisee to follow the client's stated goals

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). Reassignment removes the learning opportunity and may disrupt the client's care. It fits if the supervisee cannot change course after supervision, not as a first response.

B **Correct.** Cultural competence standards ask social workers to recognize their own privilege and power and how these shape their work, and supervision supports that self-awareness (NASW Cultural Competence Standards, Standard 2; NASW Code 1.05(c)). The supervisee is replacing the client's goal with an assumption about what a Black client needs, and the client is pulling away. Helping the supervisee examine that pattern addresses its source and prepares them to follow the client's lead. It beats C because an order fixes this case but leaves the assumption unexamined for the next client.

C **Incorrect** (trap: Too directive). Returning to the client's goals is necessary, and the supervisor may say so. Giving the instruction alone leaves the supervisee's assumption about what the client 'really needs' unexamined.

Takeaway: Supervisors help supervisees examine how privilege and role power lead them to override clients' goals, not just correct one case.

Sources: NASW Standards and Indicators for Cultural Competence in Social Work Practice (2015), Standard 2: Self-Awareness; NASW Code of Ethics (2021), 1.05(c)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.5 Power differentials, racial privilege, internalized inferiority/superiority · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 3 of 3

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20. ETH-1644 (rev 1) · Best answer: B

A 68-year-old client at a county behavioral health clinic says that, as a child, county caseworkers removed the client from a Native family and placed the client with strangers, and the client's siblings were never found. The client has described this in detail over two sessions. Today the client asks the social worker, 'Does the county admit it was wrong?' What should the social worker do **FIRST**?

A. Explain how child welfare policies have changed since that time
B. Acknowledge that removals like this caused deep, lasting harm
C. Invite the client to say more about what the removal cost
D. Refer the question to county leaders for an official reply

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Changes such as the Indian Child Welfare Act are worth sharing, and the client may want to hear them. Offered first, they can sound like a defense of the agency and leave the question about wrongdoing unanswered.

B **Correct.** Truth and reconciliation starts with honest acknowledgment of harm by the institutions involved. Congress passed the Indian Child Welfare Act after finding that many Native children were removed by nontribal agencies, often without good cause, and placed in non-Native homes (25 U.S.C. 1901(4)). The client has already told the story in detail and now asks a direct question, so a plain acknowledgment of the harm comes first; NASW has likewise apologized for the profession's role in harming people of color (NASW, 2021). It beats A because describing reforms first sounds defensive and sidesteps the question.

C **Incorrect** (trap: Good but not best). Inviting more of the story is useful when a client has not yet been heard. This client has already described the loss over two sessions and now asks a direct question, so more exploration sidesteps it.

D **Incorrect** (trap: Hands off too soon). County leaders may give an official position later, and the client can be told how to ask. Handing off the question now leaves the client without an honest answer from the person in the room.

Takeaway: When a client asks whether an institution's past harm is admitted, answer honestly and acknowledge the harm before explaining reforms.

Sources: Indian Child Welfare Act of 1978, 25 U.S.C. 1901(4); NASW (2021). Undoing Racism Through Social Work: NASW Report to the Profession on Racial Justice Priorities and Action, apology
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.8 Restorative practice, truth and reconciliation with historically oppressed groups · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

21. INT-1550 (rev 1) · Best answer: C

A social worker at a rural aging services agency finds that several older adult clients miss medical appointments because the county has no public bus service. The county has several active congregations, a senior center and a hospital system. The agency director proposes buying a van and hiring a driver. What should the social worker do **FIRST**?

A. Write a grant proposal to purchase an agency van
B. Ask county commissioners to fund a public bus route
C. Inventory existing transportation options and possible partners
D. Arrange taxi vouchers for clients one appointment at a time

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). A grant for a van may become part of the plan, but writing it now skips learning what transportation already exists. Duplicating an existing service wastes scarce funds and can strain ties with groups that already give rides.

B **Incorrect** (trap: Right action, wrong time). Pressing for public transit is legitimate advocacy and may be a later or parallel step. It is slow and uncertain and will not help clients soon. A scan of current options also gives the request supporting data.

C **Correct.** Resource development starts by finding out what a community already has before building something new (Kretzmann & McKnight, 1993). Congregations, a senior center and a hospital system may already offer rides or could become partners, which makes any solution cheaper and longer lasting. It beats A because buying a van commits the agency to vehicle, driver and insurance costs before anyone knows whether existing options could meet the need. The inventory also gives any later grant or funding request the evidence it needs.

D **Incorrect** (trap: Good but not best). Vouchers solve individual trips and may help in the short term, but they treat a county-wide gap one case at a time. Building a lasting resource starts with understanding the options and partners already present.

Takeaway: Before creating a new service, map the community's existing resources and potential partners to avoid duplication and build on local assets.

Sources: Kretzmann, J. P., & McKnight, J. L. (1993). Building Communities from the Inside Out: A Path Toward Finding and Mobilizing a Community's Assets. ACTA Publications, asset mapping: inventory of individual, associational and institutional assets

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.24 Building service networks and community resources · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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22. ASM-1083 (rev 2) · Best answer: B

A rural client in their 60s, in treatment for depression after losing the family farm, reports passive thoughts of death most days. The client keeps several hunting rifles at home and refuses to sell them, saying hunting is part of who they are. The client's sibling lives nearby and owns a gun safe. What is the social worker's **BEST** response?

A. Accept the refusal and note the rifles in the chart
B. Discuss temporary storage at a relative's home
C. Arrange a short voluntary hospital stay for safety

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). Respecting the client's attachment to hunting matters, and documenting firearm access is necessary. Recording the rifles without acting leaves the most lethal method at hand for someone with near-daily thoughts of death.

B **Correct.** Lethal-means counseling asks at-risk people and their families to keep firearms away from the person until the crisis passes, often through temporary off-site storage with a relative or friend (Barber & Miller, 2014). Suicidal crises can be brief, so distance from a gun buys time. Temporary storage with the sibling honors the refusal to sell and the client's identity as a hunter while lowering risk. It beats A because noting the rifles without action leaves the means in place.

C **Incorrect** (trap: Skips assessment). Hospitalization fits high acute risk with intent or a plan. Passive thoughts of death, without stated intent or a plan, call for less restrictive steps, and a hospital stay does nothing about the rifles waiting at home.

Takeaway: For an at-risk client who keeps firearms, counsel temporary off-site storage with a trusted person until the risk passes.

Sources: Barber, C. W., & Miller, M. J. (2014). Reducing a suicidal person's access to lethal means of suicide: A research agenda. American Journal of Preventive Medicine, 47(3S2), S264-S272, Firearms: temporary off-site storage

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.1 Assessing risk of harm to self or others · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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23. INT-1003 (rev 1) · Best answer: D

A social worker and client agree to 16 sessions. Early on, they identify a recurring pattern: the client longs for closeness, expects rejection, and pulls away first. The social worker actively keeps sessions on this pattern, tracing it with the client's partner, with coworkers, and with the social worker. Which approach is the social worker **MOST** likely using?

A. Interpersonal psychotherapy
B. Task-centered practice
C. Solution-focused therapy
D. Time-limited dynamic psychotherapy

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Interpersonal psychotherapy is also time-limited and relationship-centered, but it links symptoms to a current problem area such as grief, a role dispute, or a role transition. It does not trace the pattern into the relationship with the worker.

B **Incorrect** (trap: Wrong model or concept). Task-centered practice is short-term and structured, but it works on client-chosen target problems through tasks done between sessions. It does not explore a recurring relational pattern or how it appears with the worker.

C **Incorrect** (trap: Wrong model or concept). Solution-focused work is brief and future-focused, building on exceptions and small steps. It avoids analyzing problem patterns, which is the core activity in this stem.

D **Correct.** Time-limited dynamic psychotherapy, a brief psychodynamic model, sets a session limit, keeps an active focus on one recurring maladaptive relational pattern,

and attends to how that pattern shows up with the worker (Levenson, 2017). Here the pattern is traced with the partner, coworkers, and the social worker, which brings in the transference. It beats A because interpersonal psychotherapy is also time-limited and relational, but it targets a current problem area, such as a role dispute or transition, rather than the relationship with the worker.

Takeaway: Time-limited dynamic psychotherapy, a brief psychodynamic model, focuses on one recurring relational pattern and examines it in the relationship with the worker.

Sources: Levenson, H. (2017). Brief Dynamic Therapy (2nd ed.). American Psychological Association, common features of brief dynamic therapy
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

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24. ASM-1043 (rev 3) · Best answer: D

Which change in a 74-year-old fits normal cognitive aging rather than a neurocognitive disorder?

- **A.** Getting lost while driving a familiar route
- **B.** Needing help to pay monthly bills
- **C.** Asking the same question minutes after an answer
- **D.** Recalling a name slowly but remembering it later

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Becoming lost on a well-known route suggests decline in memory or visuospatial skills. It goes beyond normal slowing and warrants a cognitive evaluation.

B **Incorrect** (trap: Near-miss diagnosis). Needing help with bills, when it reflects cognitive change, is lost independence in a complex daily task. That kind of decline marks major neurocognitive disorder rather than normal aging.

C **Incorrect** (trap: Near-miss diagnosis). Repeating a question minutes after hearing the answer shows trouble holding new information. This is a typical early sign of a neurocognitive disorder, not ordinary slowing.

D **Correct.** Normal aging brings slower processing and occasional delays in retrieving names or words that come back later, with no loss of independence. Neurocognitive disorders involve decline from a previous level, and major neurocognitive disorder interferes with independence in daily activities such as managing bills (DSM-5-TR). It beats C because quickly forgetting a just-answered question reflects impaired learning of new information, a common early sign of Alzheimer's disease rather than ordinary slowing.

Takeaway: Slower recall that resolves is normal aging; losing new information, getting lost or needing help with daily tasks suggests a neurocognitive disorder.

Sources: DSM-5-TR (APA, 2022), Neurocognitive Disorders: Major and Mild Neurocognitive Disorders
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.16 Typical vs. atypical development across the lifespan · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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25. ETH-1035 (rev 1) · Best answer: C

A social worker at a community youth center learns from a 7-year-old that a parent's partner burned the child's arm with a lighter, and sees a fresh round burn. The social worker informs the center director, who says staff will talk with the parent at pickup tomorrow and asks the social worker to hold off. What should the social worker do **NEXT**?

A. Wait until the director has spoken with the parent
B. Ask the child more questions to firm up the details
C. Call the child abuse hotline today

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Agency staff cannot delay or block an individual's reporting duty. Waiting for the director to talk with the parent postpones protection and may alert the person who harmed the child.

B **Incorrect** (trap: Skips assessment). Gathering more detail can feel careful, but the disclosure and the burn already meet the reporting standard. Extra questioning risks shaping the child's account; investigative interviews belong to child protective services and law enforcement.

C **Correct.** The duty to report known or suspected child abuse rests with each mandated reporter and is not met by telling a director (CAPTA, 42 U.S.C. 5106a(b)(2)(B)(i)). A disclosure plus a fresh burn clearly meets the threshold, and legal duties like this can outweigh other loyalties (NASW Code 1.01). C beats A because waiting lets a supervisor delay a mandated report, and contacting the household first could put the child at greater risk.

Takeaway: Each mandated reporter must report reasonable suspicion personally; a supervisor's plan or request never replaces or delays the report.

Sources: Child Abuse Prevention and Treatment Act, 42 U.S.C. 5106a(b)(2)(B)(i); NASW Code of Ethics (2021), 1.01
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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26. ETH-1671 (rev 2) · Best answer: A

Under the NASW Code of Ethics, when should a social worker avoid a dual relationship with a client or former client?

A. When it carries a risk of exploitation or harm to the client
B. When it begins less than 2 years after services have ended
C. When it involves social contact outside the office

Why each option is right or wrong

A **Correct.** The Code says to avoid dual or multiple relationships with clients or former clients when there is a risk of exploitation or potential harm, and when overlap cannot be avoided, the social worker sets clear, culturally sensitive boundaries (NASW Code 1.06(c)). The test is risk, not timing or setting. It beats B because the Code sets no waiting period after which a dual relationship becomes acceptable; time-based rules come from some state laws and other professions' codes, mostly about sexual relationships.

B **Incorrect** (trap: Misreads law or ethics). Waiting periods appear in some state rules and other professions' codes, usually for sexual relationships. The NASW Code uses a risk test for dual relationships and sets no time after which they become acceptable.

C **Incorrect** (trap: Misreads law or ethics). Incidental social contact, such as meeting at a store or a child's game, is not prohibited in itself. In small communities overlap is often unavoidable, and the Code asks the social worker to manage it with clear boundaries.

Takeaway: The Code's test for dual relationships is risk of exploitation or harm; unavoidable overlaps are managed with clear boundaries.

Sources: NASW Code of Ethics (2021), 1.06(c)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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27. ETH-1004 (rev 2) · Best answer: A

A social worker at a community mental health center receives a subpoena from an insurance company's attorney in a disability lawsuit brought by a client. The social worker calls the client, who says firmly that they do not want the records released. The response date is 10 days away. What should the social worker do **NEXT**?

A. Get legal advice on asserting the client's privilege
B. Send the records, since the subpoena is a legal demand
C. Discuss with the client why they oppose the release
D. Ask the client to contact the attorney who issued it

Why each option is right or wrong

A **Correct.** A subpoena must be answered, but it is not a court order and does not by itself authorize release. Once the client refuses consent, the worker asserts privilege on the client's behalf and gets legal advice on how to respond, such as objecting or seeking to quash, before the deadline (NASW Code 1.07(j); 45 CFR 164.512(e)(1)(ii)). A beats C because the client's decision is already clear; more discussion does not answer the subpoena, and the deadline keeps running.

B **Incorrect** (trap: Misreads law or ethics). An attorney's subpoena is a demand to respond, not a judge's order to disclose. Releasing records over the client's refusal, without a court order or the assurances HIPAA requires, breaches confidentiality.

C **Incorrect** (trap: Good but not best). Understanding the client's reasons can help the relationship, and it would be the right move if the client were unsure. Here the client has clearly refused, so the pressing task is a lawful response to the subpoena before the deadline.

D **Incorrect** (trap: Hands off too soon). The subpoena was served on the social worker, who must respond in time. Sending the client to the attorney who issued it hands off the worker's own duty, and that attorney represents the insurer, not the client.

Takeaway: When a client refuses to authorize release under subpoena, assert privilege and get legal advice before the response deadline.

Sources: NASW Code of Ethics (2021), 1.07(j); HIPAA Privacy Rule, 45 CFR 164.512(e)(1)(ii)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 3 of 3

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28. ETH-1130 (rev 1) · Best answer: A

At a clinic intake, a client explains that their partner monitors the home phone and mail and becomes violent over medical bills. The client asks the clinic to contact them only by work email. The front desk says its system sends automatic reminder calls and letters to the home. What should the social worker do **FIRST**?

- **A.** Have the client's contact method changed before any reminder goes out
- **B.** Develop a safety plan for what to do if the partner finds a letter
- **C.** Explain the reminder system and ask the client to accept home contact

Why each option is right or wrong

A **Correct.** HIPAA requires health care providers to accommodate reasonable requests to receive communications by other means or at other locations, and they may not require an explanation (45 CFR 164.522(b)). A reminder call or letter could reach the partner and put the client in danger, so the contact change must come before anything else. A beats B because safety planning is valuable but comes later; preventing the risky contact in the first place protects the client now.

B **Incorrect** (trap: Right action, wrong time). Safety planning belongs in the work with this client and should follow soon. Doing it first, while reminders are still set to go to the home, plans around a danger the clinic can simply remove.

C **Incorrect** (trap: Ignores risk). A clinic's system settings do not override the client's right to a reasonable request for confidential contact. Asking the client to accept home reminders could expose them to violence.

Takeaway: Providers must honor reasonable requests for confidential contact methods; change the settings before any communication goes out.

Sources: HIPAA Privacy Rule, 45 CFR 164.522(b)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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29. INT-1180 (rev 1) · Best answer: B

An 80-year-old client comes to a senior center for help with a benefits renewal form that is due tomorrow. For 20 minutes the client has talked about past jobs and old friends, and the form is still blank. The client seems to enjoy the visit and has no other appointments today. Which response is **MOST** helpful?

- **A.** Keep listening warmly and let the client set the pace of the visit
- **B.** Briefly reflect the stories' meaning, then return to the form
- **C.** Explain that the form comes first and the stories can wait

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Following the client's lead fits open-ended sessions and may be what the client enjoys most. Here the client came for a form due tomorrow, and letting the visit drift risks a lapse in benefits the client is counting on.

B **Correct.** Focusing brings a conversation back from tangents to the task the client came for, and it works best when the worker first shows the client was heard (Hepworth et al.). A brief reflection honors what reminiscing means to this client, and steering back to the form protects benefits that are due tomorrow. It beats A because following the client's lead is usually right, but here open-ended listening lets an urgent goal the client chose slip away; active listening includes judging when to refocus.

C **Incorrect** (trap: Misuses the technique). Returning to the task is right, but doing it without first acknowledging the stories can feel dismissive to an older adult who values the conversation, and it may make the rest of the visit feel rushed and impersonal.

Takeaway: When tangents threaten an urgent client goal, briefly reflect what was shared and then refocus; listening includes knowing when to redirect.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Focusing skills
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.3 Active listening and observation skills · **Qualifier** MOST · **Cognitive** reasoning · **Difficulty** 2 of 3

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30. INT-1651 (rev 2) · Best answer: C

PRETEST — unscored on this form; do not count it

Two days after discharge from a psychiatric unit, a client with bipolar disorder tells the social worker on a scheduled follow-up call that the pharmacy would not fill the new mood stabilizer. The insurer needs a prior authorization form from the prescriber. The client has called the insurer twice, wants to restart the medication, and says, 'I've been on the phone all day. I don't have the energy to keep calling.' What should the social worker do **FIRST**?

A. Move the client's outpatient appointment to an earlier date
B. Coach the client to ask the prescriber's office for the form
C. Call the prescriber's office today about the form
D. Ask how the client feels about taking a mood stabilizer

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Seeing the prescriber sooner may help and can be arranged, but the earliest opening may still be days away. The form can be handled today, so moving the visit first leaves the client without the medication in the meantime.

B **Incorrect** (trap: Good but not best). Supporting self-advocacy is usually a strong first move when a client has the energy and time allows. Here the client has made repeated calls and is running out of energy, and the barrier needs only a form from the prescriber. Coaching fits once the medication gap is closed.

C **Correct.** Follow-up after discharge exists to catch and remove barriers before they undo the plan; case management standards call for monitoring service delivery and acting when a plan stalls (NASW, 2013, Standard 6). The client wants the medication, has already called the insurer twice, and is running out of energy, while the weeks

after discharge are a period of raised risk (Chung et al., 2017), so a lapse in the new mood stabilizer should not wait. Calling the prescriber's office with the client's agreement clears the block today. It beats B because coaching builds skills, but this client has done their part and is losing steam.

D **Incorrect** (trap: Good but not best). Exploring feelings about medication fits when a client is unsure about taking it. This client has said they want to restart, and the problem is a paperwork barrier. Asking about feelings now delays the practical fix.

Takeaway: When follow-up after discharge finds a barrier to treatment the client wants, remove it promptly rather than adding steps for an exhausted client.

Sources: NASW Standards for Social Work Case Management (2013), Standard 6: Service Planning, Implementation, and Monitoring; Chung, D. T., Ryan, C. J., Hadzi-Pavlovic, D., Singh, S. P., Stanton, C., & Large, M. M. (2017). Suicide rates after discharge from psychiatric facilities: A systematic review and meta-analysis. JAMA Psychiatry, 74(7), 694-702, whole article

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.22 Discharge planning, aftercare, wraparound, follow-up · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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31. ASM-1539 (rev 1) · Best answer: A

At intake, a client seeks help for anxiety after losing a job. The client mentions living with two partners in a committed polyamorous relationship. Both partners have offered to help with bills and job leads. What should the social worker do **FIRST**?

A. Include both partners, as the client defines them, in assessing supports

B. Assess whether the arrangement with both partners is causing the anxiety

C. Refer the client to a provider who specializes in polyamory

Why each option is right or wrong

A **Correct.** Cultural humility means treating clients as experts on their own lives and cultures (NASW Code 1.05(c)), and the Code calls for understanding social diversity, including marital status (1.05(d)). This client presents with job-loss anxiety and names two partners who are actively helping, so the assessment should map them as supports in the way the client defines the relationship. It beats B because singling out the relationship structure as a likely cause has no basis in the stem and signals bias; relationship stress can be explored later if the client raises it.

B **Incorrect** (trap: Skips assessment). Relationships can add stress for any client, and the topic may come up. Assuming that a polyamorous structure is the cause, when nothing in the stem points there, pathologizes a relationship form and can damage trust at intake.

C **Incorrect** (trap: Hands off too soon). Referral fits when a worker lacks competence for the presenting concern. Job-loss anxiety is ordinary practice, and referring because of the client's relationship structure hands off the client for who they are rather than for what they need.

Takeaway: Assess consensually nonmonogamous relationships as the client defines them, including partners as supports; do not presume the structure causes the problem.

Sources: NASW Code of Ethics (2021), 1.05(c)-(d)

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.3 Relationship dynamics and development: families, couples, groups, polyamory · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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32. ETH-1013 (rev 1) · Best answer: C

Records kept by a social worker employed by a public school district are generally governed by which federal privacy law?

A. The HIPAA Privacy Rule
B. 42 CFR Part 2
C. FERPA

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). HIPAA governs most health care providers, which makes it the usual answer. Records that a public school keeps as education records are excluded from HIPAA's protected health information, so FERPA applies instead.

B **Incorrect** (trap: Wrong model or concept). 42 CFR Part 2 protects records of federally assisted substance use disorder programs. It does not govern a school social worker's general student records.

C **Correct.** Health records kept by school employees, or by providers acting for a public school, are generally education records under FERPA, the Family Educational Rights and Privacy Act. Most elementary and secondary schools are not HIPAA covered entities, and education records are excluded from HIPAA's definition of protected health information (Joint Guidance, HHS and U.S. Department of Education, 2019). C beats A because HIPAA governs most health care settings, but not education records kept by a school.

Takeaway: A public school social worker's records are usually FERPA education records, not HIPAA protected health information.

Sources: U.S. Department of Health and Human Services & U.S. Department of Education (2019). Joint Guidance on the Application of FERPA and HIPAA to Student Health Records, Education records and the HIPAA definition of PHI; Family Educational Rights and Privacy Act (FERPA), 20 U.S.C. 1232g; 34 CFR Part 99
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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33. INT-1661 (rev 1) · Best answer: B

A single-system design sets a goal for one behavior. The goal then moves in steps, and each new level starts only after the client meets the current one. What is this design called?

A. Multiple-baseline design
B. Changing-criterion design
C. ABC design
D. ABAB reversal design

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). A multiple-baseline design starts the same intervention at staggered times across behaviors, settings or clients. It does not raise or lower a goal for one behavior in steps.

B **Correct.** A changing-criterion design sets a performance level and then shifts it step by step once the client meets each level, such as lowering a daily cigarette limit from 20 to 15 to 10. When behavior tracks each new level closely, that pattern supports the intervention's effect without withdrawing it (Kazdin, 2011). It beats C because an ABC

design brings in a different intervention in each phase, while this design keeps one intervention and moves the goal.

C **Incorrect** (trap: Wrong model or concept). An ABC design follows the baseline with one intervention (B) and then a different one (C). The phases differ by treatment, not by a stepped goal for the same treatment.

D **Incorrect** (trap: Wrong model or concept). An ABAB design removes and then restores the intervention to show that behavior shifts with it. It tests effect by withdrawal, not by step-by-step goals.

Takeaway: A changing-criterion design shows effect when behavior follows each stepwise target while the same intervention continues.

Sources: Kazdin, A. E. (2011). Single-Case Research Designs: Methods for Clinical and Applied Settings (2nd ed.). Oxford University Press, changing-criterion designs
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.1 Evaluating progress and effectiveness of intervention and service plans · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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34. ETH-1578 (rev 1) · Best answer: A

During a child welfare team meeting, staff are deciding whether a family can keep a safety plan at home or needs a placement. A colleague says, 'Families from that housing complex never follow through.' Others nod, and the supervisor moves toward a vote. The family's record shows they have met each plan task so far. What should the social worker do **FIRST**?

A. Name the generalization and redirect to the case facts
B. Speak with the colleague privately after the meeting
C. Report the colleague's remark to the program director

Why each option is right or wrong

A **Correct.** Anti-racist and anti-oppressive practice calls for challenging stereotypes when they shape decisions about clients; the Code bars derogatory talk about clients and condoning discrimination (NASW Code 1.12; 4.02). Here a group stereotype is steering a placement decision in real time, and the family's own record contradicts it. Naming the generalization and returning the team to the evidence protects the family now. It beats B because a private talk later may help the colleague, but by then the vote would already rest on the stereotype.

B **Incorrect** (trap: Right action, wrong time). A private conversation is a respectful way to address a colleague and can follow. Waiting until after the meeting lets a stereotype shape a placement vote that is happening now.

C **Incorrect** (trap: Hands off too soon). Going to the director skips the chance to correct the decision in the room and bypasses a direct step. A formal report fits if the pattern continues after it has been addressed directly.

Takeaway: When a stereotype is shaping a decision about a client in the moment, challenge it then and redirect to the evidence.

Sources: NASW Code of Ethics (2021), 1.12; NASW Code of Ethics (2021), 4.02
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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35. ETH-1688 (rev 2) · Best answer: B

An outpatient client with bipolar I disorder is admitted to a state psychiatric hospital and is expected to stay about 6 more weeks. The client has signed a release for the inpatient team. Agency policy automatically closes cases after 30 days without contact, and day 30 is next week. What is the social worker's **MOST** appropriate action?

A. Close the case under the policy and mail a letter inviting return
B. Ask for an exception that keeps the case open through discharge
C. Call the client weekly to keep the case from closing

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Automatic closure fits clients who stop coming without explanation. Here the absence has a known cause and an expected end, and closing would cut the client off just before a high-risk move out of the hospital.

B **Correct.** Social workers make reasonable efforts to keep services going when illness interrupts them, and they work to keep agency policies from interfering with ethical practice (NASW Code 1.15; 3.09(d)). A hospitalization is not a dropout. Asking for an exception, then using the signed release to plan discharge follow-up with the hospital, keeps care continuous at a high-risk transition. It beats C because calls timed to reset the closure clock work around the policy instead of addressing it, and they press contact on a client who may be too ill to engage.

C **Incorrect** (trap: Good but not best). Keeping in touch during a hospital stay can help, but calls arranged to stop the closure clock serve the policy rather than the client, who may be too ill to engage. Raising the conflict openly fixes the rule for this client and others who are hospitalized.

Takeaway: When illness interrupts services, protect continuity: seek exceptions to rigid closure rules and coordinate the care transition rather than closing the case.

Sources: NASW Code of Ethics (2021), 1.15; NASW Code of Ethics (2021), 3.09(d)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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36. INT-1107 (rev 1) · Best answer: A

In an integrated primary care clinic, a client with diabetes and depression has just agreed to a plan for daily walks and a mood log. While agreeing, the client speaks softly, looks at the floor, and rubs the back of their neck. What should the social worker do **NEXT**?

A. Ask which part of the plan might be hardest to do
B. Ask the client why they seem unsure about the plan
C. Give the client a written copy of the agreed plan

Why each option is right or wrong

A **Correct.** The client's soft voice, lowered gaze and neck rubbing do not match the spoken agreement. When words and nonverbal cues conflict, the nonverbal message often carries doubts the client has not voiced. A question that assumes some difficulty makes it easy to name barriers without seeming to back out (Hepworth et al.). It beats B because 'why' questions tend to sound like demands for justification and can make clients defensive, so the doubts are less likely to surface.

B **Incorrect** (trap: Misuses the technique). This option notices the mixed message, but 'why' questions tend to put clients on the defensive and ask them to justify their feelings. An open question about what might get in the way invites the same information more safely.

C **Incorrect** (trap: Right action, wrong time). A written copy supports follow-through, but handing it over now treats the agreement as settled when the client's nonverbal cues suggest doubts. It fits once barriers have been discussed and the plan adjusted.

Takeaway: When nonverbal cues contradict a client's verbal agreement, invite possible barriers with an open, nonjudgmental question before finalizing the plan.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Physical attending and nonverbal communication; Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Avoiding counterproductive communication patterns: 'why' and leading questions

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.2 Verbal and nonverbal communication; congruence, body language · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 3 of 3

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37. ASM-1053 (rev 2) · Best answer: A

An adult with bipolar I disorder comes to a community mental health center for help with worsening depression. The client drinks 2 or 3 beers most evenings and has never had withdrawal symptoms. The intake coordinator says center policy sends anyone who drinks to an addiction program before mental health care. What is the social worker's **BEST** response?

A. Assess both the mood episode and alcohol use today

B. Refer the client to the addiction program first

C. Help the client find an integrated program elsewhere

Why each option is right or wrong

A **Correct.** Co-occurring disorders are expected, and there is no wrong door: whichever program a person enters should assess and begin to address both conditions (SAMHSA, 2021). This client has worsening bipolar depression and regular evening drinking without withdrawal history, so an integrated intake now meets the immediate need. It beats C because sending the client to another program, even a good one, delays care for a worsening mood episode and risks losing the client between agencies. The policy itself can be raised afterward through channels.

B **Incorrect** (trap: Misreads law or ethics). Sequential care, addiction first, follows the policy but leaves a worsening bipolar depression untreated and risks losing the client in transfer. Policy does not outweigh the duty to serve a client who needs care now.

C **Incorrect** (trap: Hands off too soon). Linking to an integrated program sounds client-centered but still turns the client away from a door they already came through. A transfer fits only if assessment shows needs this center cannot meet.

Takeaway: There is no wrong door for co-occurring disorders: the program a client enters assesses and begins treating both conditions.

Sources: SAMHSA (2021). Advisory: Substance Use Disorder Treatment for People With Co-Occurring Disorders (PEP20-06-04-006), Key Messages
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.6 Comorbid (co-occurring) disorders and conditions · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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38. INT-1022 (rev 2) · Best answer: B

PRETEST — unscored on this form; do not count it

A client with schizophrenia is moving into a supported apartment and wants to learn to cook simple meals safely. The social worker has explained why each step matters, such as turning off the stove. What should the social worker do **NEXT**?

A. Give the client a written checklist to follow at home
B. Demonstrate cooking a meal while the client watches
C. Have the client try cooking a meal with coaching

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). A written checklist is a helpful memory aid during home practice. Given now, it replaces seeing and rehearsing the skill, which is how clients with schizophrenia tend to learn most reliably.

B **Correct.** Skills training follows a set sequence: explain the rationale, model the skill, have the client rehearse it with feedback, then assign practice in real settings (Bellack et al., 2004). With the rationale covered, the social worker demonstrates the skill so the client sees each step done correctly. It beats C because coached rehearsal comes after modeling; asking the client to perform before seeing each step done correctly skips the demonstration that rehearsal builds on.

C **Incorrect** (trap: Right action, wrong time). Coached rehearsal with feedback is the step after modeling. Asking the client to cook before seeing the skill demonstrated skips that step, so mistakes with safety steps such as the stove are more likely and harder to correct.

Takeaway: Teach self-care skills in sequence: explain the rationale, model, rehearse with feedback, then assign independent practice.

Sources: Bellack, A. S., Mueser, K. T., Gingerich, S., & Agresta, J. (2004). Social Skills Training for Schizophrenia: A Step-by-Step Guide (2nd ed.). Guilford Press, steps of skills training
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.8 Teaching clients coping and self-care skills · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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39. ASM-1188 (rev 1) · Best answer: A

In an integrated primary care clinic, a social worker completes a routine intake screening with a new adult client. When the social worker reads a question about past physical or sexual violence, the client's hands begin to shake, and the client says quietly, 'I don't really talk about that.' What is the **BEST** response?

A. Reflect the distress and offer a choice to pause
B. Gently ask what happened so the screening is complete
C. Move on to the next question to finish the screening

Why each option is right or wrong

A **Correct.** Trauma-informed practice puts safety and the client's voice and choice first (SAMHSA, 2014), and screening should not push for details a client is not ready to share (TIP 57). Naming the visible distress and letting the client decide whether to pause or go on returns control at the moment it feels lost. It beats C because skipping ahead without comment avoids pressing for details but leaves the client alone with a visible reaction and gives no choice.

B **Incorrect** (trap: Ignores risk). Asking for details during a screening can retraumatize a client who has just signaled they are not ready. Screening aims to identify a need for later assessment, not to gather the trauma story.

C **Incorrect** (trap: Good but not best). Moving on avoids pressing for details, which is better than probing. But ignoring a visible stress reaction misses a chance to restore the client's sense of control and can feel dismissive. It is reasonable after the client has chosen to continue.

Takeaway: When trauma questions trigger visible distress, acknowledge it and give the client control over pacing; don't probe for details during screening.

Sources: SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), Six key principles: safety; empowerment, voice and choice; SAMHSA TIP 57 (2014), Ch. 4: Screening and Assessment
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.1 Trauma, stress, violence, crisis: signs and impact (historical, complex) · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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40. ASM-1154 (rev 1) · Best answer: B

A client with schizoaffective disorder served by an assertive community treatment team has gained weight over the past year. At the client's apartment, the client mentions being thirsty all the time, getting up several times a night to urinate, and having blurry vision. The client distrusts doctors and plans to bring it up at a psychiatric appointment in 5 weeks. What is the **MOST** appropriate action?

A. Agree to the client's plan and check on the symptoms next month
B. Offer to go with the client to a primary care visit this week
C. Encourage the client to switch to water and sugar-free drinks

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). The plan respects the client's wishes, but it leaves possible high blood sugar unchecked for more than a month. Untreated diabetes can worsen quickly, so a sooner medical visit is needed.

B **Correct.** People with serious mental illness have higher rates of diabetes than the general population, and their physical illnesses are often missed or treated late (De Hert et al., 2011). Constant thirst, frequent urination and blurry vision are classic signs of high blood sugar (CDC). Offering to go along respects the client's distrust while getting a timely evaluation. It beats A because honoring the client's timeline suits minor concerns, but 5 weeks is a long delay for possible untreated diabetes, and a psychiatric visit is not set up to test for it.

C **Incorrect** (trap: Good but not best). Cutting sugary drinks is healthy advice, but it treats a possible medical condition without a diagnosis. The symptoms need testing by a medical provider before lifestyle steps are planned.

Takeaway: Clients with serious mental illness face high, often missed, rates of diabetes; link new classic symptoms to timely medical care.

Sources: De Hert, M., Correll, C. U., Bobes, J., et al. (2011). Physical illness in patients with severe mental disorders. I. Prevalence, impact of medications and disparities in health care. World Psychiatry, 10(1), 52-77, whole article; CDC (Centers for Disease Control and Prevention). Symptoms of Diabetes, Symptoms shared by type 1 and type 2 diabetes

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.6 Comorbid (co-occurring) disorders and conditions · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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41. ASM-1653 (rev 2) · Best answer: C

A teacher refers a 9-year-old to the school social worker for an ADHD evaluation, noting that the child cannot focus after mid-morning and puts their head down by lunch. In a phone call, the parent explains that their work hours were cut and food often runs out before payday. The child usually skips breakfast. What should the social worker do **FIRST**?

A. Collect parent and teacher ratings of the child's attention

B. Suggest the parent ask the pediatrician about ADHD medication

C. Connect the family to free meal programs and food aid

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Multi-informant rating scales are a sound part of an ADHD evaluation, but completing them while the child is hungry would blur the picture. They belong after the food problem is addressed, if attention difficulties continue.

B **Incorrect** (trap: Skips assessment). Raising medication jumps past both assessment and the plain explanation in the stem. A social worker also does not steer families toward a specific medication; a pediatric evaluation fits if problems persist after basic needs are met.

C **Correct.** Basic needs come before deeper assessment, and hunger alone can impair a child's concentration. The American Academy of Pediatrics links food insecurity to trouble concentrating in school and to behavior problems (AAP, 2015). Here focus fades as the morning goes on, and the child arrives without breakfast from a household that runs out of food. Linking the family to free school meals and other food aid addresses the likely cause quickly. It beats A because rating scales collected now would be clouded by hunger; if focus problems persist once the child eats regularly, an ADHD evaluation is the right next step.

Takeaway: Address unmet basic needs such as hunger before attributing a child's inattention to ADHD; reassess attention once needs are met.

Sources: American Academy of Pediatrics (AAP), Council on Community Pediatrics & Committee on Nutrition (2015). Promoting food security for all children. Pediatrics, 136(5), e1431-e1438. doi:10.1542/peds.2015-3301, effects of food insecurity on children

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.9 Poverty's impact; social determinants of health · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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42. INT-1677 (rev 1) · Best answer: A

An agency compares the cost per youth served of two mentoring models. One model uses paid mentors. The other relies on volunteer mentors and meets in space a church donates. The draft analysis counts budgeted expenses alone, so the volunteer model looks far cheaper. How should the social worker revise the cost analysis?

A. Add a market value for unpaid mentor hours and the room

B. Keep budgeted expenses, since unpaid mentors cost the agency nothing

C. Convert both models' outcomes into dollar benefits

Why each option is right or wrong

A **Correct.** A sound cost analysis counts every resource a program uses, including volunteer labor and donated goods or space, valued at what they would cost to obtain (their opportunity cost). Leaving them out makes the volunteer model look cheaper than it is and misleads anyone who might replicate it where donations are unavailable. It beats B because the question is the real resource cost of each model, not the agency's cash outlay (Rossi, Lipsey & Henry, 2019; Levin et al., 2018).

B **Incorrect** (trap: Misuses the technique). Cash expenses matter for the agency's own budget, but a comparison of program models must include every resource consumed. Excluding volunteer hours and donated space understates the volunteer model's true cost and misleads replication decisions.

C **Incorrect** (trap: Wrong model or concept). Putting outcomes in dollars turns the study into a cost-benefit analysis, which answers a different question. The flaw here is on the cost side: missing resources. A cost-benefit analysis with the same omission would be just as biased.

Takeaway: Cost analyses value every resource used, including volunteer time and donated space, not just budgeted cash expenses.

Sources: Rossi, P. H., Lipsey, M. W., & Henry, G. T. (2019). Evaluation: A Systematic Approach (8th ed.). SAGE, Ch. 10: Assessing the Economic Efficiency of Programs; Levin, H. M., McEwan, P. J., Belfield, C., Bowden, A. B., & Shand, R. (2018). Economic Evaluation in Education: Cost-Effectiveness and Benefit-Cost Analysis (3rd ed.). SAGE, ingredients method: volunteer time and donated resources

Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.4 Agency program evaluation: needs assessment, formative/summative, cost analyses · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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43. ETH-1640 (rev 1) · Best answer: C

At a pediatric clinic, a social worker helps a parent whose 7-year-old's asthma keeps worsening. The apartment has visible mold and roaches. With the social worker's help, the parent sent the landlord a written repair request and a doctor's letter 6 weeks ago. The landlord has not responded and has hinted that the lease may not be renewed. What should the social worker do **NEXT**?

A. Ask the doctor to strengthen the child's asthma treatment plan
B. Help the parent search for a different apartment
C. Connect the parent with a tenant legal aid program
D. Organize other tenants to confront the landlord together

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Medical treatment matters, and the doctor has already documented the link to housing. Adjusting medicines while the child stays exposed to mold and pests treats the symptoms and leaves the environmental cause in place.

B **Incorrect** (trap: Good but not best). Moving can help if the family wants it, but it is costly, may not be possible, and shifts the burden onto the family. The landlord is responsible for repairs, so learning the family's rights should come before giving up the home.

C **Correct.** Social work's commitment to environmental justice (CSWE, 2022) means treating unhealthy housing as a structural cause of illness, not only a medical problem. The written request with a doctor's letter has failed, and the landlord's hint about the lease raises legal questions beyond the social worker's expertise. A tenant legal aid program can explain repair rights and any protection against retaliation (NASW Code 1.16(a)). It beats D because collective action may come later, but organizing before the parent knows their rights could expose the family to the retaliation they fear.

D **Incorrect** (trap: Right action, wrong time). Tenant organizing can win building-wide repairs and is a strong environmental justice tool. It is premature now: the parent fears losing the lease and has not chosen collective action, so legal advice comes first.

Takeaway: When unhealthy housing harms a client and direct requests fail, link the family to legal help on tenant rights before riskier collective steps.

Sources: Council on Social Work Education (2022). Educational Policy and Accreditation Standards, Competency 2: Advance Human Rights and Social, Racial, Economic, and Environmental Justice; NASW Code of Ethics (2021), 1.16(a)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.2 Principles of social, racial, environmental, economic justice · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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44. INT-1634 (rev 2) · Best answer: C

A voluntary client who has never had counseling comes to an outpatient clinic about job stress. A few minutes into the first session, the client asks, 'So do you just tell me what to fix? I've never done this before.' What should the social worker do **FIRST**?

A. Offer two or three practical tips for handling job stress
B. Ask the client to describe the job stress in detail
C. Explain how sessions work and ask what the client expects

Why each option is right or wrong

A **Incorrect** (trap: Too directive). Giving tips confirms the idea that the social worker fixes problems for the client, and it comes before any assessment of the job stress. Advice may have a place later, once the situation is understood.

B **Incorrect** (trap: Right action, wrong time). Exploring the presenting problem is central to early sessions and will come soon. Starting it now passes over the client's direct question about how counseling works; clients engage more readily in assessment once they understand what to expect.

C **Correct.** Orienting a new client to the helping process is a core engagement task: the social worker explains how sessions work and what each person does, and draws out the client's expectations (Hepworth et al.). Expectations that do not match the service are a common reason clients drop out early. This client has just said they do not know what counseling involves. It beats B because exploring the stress before answering the question leaves the client unsure what counseling involves or why the questions matter; the exploration follows once the client knows what to expect.

Takeaway: Early in engagement, orient new clients to how the work proceeds and draw out their expectations; mismatched expectations can lead to early dropout.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Orienting clients to the helping process **Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.11 Engaging and motivating voluntary and mandated clients · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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45. INT-1150 (rev 3) · Best answer: B

Parents bring their 9-year-old to family therapy for tantrums. The family used a behavior chart consistently for 4 weeks with no change. In sessions, the child starts acting out whenever the parents begin to disagree about money, and the parents then stop arguing and work together to calm the child. Both parents say they 'haven't been a team lately.' What is the **BEST** intervention at this point?

A. Coach the parents to stay calm and united during tantrums

B. Meet with the parents as a couple about their conflict

C. Tell the family that the tantrums protect the parents from fighting

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Calm, united responses to tantrums are sound parenting advice. But these parents already unite whenever the child acts out; that is the pattern keeping the symptom going. More teamwork around the tantrums leaves the money conflict untouched and keeps the child in the go-between role.

B **Correct.** Structural family therapy holds that a symptom can serve the family's structure. Here the parents detour their conflict through the child: tantrums begin when money disagreements surface and end the argument by uniting the parents. A consistent 4-week chart trial failed, so this is not mainly a skills problem. Meeting with the parents as a couple strengthens the spousal subsystem and draws a boundary that frees the child from the conflict (Minuchin, 1974). It beats C because announcing the pattern to the whole family, child included, invites defensiveness; structural work changes the pattern by restructuring rather than interpreting it.

C **Incorrect** (trap: Too directive). The hypothesis may well be accurate, but announcing it to the whole family this early invites defensiveness and can leave the child feeling responsible for the parents' relationship. Structural therapists change the pattern by restructuring, not by interpreting it to the family.

Takeaway: When a child's symptom detours parental conflict, strengthen the parents' couple subsystem instead of intensifying child-focused behavior plans.

Sources: Minuchin, S. (1974). Families and Family Therapy. Harvard University Press, detouring; spousal subsystem and boundaries

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.21 Family therapy models and interventions · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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46. ASM-1175 (rev 2) · Best answer: B

Two days after a 10th grader died by suicide, a high school social worker reviews the students staff are worried about. One was the student's closest friend, was hospitalized last year after an overdose, and has not come to school yet today. Another sat beside the student in chemistry and keeps crying in class. A third posted a tribute video that friends shared widely. Whom should the social worker contact **FIRST**?

A. The classmate crying in chemistry class

B. The friend who is absent from class

C. The student whose tribute video friends shared

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Open grief in class is common after a peer's death and deserves a check-in today. Visible distress alone, without a close tie or a history of suicidal behavior, places this student behind the absent close friend in the order of outreach.

B **Correct.** After a suicide, outreach starts with the students at highest risk of contagion: close friends and students with prior suicidal behavior or known depression (AFSP & SPRC, 2018). This student has both, plus an unexplained absence, so the social worker should reach the student, or the family, today and assess for suicide risk. It beats A because crying in class is visible but is an expected grief reaction in a student with a looser tie and no known history; the quiet, absent student carries more risk factors.

C **Incorrect** (trap: Right action, wrong time). Widely shared memorial posts raise contagion concerns, and the school should guide safe messaging online. That is a schoolwide task. The student who posted shows no listed individual risk factors, so this contact comes after reaching the highest-risk student.

Takeaway: After a suicide, contact first the students with the most risk factors: close ties to the deceased and prior suicidal behavior, not the most visibly upset.

Sources: AFSP & SPRC (2018). After a Suicide: A Toolkit for Schools (2nd ed.), Crisis Response; Suicide Contagion: identifying other students at possible risk

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.1 Assessing risk of harm to self or others · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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47. ASM-0112 (rev 1) · Best answer: A

Which assessment tool **BEST** maps a family's current connections to schools, agencies, work and other outside systems?

A. Ecomap
B. Genogram
C. Culturagram

Why each option is right or wrong

A **Correct.** An ecomap places the family in a central circle and draws lines to outside systems such as school, work, health care and agencies, showing which connections are strong, weak or stressful (Hartman, 1978). It captures the family's present-day person-in-environment picture. It beats B because a genogram maps family structure and patterns across at least three generations, not current ties to outside systems (McGoldrick et al., 2020).

B **Incorrect** (trap: Wrong model or concept). A genogram diagrams family members, relationships and patterns across three or more generations, such as illness, substance use or cutoffs. It is the right tool for intergenerational patterns, not for mapping current outside resources.

C **Incorrect** (trap: Wrong model or concept). A culturagram assesses an immigrant family's cultural context, such as reasons for migration, legal status, language and health beliefs (Congress, 1994). It is useful for cultural assessment but does not chart the family's links to outside systems.

Takeaway: Ecomap shows current ties to outside systems; genogram shows intergenerational family patterns; culturagram shows an immigrant family's cultural context.

Sources: Hartman, A. (1978). Diagrammatic assessment of family relationships. Social Casework, 59(8), 465-476, whole article; McGoldrick, M., Gerson, R., & Petry, S. (2020). Genograms: Assessment and Treatment (4th ed.). W. W. Norton, genogram construction; Congress, E. P. (1994). The use of culturagrams to assess and empower culturally diverse families. Families in Society, 75(9), 531-540, whole article
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.6 Tools for assessing strengths, resources, needs, challenges · **Qualifier** BEST · **Cognitive** recall · **Difficulty** 1 of 3

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48. ETH-0004 (rev 2) · Best answer: C

An adult client with schizophrenia has been stable for a year on an antipsychotic medication. The client tells the social worker that weight gain from the medication is unbearable and that they plan to stop taking it. The client is coherent and understands the risk of relapse. How should the social worker respond?

A. Tell the client that stopping the medication would be a mistake
B. Ask the client's family to encourage continued medication use
C. Explore the concerns and link the client with the prescriber
D. Suggest that the client lower the dose gradually before stopping

Why each option is right or wrong

- **A** **Incorrect** (trap: Too directive). Telling the client the decision is a mistake is advice-giving before the reasons are understood. It tends to provoke defensiveness and can make the client less likely to share future medication changes with the treatment team.
- **B** **Incorrect** (trap: Overrides the client's choice). Recruiting family to push adherence goes around a competent adult's decision and discloses clinical information without consent. Family involvement is appropriate when the client agrees to it.
- **C** **Correct.** A competent adult has the right to make treatment decisions, including stopping a medication (NASW Code 1.02). The social worker's role is to explore what drives the decision, here a real and common side effect, and to help the client bring it to the prescriber, who may have alternatives. This keeps the client informed and engaged without overriding the choice. It beats A because pressure tends to end the conversation, while exploration keeps the client talking with the team. Recovery-oriented care treats the person as the driver of treatment choices (SAMHSA, 2012).
- **D** **Incorrect** (trap: Outside the social work role). Advising how to taper is medication management, which is outside the social work role and the worker's competence (NASW Code 1.04(a)). The prescriber guides any dose change, so the social worker connects the client with the prescriber instead.

Takeaway: A competent adult may refuse medication; the social worker explores the reasons and links the client to the prescriber.

Sources: NASW Code of Ethics (2021), 1.02; SAMHSA (2012) Working Definition of Recovery (PEP12-RECDEF), guiding principles: recovery is person-driven; NASW Code of Ethics (2021), 1.04(a)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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49. ASM-1632 (rev 2) · Best answer: A

A 10-year-old in a residential treatment program was removed from a home where a caregiver often screamed threats and insults. Staff report that the child now corners younger residents and shouts at them using the caregiver's exact words, tone and gestures. When staff step in, the child laughs and says, 'Nobody can push me around now.' Which defense mechanism is the child **MOST** likely using?

A. Identification with the aggressor
B. Displacement
C. Acting out
D. Projection

Why each option is right or wrong

A **Correct.** Identification with the aggressor, described by Anna Freud, is coping with fear of a threatening person by taking on that person's traits and behavior, turning from passive victim into active aggressor. The child copies the caregiver's words, tone and gestures and says no one can push them around now, which shows the role reversal. It beats B because displacement only redirects feelings to a safer target; it does not explain why the child reproduces the caregiver's own behavior so exactly (Freud, A., 1936).

B **Incorrect** (trap: Wrong model or concept). Displacement shifts feelings from a dangerous target to a safer one, and younger residents are safer targets. But displacement does not explain the child copying the caregiver's exact words and manner, which shows taking on the aggressor's role.

C **Incorrect** (trap: Wrong model or concept). Acting out expresses an impulse through behavior instead of words, and the child's aggression is behavioral. The term names the outlet, not the defining feature here: the child has adopted the caregiver's own role and style.

D **Incorrect** (trap: Wrong model or concept). Projection attributes one's own unacceptable feelings to others, such as accusing younger residents of being hostile. The child is not assigning feelings to them but openly enacting the caregiver's aggression.

Takeaway: In identification with the aggressor, a frightened person adopts the threatening person's behavior, turning helplessness into power over others.

Sources: Freud, A. (1936/1966). The Ego and the Mechanisms of Defense (rev. ed.). International Universities Press, Ch. 9, Identification with the aggressor

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.11 Defense mechanisms and how they shape behavior and relationships · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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50. ETH-0106 (rev 1) · Best answer: B

In a remote town with one mental health provider, a social worker treats a client who owns the only hair salon. At intake, the two agreed on how to handle contact at the salon. During the social worker's next haircut, the client refuses payment, saying, 'You've helped me so much.' What is the social worker's **BEST** response?

A. Accept the free haircut as a sign of the client's gratitude

B. Pay the usual price and explore the gesture at the next session

C. Use a salon in another town for the rest of treatment

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). Accepting a free service adds a financial benefit to an existing overlap and blurs the boundary the two agreed on. Small tokens of gratitude can sometimes be accepted, but a service with real value given at the client's expense is different.

B **Correct.** In small communities, overlapping roles can be unavoidable, and the social worker is responsible for keeping them from becoming exploitative (NASW Code 1.06(c)). Paying as any customer would keeps the salon contact ordinary and avoids taking a financial benefit from a client. Exploring the gesture in session respects the

client's gratitude and uses it clinically. It beats C because the overlap was already managed at intake; the problem is the free service, not the salon visits, and avoiding the only salon disrupts community life without addressing what the gesture means (Reamer, 2020).

C **Incorrect** (trap: Good but not best). Avoiding the salon could be reasonable if the contact became harmful, but here the overlap was planned and working. Changing salons does not address the gift and treats an unavoidable small-town overlap as something that has to be eliminated.

Takeaway: When overlapping roles are unavoidable, keep outside contact ordinary, decline financial benefits from clients, and explore gestures clinically.

Sources: NASW Code of Ethics (2021), 1.06(c); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, unavoidable dual relationships in small communities
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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51. ETH-1194 (rev 1) · Best answer: B

A social worker newly assigned to a sexual assault program finds that clients' accounts stir memories of an assault the social worker experienced years ago. The social worker has discussed this in supervision, and the supervisor suggested personal counseling. The social worker worries that seeking counseling would look like incompetence. Which step is **MOST** consistent with ethical self-care?

A. Ask the supervisor to provide personal counseling in supervision
B. Begin personal counseling while continuing in supervision
C. Request a transfer away from the sexual assault program

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). Supervision addresses the social worker's practice, not personal treatment. Turning supervision into therapy blurs the supervisor's evaluative role and creates a dual relationship (Kadushin & Harkness, 2014).

B **Correct.** The NASW Code's Integrity principle states that social workers should take measures to care for themselves professionally and personally. The Code names seeking professional help as a remedy when personal difficulties interfere with practice (4.05(b)); using it earlier, as here, is sound prevention. Personal counseling addresses the worker's own history, while supervision keeps addressing the work with clients. It beats C because a transfer is a later option if reactions cannot be managed, and the social worker has not yet tried the support the supervisor recommended.

C **Incorrect** (trap: Hands off too soon). A transfer may become appropriate if reactions cannot be managed even with support. Requesting it first gives up work the social worker chose before trying the counseling the supervisor recommended.

Takeaway: Counseling for one's own trauma reactions is ethical self-care that runs alongside supervision, not inside it.

Sources: NASW Code of Ethics (2021), Ethical Principles: Integrity; NASW Code of Ethics (2021), 4.05(b); Kadushin & Harkness (2014). Supervision in Social Work (5th ed.). Columbia University Press, Supportive supervision: supervision versus therapy
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.7 Principles of self-care for the social worker · **Qualifier** MOST · **Cognitive** application · **Difficulty** 1 of 3

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52. ETH-1109 (rev 1) · Best answer: B

A family shelter starts requiring residents to wear colored wristbands in the dining hall that show whether they receive public benefits, so staff can track meal funding. Residents tell a social worker they feel marked in front of other families. Which core value of the profession does this practice **MOST** directly conflict with?

A. Social justice
B. Dignity and worth of the person
C. Importance of human relationships

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Social justice addresses unequal access and discrimination, and the policy does single out low-income families. The stem describes no denied service, though; the central harm is public stigma to each resident's dignity.

B **Correct.** The Dignity and Worth of the Person principle asks social workers to treat each person in a caring and respectful fashion, mindful of individual differences (NASW Code, Ethical Principles). Marking families by benefit status in a shared dining hall exposes and stigmatizes them for an administrative purpose that could be met privately, and residents say they feel marked. B beats A because the harm here is to each resident's standing and respect in daily life; social justice concerns access, opportunity and discrimination, which overlaps but is less direct.

C **Incorrect** (trap: Wrong model or concept). This value treats relationships as a primary vehicle for change and engages people as partners in helping. The wristbands affect how residents are seen by others, but the core harm is to each person's dignity, not to a helping relationship.

Takeaway: Practices that publicly mark or stigmatize clients conflict most directly with the value of dignity and worth of the person.

Sources: NASW Code of Ethics (2021), Ethical Principles: Dignity and Worth of the Person; NASW Code of Ethics (2021), Ethical Principles: Social Justice; NASW Code of Ethics (2021), Ethical Principles: Importance of Human Relationships
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.5 Core values: competence, social justice, integrity, dignity and worth · **Qualifier** MOST · **Cognitive** recall · **Difficulty** 3 of 3

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53. ETH-1033 (rev 2) · Best answer: A

On an inpatient psychiatric unit, a patient scheduled for discharge this afternoon tells the social worker they plan to go to their former partner's home tonight and shoot the partner with the handgun kept in their closet. The patient says the decision is made. What should the social worker do **FIRST**?

A. Have the team hold the discharge for reevaluation
B. Warn the former partner and proceed with the discharge
C. Ask more about how firmly the patient intends to act
D. Notify the outpatient provider to follow up this week

Why each option is right or wrong

A **Correct.** A serious threat against an identifiable person, with a plan, a weapon and a time, calls for protective action under the duty recognized in Tarasoff (1976). The patient is still hospitalized, so the most protective step is to have the team hold the discharge and reevaluate risk. A beats B because warning the former partner is a recognized protective step, but releasing a patient who has stated a plan and has a gun at home leaves the danger in place. A warning can follow as the law allows (45 CFR 164.512(j)(1)(i)).

B **Incorrect** (trap: Ignores risk). Warning an identifiable victim is one way to meet the duty to protect and may still be needed. Here the patient is about to leave with a stated plan and access to a gun, so holding the discharge addresses the danger more directly.

C **Incorrect** (trap: Right action, wrong time). Clarifying intent is the right first move when a statement is vague. This patient has named the target, the weapon, the time and a firm decision, so more questioning delays protective action as discharge nears.

D **Incorrect** (trap: Ignores risk). Outpatient follow-up suits ongoing care after a safe discharge. A stated plan to shoot someone tonight is an imminent risk that cannot wait for a visit later in the week.

Takeaway: A serious threat with an identifiable victim, plan and means requires protective action now; while the client is hospitalized, use that protection first.

Sources: Tarasoff v. Regents of the University of California, 17 Cal. 3d 425 (1976); HIPAA Privacy Rule, 45 CFR 164.512(j)(1)(i)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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54. ETH-1115 (rev 2) · Best answer: C

A licensed clinical social worker kept the license current through continuing education during a 5-year break from practice. Before the break, the worker provided school-based counseling to children. A community clinic now offers the worker a full caseload of adults with serious mental illness, starting in 2 weeks. What is the **MOST** appropriate way for the worker to proceed?

A. Accept the full caseload after a weekend training on serious mental illness
B. Decline the position because the population is new
C. Accept with gradual intake, training and supervision

Why each option is right or wrong

A **Incorrect** (trap: Outside the social work role). Training is part of the answer, which makes this tempting. One weekend course before a full caseload starts skips the consultation, supervision and gradual intake that 1.04(b) expects for a new area, leaving adults with serious mental illness served by a worker still learning the basics.

B **Incorrect** (trap: Good but not best). Declining is unnecessary. The Code allows accepting work with the intention to acquire competence, and a structured plan can close the gap while meeting the clinic's need.

C **Correct.** Social workers accept employment only on the basis of existing competence or the intention to acquire it (NASW Code 4.01(a)), and they build competence in new areas through study, training, consultation and supervision (1.04(a), 1.04(b)). The license is current, but the worker's recent experience is with children, not adults with serious mental illness. A gradual caseload with targeted training and clinical supervision builds competence while serving clients safely. C beats B because the Code does not require existing expertise before accepting work, only a real plan to acquire it.

Takeaway: Accept work in a new area only with a concrete plan to gain competence through training, consultation and supervision.

Sources: NASW Code of Ethics (2021), 4.01(a); NASW Code of Ethics (2021), 1.04(a); NASW Code of Ethics (2021), 1.04(b)

Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.8 Keeping knowledge current: training, licensing CE, supervision, consultation · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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55. INT-1192 (rev 3) · Best answer: C

A client with bipolar I disorder, stable for 6 months after a manic episode, asks for help avoiding another hospitalization. The client's partner joins the session and asks for a complete list of symptoms to watch for each day. The client says the partner's constant questions about mood feel like being watched. Which psychoeducation approach is **MOST** helpful?

A. Give the partner a daily checklist of bipolar symptoms to watch for

B. Meet with the client alone to protect the client's privacy

C. Help the client name their own early signs and the partner's role

D. Teach the partner to watch for reduced sleep as the key warning sign

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). A daily symptom checklist in the partner's hands makes the partner a monitor and increases the conflict the client described. Generic lists also miss the client's personal early signs, which are more useful for preventing relapse.

B **Incorrect** (trap: Good but not best). Privacy matters, but the client did not ask to exclude the partner, and involving family in relapse planning can help prevent episodes. Leaving the partner out loses an ally and leaves the conflict over monitoring unresolved.

C **Correct.** Bipolar psychoeducation centers on each person's own pattern of early warning signs that come before an episode, plus an agreed plan for responding (Colom

& Vieta, 2006). Family-focused treatment brings relatives into that plan as allies with defined roles, which eases conflict over monitoring (Miklowitz, 2010). The client wants to prevent hospitalization but feels watched, so building the client's own list and deciding together what the partner will notice and do respects autonomy while using the partner's support. It beats A because a generic checklist hands control to the partner and intensifies the monitoring the client resents.

D **Incorrect** (trap: Good but not best). Reduced sleep is a common early sign of mania, so this sounds targeted. It still makes the partner the monitor and swaps one general sign for the client's own pattern; relapse plans work best when the client names personal warning signs and agrees on the partner's role.

Takeaway: In bipolar psychoeducation, build the client's personal early warning signs and a shared response plan that gives family an agreed role.

Sources: Colom, F., & Vieta, E. (2006). Psychoeducation Manual for Bipolar Disorder. Cambridge University Press, Early detection of new episodes; Miklowitz, D. J. (2010). Bipolar Disorder: A Family-Focused Treatment Approach (2nd ed.). Guilford Press, Relapse prevention plan with family

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.14 Delivering psychoeducation · **Qualifier** MOST · **Cognitive** reasoning · **Difficulty** 2 of 3

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56. ETH-1064 (rev 2) · Best answer: B

A family caregiver support program moves to remote-only services. An 80-year-old caregiver in a rural area has a basic cell phone and no home internet, and wants to keep meeting with the social worker. Which response is **MOST** helpful?

A. Lend a tablet and teach the caregiver to use video

B. Hold the sessions as regular voice calls

C. Refer the caregiver to an in-person group 60 miles away

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Loaning devices can close access gaps, but this caregiver has no home internet, so a tablet would also need a data plan or hotspot plus training before any session could happen. Services would stall while those barriers are solved.

B **Correct.** Social workers recognize differences in clients' access to technology and work to keep those differences from becoming barriers to service (NASW Code 1.05(e)); they also assess each client's ability to use a given technology (1.03(g)). The caregiver already has a cell phone and wants to continue, so phone sessions keep services going right away. B beats A because a tablet would need a separate data connection and new skills before sessions could resume, which adds strain for an already burdened caregiver, while the phone works now.

C **Incorrect** (trap: Hands off too soon). A distant in-person group hands off a client who can be served now. The travel burden for a rural caregiver makes this likely to end services rather than continue them.

Takeaway: Match remote services to the client's real access; use the technology the client already has rather than requiring new tools.

Sources: NASW Code of Ethics (2021), 1.05(e); NASW Code of Ethics (2021), 1.03(g)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** MOST · **Cognitive** reasoning · **Difficulty** 2 of 3

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57. ETH-1659 (rev 1) · Best answer: A

PRETEST — unscored on this form; do not count it

A client in outpatient treatment arrives upset. The client found the social worker's personal social media account and saw a public post supporting a cause the client strongly opposes. 'I don't know if I can trust you now,' the client says. What should the social worker do **FIRST**?

A. Invite the client to describe how the post affected the relationship
B. Make the account private and remove the post
C. Explain that personal views stay separate from the client's care

Why each option is right or wrong

A **Correct.** A client's report of shaken trust signals an alliance rupture, and repair starts by inviting the client to say more and taking the concern seriously without defending (Safran & Muran, 2000). The Code warns that personal information posted online can cause boundary confusion or harm to clients (NASW Code 1.06(f)). Hearing the client's experience comes before deciding what to change. It beats B because tightening privacy settings is a sensible later step for the worker, but doing it first leaves the client's question about the relationship unanswered in the room.

B **Incorrect** (trap: Right action, wrong time). Reviewing privacy settings is worth doing, and the worker should do it soon. As a first response, it attends to the worker's exposure while the client sits with an open question about the relationship.

C **Incorrect** (trap: Good but not best). Reassurance that personal views won't affect care may be true, but offered first it defends the worker and can feel dismissive. It is more useful after the client has been heard, as part of repairing the relationship.

Takeaway: When a client finds the worker's personal online content and trust is shaken, explore the rupture before defending or changing settings.

Sources: Safran, J. D., & Muran, J. C. (2000). Negotiating the Therapeutic Alliance: A Relational Treatment Guide. Guilford Press, rupture resolution; NASW Code of Ethics (2021), 1.06(f)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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58. INT-1654 (rev 1) · Best answer: C

An 84-year-old client who lives alone in a rural area needs help with meals, rides to the store, and a daily check-in while recovering from a hip fracture. The regional aging agency's in-home program has an 8-month waitlist. The client has belonged to a local congregation for 50 years, and two neighbors already stop by now and then. The client wants to stay home. What is the social worker's **BEST** plan for the coming months?

A. Recommend a move to an assisted living facility
B. Add the client to the in-home program waitlist and check in monthly
C. Turn the client's informal contacts into a weekly helper schedule

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Assisted living could meet these needs, but the client wants to stay home and has supports that can be organized. Recommending a move overrides a capable client's preference before less restrictive options are tried.

B **Incorrect** (trap: Good but not best). Joining the waitlist is sensible and can proceed alongside other steps. On its own, it leaves an 84-year-old recovering from a hip fracture without meals, rides or daily check-ins for months.

C **Correct.** Building a client's service network includes organizing natural supports, not only formal programs; the strengths model treats the community, including congregations and neighbors, as a source of resources (Rapp & Goscha, 2012). Here the client has deep congregation ties, neighbors who already visit, and an 8-month gap before formal help, so turning informal contact into a planned schedule for meals, rides and daily check-ins meets the needs now and fits the wish to stay home. It beats B because joining the waitlist is worth doing, but by itself it leaves the client without daily help for months (NASW, 2013, Standard 6).

Takeaway: When formal services are delayed, organize the client's natural supports into a planned network while waiting for formal help.

Sources: Rapp, C. A., & Goscha, R. J. (2012). The Strengths Model: A Recovery-Oriented Approach to Mental Health Services (3rd ed.). Oxford University Press, principles of the strengths model; NASW Standards for Social Work Case Management (2013), Standard 6: Service Planning, Implementation, and Monitoring
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.24 Building service networks and community resources · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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59. INT-1532 (rev 1) · Best answer: C

In an early intervention home-visiting program, one social worker is the only provider who visits a toddler's family. A speech pathologist and an occupational therapy practitioner coach the social worker, who then teaches their strategies to the parents during visits. The three plan goals together with the family. Which team model is this program **MOST** likely using?

A. Interdisciplinary
B. Multidisciplinary
C. Transdisciplinary
D. Intradisciplinary

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Interdisciplinary teams meet and build an integrated plan, which fits the joint goal setting. But each professional still delivers their own services. Here only the social worker visits and uses the other professions' strategies.

B **Incorrect** (trap: Wrong model or concept). Multidisciplinary teams work in parallel: each profession assesses and treats separately, with little coordination. The joint planning and the single provider carrying the others' methods do not fit.

C **Correct.** In a transdisciplinary team, members share assessment and planning with the family and practice role release: one primary provider delivers services while teammates teach and back up that provider (King et al., 2009). Here a single social worker carries the speech and occupational therapy strategies into the home after coaching. It beats A, interdisciplinary, where members plan together and share findings but each still delivers their own profession's services. Joint planning fits both; the single provider using others' strategies is what makes this transdisciplinary.

D **Incorrect** (trap: Wrong model or concept). Intradisciplinary collaboration happens among members of one profession, such as two social workers co-leading a group. This team includes a speech pathologist and an occupational therapist.

Takeaway: Transdisciplinary teams use role release: one primary provider delivers care, using strategies other professions teach, within a shared plan.

Sources: King, G., Strachan, D., Tucker, M., et al. (2009). The application of a transdisciplinary model for early intervention services. Infants & Young Children, 22(3), 211-223, whole article
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.3 Inter- and intradisciplinary teamwork (co-therapy, care conferences, IEPs) · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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60. ASM-1010 (rev 1) · Best answer: A

A client is referred to a community agency after an emergency department visit labeled 'panic attack.' The social worker has taken a history: during a heated family argument, the client screamed, shook, felt heat rising to the head and fell to the floor. The client calls it an ataque de nervios and says such episodes run in the family after major conflicts. What should the social worker do **NEXT**?

A. Use the Cultural Formulation Interview to learn the client's view
B. Record panic disorder and refer for exposure-based treatment
C. Transfer the client to a practitioner from the client's culture

Why each option is right or wrong

A **Correct.** DSM-5-TR asks clinicians to understand a client's cultural concept of distress before assigning a diagnosis. Ataque de nervios is a recognized cultural syndrome, often set off by family conflict, that can resemble panic but differs from panic disorder, whose attacks are unexpected and followed by worry about more attacks. The Cultural Formulation Interview gathers the client's own explanation, supports and help-seeking preferences (DSM-5-TR). It beats B because labeling the episode panic disorder skips its cultural meaning and ignores that this episode was triggered, not unexpected.

B **Incorrect** (trap: Near-miss diagnosis). Panic disorder requires recurrent unexpected attacks and at least a month of worry or avoidance. This episode followed a family

argument and has a cultural meaning the social worker has not explored, so the diagnosis is premature.

C **Incorrect** (trap: Hands off too soon). Transferring assumes only a same-culture practitioner can help and abandons the assessment. Any social worker can use the Cultural Formulation Interview; a transfer fits only if the client asks for one.

Takeaway: When a client describes distress in cultural terms, use the Cultural Formulation Interview to understand its meaning before applying a DSM diagnosis.

Sources: DSM-5-TR (APA, 2022), Culture and Psychiatric Diagnosis: Cultural Formulation Interview; DSM-5-TR (APA, 2022), Culture and Psychiatric Diagnosis: Cultural Concepts of Distress
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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61. INT-1118 (rev 1) · Best answer: A

After a young adult's first hospitalization for schizophrenia, the parents meet with the inpatient social worker. One parent asks, 'Did our strict parenting cause this?' What is the **BEST** response?

A. Explain that the illness comes from biological vulnerability plus stress
B. Explore the parent's guilt about past parenting decisions in detail
C. Recommend family therapy to change the household's parenting style

Why each option is right or wrong

A **Correct.** Psychoeducation answers families' questions with accurate, non-blaming information. The stress-vulnerability model holds that schizophrenia arises when a biological vulnerability meets stressors, not from parenting style (Zubin & Spring, 1977). Family psychoeducation programs take this non-blaming stance and then teach families how to support recovery (Dixon et al., 2000). The parent asked a direct question, so a clear answer relieves misplaced blame and opens the door to learning how the family can help. It beats B because exploring guilt is supportive, but leaving the question unanswered can deepen the belief that the parents caused the illness.

B **Incorrect** (trap: Good but not best). Guilt is common and worth acknowledging, but the parent asked a specific question that has an evidence-based answer. Exploring the guilt without correcting the mistaken belief can reinforce self-blame. Exploration fits after accurate information is given.

C **Incorrect** (trap: Wrong model or concept). Recommending therapy to change parenting implies the family caused the illness, echoing discredited theories that blamed parents. Families do benefit from support and skills, but through non-blaming family psychoeducation, not treatment framed around their faults.

Takeaway: When families ask whether they caused schizophrenia, explain the stress-vulnerability model in non-blaming terms, then teach ways to support recovery.

Sources: Zubin, J., & Spring, B. (1977). Vulnerability: A new view of schizophrenia. Journal of Abnormal Psychology, 86(2), 103-126, whole article; Dixon, L., Adams, C., & Lucksted, A. (2000). Update on family psychoeducation for schizophrenia. Schizophrenia Bulletin, 26(1), 5-20, whole article
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.14 Delivering psychoeducation · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

Section 2

62. ASM-1141 (rev 1) · Best answer: A

At a crisis residential program, a client with borderline personality disorder becomes convinced for an afternoon that staff are plotting to discharge them, after a close peer leaves the program. By evening, the client laughs about the idea. A psychiatry resident proposes adding delusional disorder. Which response by the social worker is **MOST** helpful to the team?

A. Note that brief, stress-linked suspicion fits the existing diagnosis
B. Agree to add the diagnosis and monitor the beliefs closely
C. Suggest psychological testing to clarify the diagnosis first

Why each option is right or wrong

A **Correct.** Borderline personality disorder includes transient, stress-related paranoid ideas or severe dissociation. Delusional disorder requires a fixed delusion lasting at least 1 month. The client's suspicion followed a loss, lasted hours and faded by evening, which fits the existing diagnosis rather than a new psychotic disorder. It beats C because testing adds delay when the course alone shows that delusional disorder criteria are not met (DSM-5-TR).

B **Incorrect** (trap: Near-miss diagnosis). Delusional disorder requires a fixed belief for at least 1 month. An afternoon of suspicion that the client later laughs about falls far short and fits the borderline pattern.

C **Incorrect** (trap: Good but not best). Psychological testing can help when a picture is unclear. Here the brief, stress-linked course already answers the question, and testing would delay an understanding the team can reach now.

Takeaway: Brief, stress-linked paranoid ideas in borderline personality disorder are part of that pattern, not a separate psychotic disorder.

Sources: DSM-5-TR (APA, 2022), Personality Disorders: Borderline Personality Disorder; DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Delusional Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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63. ASM-1587 (rev 2) · Best answer: D

On a smoke-free inpatient psychiatric unit, a patient with schizophrenia who smoked heavily before admission tells the social worker, 'I don't want to start again when I leave. I'm picking a quit date with my doctor.' Discharge is in 5 days. What should the social worker do **FIRST**?

A. Review the pros and cons of smoking with the patient
B. Arrange a quitline referral to begin after discharge
C. Praise the decision and check in after discharge
D. Ask what could make it hard to avoid smoking at home

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Weighing pros and cons helps clients who are still ambivalent. This patient has already decided and is setting a date, so returning to decisional balance mismatches the stage and can stall momentum.

B **Incorrect** (trap: Right action, wrong time). A quitline is a useful support and belongs in the plan, but arranging it before knowing the patient's barriers may miss what the patient most needs. Identify obstacles first, then choose supports that address them.

C **Incorrect** (trap: Good but not best). Affirming the decision is good, but stopping there leaves the patient without a plan for the risks that follow discharge. Encouragement works best alongside concrete preparation.

D **Correct.** A client in preparation intends to act soon, and a key part of building the change plan is to explore and lower barriers to action, letting the client name likely obstacles (SAMHSA TIP 35, Ch. 6). The patient has a decision and a quit date, but leaving a smoke-free unit for home brings new triggers. Asking what could get in the way shapes the rest of the plan. It beats B because a referral chosen before barriers are known may not match what the patient will face.

Takeaway: In the preparation stage, have the client name likely barriers so the change plan and supports address them.

Sources: SAMHSA TIP 35 (2019), Ch. 6: From Preparation to Action: Initiating Change
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.4 Assessing motivation, barriers, and readiness for change · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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64. ASM-1626 (rev 2) · Best answer: B

A hospital patient awaiting surgery describes an argument with an adult sibling. In anger, the patient told the sibling, 'I hope you get what's coming to you.' Since then, whenever the sibling is mentioned, the patient knocks three times on the bed rail, repeats a set phrase and asks staff to send the sibling a small gift. The patient says, 'I keep worrying my words could make something bad happen.' Which defense mechanism is the patient **MOST** likely using?

A. Reaction formation
B. Undoing
C. Isolation of affect
D. Displacement

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Reaction formation appears as a lasting attitude opposite to the hidden impulse, such as excessive sweetness toward someone the person resents. The gifts might suggest it, but the ritual knocking and phrase target one remark, which points to cancelling an act rather than reversing a feeling.

B **Correct.** Undoing is an attempt to cancel, or symbolically reverse, an unacceptable thought or act through a later act. The patient's ritual knocking, set phrase and gifts all follow the hostile remark and aim to neutralize it, as the patient's fear that the words could cause harm shows. It beats A because reaction formation is a lasting stance opposite to a hidden feeling, such as constant warmth toward a resented person; here the behavior is tied to cancelling one remark and repeats as a ritual (Freud, A., 1936).

C **Incorrect** (trap: Wrong model or concept). Isolation of affect separates a thought from its feeling, so a person can describe something painful without emotion. This patient is plainly troubled by the remark and acts to neutralize it, rather than discussing it in a detached way.

D **Incorrect** (trap: Wrong model or concept). Displacement shifts a feeling onto a safer target, such as snapping at a nurse after a fight with the sibling. The patient's attention stays on the sibling, and the behavior aims to cancel the hostility, not redirect it.

Takeaway: Undoing uses a later act or ritual to symbolically cancel a hostile thought or deed; reaction formation is a lasting opposite attitude.

Sources: Freud, A. (1936/1966). The Ego and the Mechanisms of Defense (rev. ed.). International Universities Press, Ch. 4, The mechanisms of defense (undoing, isolation, reaction formation)
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.11 Defense mechanisms and how they shape behavior and relationships · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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65. ETH-1575 (rev 1) · Best answer: C

A social worker links each client's presenting problem to structural power, shares decisions with clients, and examines how the worker's own social position shapes the work. Which framework does this approach reflect?

A. Strengths perspective
B. Ecological systems perspective
C. Anti-oppressive practice
D. Person-centered approach

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). The strengths perspective centers clients' capacities and resources. It shares respect for clients, but it does not tie problems to structural power or ask the worker to examine their own social position, which define this description.

B **Incorrect** (trap: Wrong model or concept). The ecological perspective looks at person-environment fit and the exchanges between systems. It links problems to context, but it does not analyze power and oppression or require the worker to examine their own privilege.

C **Correct.** Anti-oppressive practice locates clients' difficulties within structures of power, works to share power with clients, and requires critical reflection on the worker's own social position (Dominelli, 2002). All three features in the description match. It beats B because the ecological perspective links problems to environmental systems but does not center power and oppression or ask workers to examine their own privilege. The Code likewise calls for critical self-reflection and recognizing clients as experts on their own culture (NASW Code 1.05(c)).

D **Incorrect** (trap: Wrong model or concept). The person-centered approach stresses empathy, genuineness and unconditional positive regard within the relationship. It is collaborative, but it does not connect problems to structural power or the worker's social position.

Takeaway: Anti-oppressive practice ties problems to structural power, shares power with clients, and requires reflection on the worker's own social position.

Sources: Dominelli, L. (2002). Anti-Oppressive Social Work Theory and Practice. Palgrave Macmillan, anti-oppressive practice: definition and principles; NASW Code of Ethics (2021), 1.05(c)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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66. ETH-1000 (rev 1) · Best answer: B

A social worker at a program known in the community for treating substance use disorders gets a phone call from a client's employer. The caller asks whether the client has been attending sessions this month. The client has signed no release for the employer. How should the social worker respond?

A. Confirm attendance but decline to give clinical details
B. Neither confirm nor deny that the person is a client
C. Ask the employer to send the request in writing

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Withholding clinical details sounds careful, but confirming attendance already tells the employer that the person is in substance use treatment. That fact is itself protected. With a signed release naming the employer, sharing attendance would be appropriate.

B **Correct.** Under 42 CFR Part 2, even acknowledging that someone is a patient of a program known for substance use treatment is a protected disclosure. It is allowed only with the patient's written consent or a qualifying court order (42 CFR 2.13(c)). The Code also requires valid consent before any disclosure (NASW Code 1.07(b)). With no release on file, the social worker neither confirms nor denies. B beats A because confirming attendance reveals the very fact the rule protects: that the person is in substance use treatment.

C **Incorrect** (trap: Misuses the technique). A written request changes nothing. Without the client's written consent or a court order, the program still cannot disclose, and asking for paperwork hints that a record exists. The program may say only that federal rules restrict disclosure.

Takeaway: Without written consent, a substance use program neither confirms nor denies that a person is a patient.

Sources: 42 CFR Part 2, 42 CFR 2.13(c); NASW Code of Ethics (2021), 1.07(b)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** none · **Cognitive** application · **Difficulty** 2 of 3

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67. INT-1612 (rev 2) · Best answer: C

An emergency department social work supervisor is under pressure to shorten patient stays. The supervisor has started cutting supervision short and giving quick orders. Now a supervisee has begun ending patient assessments abruptly and rushing patients through instructions. Which response by the supervisor is **BEST**?

A. Coach the supervisee to slow assessments with patients
B. Review the supervisee's recent assessments for missed details
C. Slow down and model an attentive pace with the supervisee

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Coaching on pacing is reasonable, but it treats the behavior as the supervisee's alone. Delivered within the same rushed, directive meetings, it is likely to repeat the dynamic. It fits better once the supervisor's own pace has changed.

B **Incorrect** (trap: Good but not best). Checking recent assessments may catch gaps that affect patients and is a fair quality step. It treats the rushing as the supervisee's performance problem, though, and leaves untouched the hurried supervision that the pattern mirrors.

C **Correct.** Parallel process can run in both directions: dynamics in the supervisory relationship can carry into the supervisee's work with clients (St. Arnaud, 2017). The supervisee's rushed, directive style with patients matches how the supervisor has been treating the supervisee. Changing the supervision itself addresses the likely source and shows the stance wanted with patients. It beats A because telling the supervisee to slow down, in the same hurried and directive way, repeats the pattern instead of changing it.

Takeaway: Parallel process can flow from supervision to practice; a supervisor first changes their own conduct in supervision.

Sources: St. Arnaud, K. O. (2017). Encountering the wounded healer: Parallel process and supervision. Canadian Journal of Counselling and Psychotherapy, 51(2), 131-144, whole article
Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.3 Transference and countertransference in supervision · **Qualifier** BEST · **Cognitive** application · **Difficulty** 3 of 3

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68. ETH-1076 (rev 1) · Best answer: C

A university study will interview youth in residential treatment about their placement experiences. A 15-year-old's guardian has signed permission. The social worker helping with recruitment explains the study to the youth, who listens, asks one question, and then says they do not want to take part. What is the **MOST** appropriate response?

A. Enroll the youth, since the guardian gave written permission
B. Review the study's benefits again and encourage joining
C. Accept the decision and note the refusal in the study log

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). A guardian's permission is necessary but not sufficient. For a youth able to understand the study, the youth's own agreement is also sought, so permission alone cannot override a clear refusal.

B **Incorrect** (trap: Overrides the client's choice). Answering questions is appropriate, but re-selling the study after a clear refusal pressures a youth in residential care, where staff shape daily life. Consent must be free of coercion (5.02(e)).

C **Correct.** When participants cannot give legal consent, researchers obtain permission from a proxy and also seek the person's own assent to the extent they are able (NASW Code 5.02(g)). Federal rules direct IRBs to provide for soliciting the assent of children capable of giving it (45 CFR 46.408(a)), and an interview about placement offers no direct benefit that could justify setting a refusal aside. Declining must carry no penalty (5.02(e)). C beats B because repeating the pitch to a youth who depends on program staff turns encouragement into pressure.

Takeaway: In research, a capable minor's refusal stands even when a parent or guardian has given permission.

Sources: NASW Code of Ethics (2021), 5.02(g); NASW Code of Ethics (2021), 5.02(e); 45 CFR Part 46 (Common Rule), 45 CFR 46.408(a)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.11 Research ethics: IRBs, human subjects protections, consent · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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69. INT-1001 (rev 2) · Best answer: C

A client in treatment informed by self psychology often describes successes at work and brightens when the social worker responds with interest. In one session, the social worker glances at a phone message while the client is speaking. The client falls silent, then says nothing they do really matters. What is the **MOST** appropriate response within this approach?

A. Explore what at work has left the client feeling unimportant
B. Reassure the client that their work successes are impressive
C. Acknowledge the lapse in attention and how it affected the client

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Exploring the feeling is sound in many approaches, but this option looks for its source at work. The client deflated right after the worker's glance at the phone, and self psychology attends first to that in-session empathic break. Exploring elsewhere leaves the rupture unnamed and unrepaired.

B **Incorrect** (trap: Good but not best). Mirroring achievements is part of self psychology, but praise now steps over what just happened. The client withdrew right after the worker's lapse, and that rupture needs to be recognized before affirmation can land.

C **Correct.** Self psychology holds that clients build a cohesive self through selfobject experiences such as mirroring, and that the worker's empathic failures are inevitable. What heals is recognizing and repairing them, which over time strengthens the client's own self-functions (Kohut, 1971; Kohut, 1984). The client deflated right after the worker's glance at a phone. Naming the lapse and its effect repairs the bond. It beats B because praise alone skips the rupture and leaves the client's experience of being dropped unspoken, and it beats A because exploring the work setting misses the in-session trigger.

Takeaway: In self psychology, recognizing and repairing the worker's empathic failures builds self-cohesion; praise alone does not repair a rupture.

Sources: Kohut, H. (1971). The Analysis of the Self. International Universities Press, mirror transference; Kohut, H. (1984). How Does Analysis Cure? University of Chicago Press, empathic failure and its repair
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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70. ASM-1521 (rev 2) · Best answer: B

At a memory clinic, the adult child who cares full-time for a parent with major neurocognitive disorder meets with the social worker. The caregiver quit a job last month, sleeps about 4 hours a night because the parent wanders, and has begun snapping at their own teenagers. Siblings out of state send money but no hands-on help. Which response is **BEST**?

A. Give the caregiver a standardized caregiver burden questionnaire
B. Connect the caregiver with respite care and a day program
C. Refer the caregiver to individual psychotherapy for stress

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). A standardized burden measure is useful when the level of strain is unclear or to track change over time. Here the job loss, sleep deprivation and family conflict already show heavy burden, so measuring it first delays relief.

B **Correct.** Caregiving for a parent with neurocognitive disorder strains the whole family system, and practical relief such as respite care is a core caregiver support (42 U.S.C. 3030s-1). This caregiver has lost a job and sleep and is straining relationships with their own children, signs of severe burden already evident (Zarit et al., 1980). It beats A because a burden questionnaire would mainly confirm what the stem shows, while respite care and a day program for the parent reduce the overload directly. Engaging siblings and planning for wandering safety can follow.

C **Incorrect** (trap: Hands off too soon). Counseling can help caregivers cope and may be added later. Referring out first treats the strain as an individual mental health problem when the main driver is an unrelieved caregiving load that respite can reduce.

Takeaway: When caregiver burden is already evident, prioritize concrete relief such as respite care and day programs over further measurement or referral out.

Sources: Zarit, S. H., Reever, K. E., & Bach-Peterson, J. (1980). Relatives of the impaired elderly: Correlates of feelings of burden. The Gerontologist, 20(6), 649-655, whole article; Older Americans Act, National Family Caregiver Support Program, 42 U.S.C. 3030s-1(b)
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.14 How caregiving affects families · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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71. INT-1046 (rev 2) · Best answer: C

PRETEST — unscored on this form; do not count it

A 15-year-old on probation has two new arrests, skips school, and spends time with peers who steal cars. Six months of weekly clinic-based family therapy have not changed the pattern. The probation officer raises residential placement, and the family wants the teen to stay home. What should the social worker do **NEXT**?

A. Recommend placement in a residential treatment center
B. Add school-based mentoring alongside the current family therapy
C. Refer the family to a multisystemic therapy program

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Residential care may be needed if intensive community treatment fails or safety requires it. Moving there now skips a less restrictive, evidence-based option that keeps the teen at home, as the family wants.

B **Incorrect** (trap: Good but not best). Mentoring can reach the school and peer side of the problem while the teen stays home, so it is tempting. It is a piecemeal add-on, though, to clinic-based therapy that has not changed the pattern in six months; an integrated, intensive model works on family, peers and school together.

C **Correct.** Multisystemic therapy is an intensive, home- and community-based family treatment for serious antisocial behavior. It works through the caregivers to change the teen's ties to family, peers, and school (Henggeler et al., 2009). New arrests, truancy, and delinquent peers after six months of clinic-based therapy call for a more intensive approach that reaches all of those systems while the teen stays home. It beats A because residential placement is more restrictive and removes the teen from the settings where change must last; it fits if intensive home-based treatment fails.

Takeaway: When outpatient family therapy fails for serious delinquency, try intensive home-based multisystemic therapy before more restrictive residential placement.

Sources: Henggeler, S. W., Schoenwald, S. K., Borduin, C. M., Rowland, M. D., & Cunningham, P. B. (2009). Multisystemic Therapy for Antisocial Behavior in Children and Adolescents (2nd ed.). Guilford Press, MST principles and service delivery

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.21 Family therapy models and interventions · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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72. INT-1561 (rev 1) · Best answer: C

During a structured play task in a parenting group, the social worker watches a parent say, 'Can you maybe start putting the blocks away? Let's clean up, okay?' The parent repeats this 5 times while the 6-year-old keeps playing. Which coaching suggestion would be **MOST** helpful?

A. Explain to the child why cleaning up matters first

B. Ignore the child's refusal to avoid a power struggle

C. Give one clear, specific command phrased as a statement

D. Promise the child a treat before the cleanup begins

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Brief reasons can help children understand rules, but an explanation before the request delays it and invites debate. A short reason can follow once the child complies.

B **Incorrect** (trap: Wrong model or concept). Planned ignoring works for minor attention-seeking behavior, not for refusing an instruction. Ignoring noncompliance teaches the child that requests can be skipped.

C **Correct.** Effective commands are direct, specific and given once as a statement rather than a question or a 'let's' suggestion, followed by praise for compliance or a planned consequence (McMahon & Forehand, 2003). The parent's question form invites refusal, and repeating it teaches the child that the first request can be ignored. It beats A

because an explanation before the command delays the request and gives the child more to argue with.

D **Incorrect** (trap: Misuses the technique). A reward offered before the task works like a bribe and can strengthen stalling. Praise or a privilege given after compliance reinforces following through.

Takeaway: Coach parents to give one clear, specific, statement-form command and follow through, rather than repeated questions or vague suggestions.

Sources: McMahon, R. J., & Forehand, R. L. (2003). Helping the Noncompliant Child: Family-Based Treatment for Oppositional Behavior (2nd ed.). Guilford Press, giving clear instructions
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.6 Building parenting capacity and skills · **Qualifier** MOST · **Cognitive** application · **Difficulty** 1 of 3

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73. ASM-1007 (rev 2) · Best answer: A

At an integrated primary care clinic, a 42-year-old client reports feeling low more days than not for the past 4 years, with little energy and a sense of being a failure. The client has never felt well for more than a few weeks at a time and says, 'This is just who I am.' The low mood began during a divorce that was finalized 4 years ago. The client works full time and cares for two children. What is the **MOST** likely diagnosis?

A. Persistent depressive disorder
B. Major depressive disorder
C. Adjustment disorder with depressed mood

Why each option is right or wrong

A **Correct.** Persistent depressive disorder involves depressed mood most of the day, more days than not, for at least 2 years in adults, with at least two other symptoms such as low energy or low self-esteem, and no symptom-free stretch longer than 2 months (DSM-5-TR). Four years of low mood with low energy and feeling like a failure, never well for more than a few weeks, fits. It beats C because adjustment disorder is not diagnosed when another disorder's criteria are met, and it resolves within 6 months after the stressor and its consequences end; the divorce was finalized 4 years ago.

B **Incorrect** (trap: Near-miss diagnosis). Major depressive disorder needs at least five symptoms nearly every day for 2 weeks. The client describes a chronic, lower-grade pattern with a few symptoms that still allows full-time work and parenting.

C **Incorrect** (trap: Near-miss diagnosis). Adjustment disorder starts within 3 months of a stressor, ends within 6 months after the stressor or its effects end, and is not used when another disorder's criteria are met. Four years of low mood that meets persistent depressive disorder criteria rules it out.

Takeaway: Chronic low mood more days than not for 2+ years in adults, with only brief symptom-free periods, suggests persistent depressive disorder.

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Persistent Depressive Disorder; DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Adjustment Disorders
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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74. INT-1013 (rev 1) · Best answer: C

A client with social anxiety in ACT-based treatment says, 'I'm boring, and everyone at work can tell.' The client has been skipping team lunches because of this. Which response is **MOST** consistent with acceptance and commitment therapy?

A. Ask the client to list evidence for and against being boring
B. Point out qualities that coworkers likely value in the client
C. Invite the client to observe the thought as passing words

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Examining evidence is cognitive restructuring from CBT, which disputes the accuracy of a thought. ACT does not argue with thought content; it changes how the client relates to the thought.

B **Incorrect** (trap: Good but not best). Reassurance may feel supportive, but it tries to talk the client out of the thought. It neither builds distance from the thought nor moves the client toward valued action, and the thought is likely to return.

C **Correct.** ACT aims to change a client's relationship to thoughts rather than their content. Cognitive defusion helps the client step back and see a thought as words passing through the mind, not a fact to obey (Hayes, Strosahl & Wilson, 2012). For example, the client might say, 'I'm having the thought that I'm boring.' Seeing the thought this way loosens its grip, so the client can act on values, such as joining lunch, while the thought is present. It beats A because weighing evidence aims to change the thought's content, a CBT restructuring move rather than an ACT one.

Takeaway: ACT's cognitive defusion changes how a client relates to a thought, treating it as a passing thought rather than disputing its accuracy.

Sources: Hayes, S. C., Strosahl, K. D., & Wilson, K. G. (2012). Acceptance and Commitment Therapy: The Process and Practice of Mindful Change (2nd ed.). Guilford Press, cognitive defusion
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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75. ASM-1094 (rev 1) · Best answer: B

At a health clinic, a client says their partner 'grabbed my throat until I almost blacked out' last month during an argument about the client moving out. The client laughs it off: 'It only happened once, and I'm fine now.' The client still plans to leave in a few weeks. What is the social worker's **BEST** response?

A. Accept the client's view that it was a one-time event
B. Explain that strangling sharply raises the risk of homicide
C. Explore why the client sees the incident as minor

Why each option is right or wrong

A **Incorrect** (trap: Ignores risk). The client's view deserves respect, and the client decides what to do. Accepting it without sharing what is known about strangulation leaves the client choosing without a key fact about lethal danger.

B **Correct.** Prior nonfatal strangulation is one of the strongest markers that an abusive partner may later kill (Glass et al., 2008), and leaving the relationship is a period of heightened danger (Campbell et al., 2003). The client minimizes the event and plans to

leave soon, so sharing this risk, then offering safety planning, supports an informed choice without overriding it. It beats C because exploring the client's view matters, but the client is making a high-stakes decision without knowing that this act signals lethal risk.

C **Incorrect** (trap: Good but not best). Understanding the client's minimization can guide later work. With a planned move-out weeks away, exploring first delays information the client needs to plan the departure safely.

Takeaway: Nonfatal strangulation and planned separation are major lethality markers in partner violence; share this risk so the client can plan safely.

Sources: Glass, N., et al. (2008). Non-fatal strangulation is an important risk factor for homicide of women. Journal of Emergency Medicine, 35(3), 329-335, whole article; Campbell, J. C., et al. (2003). Risk factors for femicide in abusive relationships: Results from a multisite case control study. American Journal of Public Health, 93(7), 1089-1097, Separation and lethality

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.4 Physical, sexual, psychological abuse and neglect: signs, dynamics, impact · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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76. ETH-1515 (rev 1) · Best answer: C

At intake, a social worker learns that a new client lives 3 houses away on the same block. The two have only exchanged casual greetings. The social worker discusses the overlap with the client, who wants to continue, and the supervisor agrees the risk is low. What should the social worker do **NEXT**?

A. Transfer the client to a colleague to be safe

B. Wait to record the overlap until a problem arises

C. Document the discussion and the boundary plan

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). Transfer would fit if the overlap threatened objectivity or the client wanted a change. Here the client prefers to continue and the supervisor judged the risk low, so transfer would override the client's choice without need.

B **Incorrect** (trap: Misreads law or ethics). Waiting leaves no record of how the worker assessed and managed the overlap. If a problem later arises, the missing documentation makes it hard to show that the boundary was handled with care.

C **Correct.** When a social worker continues with a client despite an overlapping relationship, the steps taken to manage it belong in the record: the discussion, the client's choice, the consultation and the plan for contact outside sessions (Reamer, 2020). Timely, sufficient documentation supports continuity of care and shows the worker owns the boundary (NASW Code 3.04(b)). It beats A because the client chose to continue, the supervisor judged the risk low, and transfer would override that choice with no clear benefit.

Takeaway: After deciding to continue despite an overlapping relationship, document the discussion, consultation and boundary plan.

Sources: NASW Code of Ethics (2021), 3.04(b); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, risk management: documenting boundary decisions
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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77. ASM-1660 (rev 2) · Best answer: B

A hospital social worker begins a biopsychosocial assessment with a 74-year-old patient admitted overnight after a fall. While describing their living situation, the patient becomes anxious and says their spouse, who has major neurocognitive disorder and needs daily help, has been home alone since the ambulance came yesterday afternoon. The patient cannot name anyone who has checked on the spouse. What should the social worker do **FIRST**?

A. Finish the assessment, then plan for the spouse's care
B. Arrange for someone to see the spouse at home right away
C. Report the spouse's situation to adult protective services

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Completing the assessment is the usual order, and the remaining domains still matter. But an adult who cannot care for themselves has been alone for many hours, and waiting until the interview ends risks harm that can be prevented now.

B **Correct.** When an assessment uncovers an immediate safety need, that need takes priority over finishing the interview; safety and basic needs come first (Maslow, 1943). A spouse with major neurocognitive disorder who needs daily help has been alone since yesterday, and no one has checked. Arranging an urgent in-person check, through a relative, a neighbor or, if needed, a police welfare check, protects someone who cannot protect themselves. It beats C because an adult protective services referral takes time to reach the home; it fits if no safe care can be arranged once the spouse has been seen.

C **Incorrect** (trap: Right action, wrong time). Adult protective services may be needed if no safe care can be arranged, but a report does not get anyone to the home quickly. The urgent check comes first, and a report can follow if needed.

Takeaway: When assessment reveals an immediate safety need, such as a dependent adult left alone, act on it before completing the assessment.

Sources: Maslow, A. H. (1943). A theory of human motivation. Psychological Review, 50(4), 370-396, hierarchy of needs: safety and physiological needs
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.7 Biopsychosocial assessment framework and its components · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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78. ETH-1554 (rev 1) · Best answer: A

In the fourth session at an outpatient clinic, a client voices strong contempt for people who receive public assistance. The social worker grew up in a family that relied on public assistance, finds the remarks upsetting and intends to refer the client elsewhere. The client is making progress on work-related anxiety. What should the social worker do **FIRST**?

A. Seek supervision about the reaction before changing the plan
B. Refer the client to a colleague with a different background
C. Tell the client how the remarks affect the social worker
D. Challenge the client's views on public assistance in session

Why each option is right or wrong

A **Correct.** Social workers do not end or transfer services simply because a client's views clash with their own. Ending care that is working risks abandonment (NASW Code 1.17(b)), and referral is meant for when special knowledge is needed or the worker is not being effective (1.16(a)). Here the worker's personal history is driving the plan to refer. Seeking supervision, as the Code encourages when it serves the client (2.05(a)), helps the worker manage the reaction and judge whether effective service can continue. It beats B because referral may become right later, but making it first ends care that is helping.

B **Incorrect** (trap: Hands off too soon). Referral may be appropriate if, after supervision, the worker cannot serve the client effectively. Making it first, because of the worker's reaction to the client's views, ends treatment that is helping the client's anxiety.

C **Incorrect** (trap: Boundary slip or collusion). Describing the worker's personal reaction shifts the session to the worker's needs and burdens the client. Any self-disclosure should serve the client's goals, which this does not.

D **Incorrect** (trap: Too directive). Arguing with the client's political views is not part of the anxiety treatment the client sought and risks a rupture. The worker's values conflict is the worker's to manage, not the client's to fix.

Takeaway: A values conflict alone does not justify ending or transferring services; take the reaction to supervision before changing the plan.

Sources: NASW Code of Ethics (2021), 1.17(b); NASW Code of Ethics (2021), 1.16(a); NASW Code of Ethics (2021), 2.05(a)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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79. ETH-1099 (rev 1) · Best answer: C

Two social workers co-lead a 12-week outpatient group for adults in early recovery from substance use. At week 6, they begin a romantic relationship; neither supervises the other. Members have said that having both leaders matters to them. What is the **MOST** appropriate step for the co-leaders?

A. Keep the relationship private and continue co-leading
B. Tell the group about the relationship at the next session
C. Discuss with their supervisor whether to change co-leaders
D. Have one co-leader leave the group before next week

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). The relationship is personal, but co-leaders whose relationship could shape their group decisions face a possible conflict of interest. Continuing without review skips the check 2.06(b) calls for.

B **Incorrect** (trap: Boundary slip or collusion). Telling members shifts the leaders' issue onto clients in early recovery and invites distraction. Whether a conflict exists should be settled by the leaders with supervision, not by the group.

C **Correct.** Social workers should avoid sexual relationships with colleagues when a conflict of interest is possible, and those involved should transfer professional responsibilities when necessary (NASW Code 2.06(b)). A co-leaders' relationship can shape group decisions and dynamics, so whether a change is needed deserves an objective look, while members' attachment to both leaders makes an abrupt exit risky. Consulting serves the clients' interests (2.05(a)). C beats D because a sudden departure mid-cycle may disrupt the group more than a planned transition the supervisor helps design.

D **Incorrect** (trap: Good but not best). Transferring responsibilities may prove necessary, and a planned change could be the outcome. Leaving abruptly mid-cycle, without preparing members who value both leaders, risks harming the group.

Takeaway: When a relationship with a colleague may create a conflict of interest, consult on whether to transfer responsibilities and plan any change.

Sources: NASW Code of Ethics (2021), 2.06(b); NASW Code of Ethics (2021), 2.05(a)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.2 Legal-ethical duties to clients, colleagues, profession, and society · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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80. ASM-1662 (rev 2) · Best answer: C

PRETEST — unscored on this form; do not count it

At an outpatient intake, a client referred for poor sleep spends the first 20 minutes describing a long dispute with a neighbor over parking. Each time the social worker reflects the frustration, the client agrees the dispute has been 'eating at' them and adds another detail about the neighbor. About 30 minutes remain. What should the social worker do **NEXT**?

A. Continue reflecting the client's anger at the neighbor
B. Ask a quick series of yes-or-no questions about sleep
C. Link the neighbor stress to sleep and shift the focus there

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Reflection built rapport and showed understanding, and the client confirmed it. Repeating it now adds little and lets the remaining time pass without exploring the sleep problem the client came for.

B **Incorrect** (trap: Misuses the technique). Closed questions gather quick facts, but a rapid string of them after a long open story feels abrupt and yields thin information. Open questions about sleep, tied to what the client shared, will reveal more.

C **Correct.** Focusing is the skill of guiding an interview toward the topics that matter for the client's goals while staying attuned (Hepworth et al.). The client has been heard, has confirmed the reflection, and has limited time left for the sleep problem that brought them in. Connecting the neighbor stress to sleep, for example by asking how the conflict affects nights, honors what the client shared and moves the intake forward. It beats A because more reflection repeats a step the client has already confirmed and uses time the sleep assessment needs.

Takeaway: Once a client feels heard, use focusing to connect their story to the presenting concern and move the interview toward it.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Focusing: selecting topics and maintaining focus

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.2 Interviewing skills: clarifying, validating, confronting, reflecting, interpreters · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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81. ETH-1610 (rev 1) · Best answer: B

Six months ago, a store guard falsely accused a 16-year-old Black student of theft. Since then, the student, who attends a mostly white high school, has joined the Black Student Union, reads Black history late into the night, wears African-print clothing, and calls longtime white friends 'part of the problem.' The parent asks the school social worker what is happening. In Cross's model, which stage **MOST** likely fits?

A. Encounter
B. Immersion-Emersion
C. Internalization
D. Pre-encounter

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Encounter is the event or realization that shatters a person's earlier view of race, here the false accusation. The student has moved past that moment into active, intense identity work.

B **Correct.** In Cross's model of Black identity development, the Immersion-Emersion stage follows a racial encounter and brings intense engagement with Black culture and history, often with anger at or withdrawal from white people (Cross, 1991). The student's new group, reading, dress and sharp rejection of white friends fit this stage. It beats A because the false accusation 6 months ago was the encounter, the event that set off the change; the student's current behavior shows what followed. Seeing a developmental stage helps the social worker reassure the parent and support the student.

C **Incorrect** (trap: Wrong model or concept). Internalization brings a secure, settled Black identity with less anger and more openness to relationships across race. Calling longtime friends 'part of the problem' shows the student has not reached this calmer stage.

D **Incorrect** (trap: Wrong model or concept). Pre-encounter describes the period before race is salient, when a person may adopt the dominant culture's views or downplay race. The student's identity is now strongly focused on race.

Takeaway: Intense engagement with one's racial group and anger at the dominant group after a racial encounter fits Cross's Immersion-Emersion stage.

Sources: Cross, W. E., Jr. (1991). Shades of Black: Diversity in African-American Identity. Temple University Press, nigrescence stages
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.3 Culture, race, gender, sexuality, intersectionality shaping identity and behavior · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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82. INT-1620 (rev 2) · Best answer: B

A family housing program's grant pays for two case managers. One position has been vacant for 4 months, leaving unspent salary funds. Meanwhile, several client families face eviction over small rent debts. The grant agreement requires the funder's written consent for spending outside its line items. Which action by the program director is **BEST**?

A. Use the savings for rent aid and inform the funder at year end
B. Ask the funder to approve moving the savings to rent aid
C. Use the savings for rent aid once the board approves it

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). The goal is worthy, but redirecting restricted grant funds without the required approval uses them for unauthorized purposes. Reporting the change later does not cure the lack of approval and puts the program's accountability to the funder at risk.

B **Correct.** Social workers should be diligent stewards of agency resources and should not use funds for purposes other than those intended (NASW Code 3.09(g)). Line-item grant budgets exist to control spending by category (Kettner, Moroney & Martin, 2022), and this agreement requires approval for changes. A budget modification request lets the director respond to the families' urgent need through legitimate channels. It beats A because spending first and reporting later uses restricted money without authorization, even for a worthy purpose.

C **Incorrect** (trap: Misreads law or ethics). Board approval is a sound internal control, but this grant agreement requires the funder's written consent for spending outside its line items. An agency board cannot authorize a change to a funder's restricted budget.

Takeaway: Restricted grant funds are redirected only with the funder's approval, even when the new purpose meets urgent client needs.

Sources: NASW Code of Ethics (2021), 3.09(g); Kettner, P. M., Moroney, R. M., & Martin, L. L. (2022). Designing and Managing Programs: An Effectiveness-Based Approach (6th ed.). SAGE, Ch. 12-13: line-item, functional and program budgeting
Area III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.6 Budgeting, fiscal oversight, and allocating resources · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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83. ETH-1038 (rev 1) · Best answer: C

PRETEST — unscored on this form; do not count it

At a residential program for adolescents, a social worker arriving for an evening group sees a colleague clocking in for the overnight shift. The colleague smells strongly of alcohol, slurs words, and will be the only staff member awake with 12 residents. What should the social worker do **FIRST**?

A. Speak with the colleague one-on-one about the alcohol smell
B. Stay late to cover and raise it with the colleague tomorrow
C. Notify the on-call supervisor right away

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Speaking with the colleague first is the Code's usual starting point and would be right if no clients were at immediate risk. An apparently intoxicated sole overnight staff member creates danger tonight, so the supervisor must know now.

B **Incorrect** (trap: Ignores risk). Covering the shift helps tonight, but it hides the problem from the people responsible for staffing and safety. The impairment and the agency's response to it still need to reach a supervisor today.

C **Correct.** The Code's first step for an impaired colleague is to talk with them, but only when feasible (NASW Code 2.08(a)). This colleague appears intoxicated and is about to be the only awake staff member for 12 adolescents, so client safety requires going straight to the agency's channel (NASW Code 2.08(b)). C beats A because a private talk suits impairment that is not putting clients at immediate risk; tonight's shift cannot wait for the colleague to respond.

Takeaway: When a colleague's impairment puts clients at immediate risk, report through agency channels at once rather than starting with a private talk.

Sources: NASW Code of Ethics (2021), 2.08(a); NASW Code of Ethics (2021), 2.08(b)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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84. INT-1578 (rev 2) · Best answer: A

A parenting program screens 400 parents with a stress scale and offers places only to those in the top 10%. Because of a staffing delay, these parents are rescored 3 weeks later, before any sessions begin. Their scores have fallen noticeably as a group. Parents with moderate scores who were rescored at the same time changed little. What **MOST** likely explains the drop?

A. Statistical regression toward the mean
B. A testing effect from completing the scale twice
C. History, such as an outside event during the wait

Why each option is right or wrong

A **Correct.** When people are chosen because their scores are extreme, their later scores tend to move toward the average even with no intervention, because part of an extreme score reflects temporary factors and measurement error (Campbell & Stanley, 1963). Only the highest scorers changed, while moderate scorers rescored on the same schedule did not. This beats B because a testing effect from taking the scale twice would also appear in the moderate group, and it did not.

B **Incorrect** (trap: Wrong model or concept). A testing effect happens when taking a measure changes later scores. It would show up in both groups, since both completed the scale twice on the same schedule, yet the parents with moderate scores barely changed.

C **Incorrect** (trap: Wrong model or concept). History means outside events between measurements. An event during the same 3 weeks would likely affect both groups' scores, not only the parents selected for the highest stress.

Takeaway: Groups chosen for extreme scores tend to score closer to average on retest, even without any intervention.

Sources: Campbell, D. T., & Stanley, J. C. (1963). Experimental and Quasi-Experimental Designs for Research. Rand McNally, threats to internal validity: statistical regression
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.5 Research design, data collection/analysis, reliability, validity · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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85. ETH-1018 (rev 2) · Best answer: A

An adult patient on an inpatient psychiatric unit has declined to let staff share any information with family. The patient's parent calls the social worker and wants to describe changes in the patient's behavior in the weeks before admission. What is the **BEST** response?

A. Hear the parent's account but disclose no patient details
B. Decline the call, since the patient has refused family contact
C. Give a brief update so the parent feels less worried

Why each option is right or wrong

A **Correct.** Confidentiality limits what the social worker discloses, not what the worker may hear. Because the patient declined disclosure to family, the worker shares nothing about the patient's care (NASW Code 1.07(b); 45 CFR 164.510(b)). The worker may still listen to what the parent offers, which can matter for safety and treatment; HHS guidance on HIPAA and mental health says providers may listen to family members. A beats B because refusing the call protects nothing and loses information the team may need.

B **Incorrect** (trap: Good but not best). The patient's refusal limits what staff disclose, not what they receive. Ending the call discards information that may bear on safety and treatment, without adding any protection for the patient.

C **Incorrect** (trap: Misreads law or ethics). The patient declined all sharing with family, so even a brief update would disclose information without consent. An update would be appropriate if the patient agreed or an emergency required it.

Takeaway: A client's refusal to allow disclosure limits what the social worker shares, not what the worker may hear from family.

Sources: NASW Code of Ethics (2021), 1.07(b); HIPAA Privacy Rule, 45 CFR 164.510(b); U.S. Department of Health and Human Services. HIPAA Privacy Rule and Sharing Information Related to Mental Health, Guidance on listening to family members

Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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86. INT-1136 (rev 1) · Best answer: C

A graduate student tells a campus social worker that worries about loans and grades pop up all day, breaking concentration during study time and lectures. The student has tried to push the worries out of mind, but they come back stronger. Which stress management technique is **BEST** to teach?

A. Shift to a quick task whenever worries start

B. Talk worries over with a friend right away

C. Postpone worries to a set daily worry period

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Brief distraction can help in a single stressful moment. Used every time a worry starts, it lets each worry interrupt study and never gives the student a set time to deal with real problems such as loans.

B **Incorrect** (trap: Good but not best). Support from friends eases stress, but talking each worry over the moment it comes up interrupts study as much as the worry does and can turn into constant reassurance seeking.

C **Correct.** Worry postponement, a stimulus control technique, has the client notice a worry, set it aside, and save it for a fixed daily period in one place (Borkovec et al., 1983). It narrows the times and cues linked to worrying, freeing study and lecture time while the worries still get attention later. The student's attempts to suppress the worries backfired, as suppression often does (Wegner et al., 1987). It beats A because distracting each time still lets every worry interrupt and leaves no planned time to address real concerns.

Takeaway: For worry that intrudes all day, postponing it to a set daily worry period works better than suppression or constant distraction.

Sources: Borkovec, T. D., Wilkinson, L., Folensbee, R., & Lerman, C. (1983). Stimulus control applications to the treatment of worry. Behaviour Research and Therapy, 21(3), 247-251, whole article; Wegner, D. M., Schneider, D. J., Carter, S. R., & White, T. L. (1987). Paradoxical effects of thought suppression. Journal of Personality and Social Psychology, 53(1), 5-13, whole article

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.9 Managing stress: client techniques · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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87. ETH-1161 (rev 1) · Best answer: A

A social worker at a group practice learns that the new video platform automatically records every telehealth session and stores the files for quality review. The consent forms do not mention recording, and clients have not been told. Only the practice's owners can change the platform's settings. The office manager says the vendor contract requires it. What should the social worker do **FIRST**?

A. Raise the recording problem with the practice's leadership
B. Tell each of their clients about the recordings at the next session
C. Continue sessions as usual since the contract requires recording
D. Report the practice to the state licensing board

Why each option is right or wrong

A **Correct.** Clients must give informed consent before sessions are recorded (NASW Code 1.03(h)), and an employer's policies or contracts should not interfere with ethical practice, which workers should improve through the organization (3.09(b), 3.09(d)). Because only the owners can stop the recording and every client is affected, the first step is to bring it to practice leadership so recording stops or consent is obtained practice-wide. It beats B because telling one's own clients matters but leaves the recordings running and other clinicians' clients uninformed. Disclosure to clients should follow promptly as part of the fix.

B **Incorrect** (trap: Right action, wrong time). Clients deserve to know, and informing them is part of the remedy. Done alone and first, it leaves the recordings running for everyone and does not reach other clinicians' clients. Disclosure should follow once the practice acts.

C **Incorrect** (trap: Misreads law or ethics). A vendor contract does not replace each client's informed consent to recording. Employer policies and contracts should not interfere with ethical practice (3.09(d)), so continuing as usual lets an unconsented practice go on.

D **Incorrect** (trap: Skips assessment). A board complaint skips the internal channel that can fix the problem today. Formal outside reporting fits if the practice refuses to act after the concern is raised, not before anyone in the practice has been asked.

Takeaway: When an agency practice violates client consent, raise it first with whoever can fix it, then make sure clients are informed.

Sources: NASW Code of Ethics (2021), 1.03(h); NASW Code of Ethics (2021), 3.09(b); NASW Code of Ethics (2021), 3.09(d)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

88. INT-1056 (rev 1) · Best answer: B

During a video session, one partner raises a complaint about household chores. The other partner goes silent, looks away from the camera, and says, 'My heart is pounding. I can't do this right now.' The social worker uses the Gottman Method. What should the social worker do **FIRST**?

A. Ask the quiet partner to share their view now
B. Call a break of at least 20 minutes to self-soothe
C. Teach both partners a structured listening exercise

Why each option is right or wrong

A **Incorrect** (trap: Misuses the technique). Pressing a flooded partner to respond usually deepens the shutdown or sparks a defensive reply. Their view matters, and it is best asked for after a self-soothing break, when they can think and listen again.

B **Correct.** Gottman describes flooding: physical arousal so high that a partner shuts down, the state behind stonewalling. The antidote is a break of at least 20 minutes spent self-soothing, because the body needs that long to calm, followed by a return to the topic (Gottman & Silver, 1999). A pounding heart and 'I can't do this' signal flooding. It beats C because listening skills are useful for this couple, but a flooded partner cannot take in new skills; they help once both partners are calm.

C **Incorrect** (trap: Right action, wrong time). Structured listening can help this couple talk about chores, but it is a skill to build once arousal drops. Teaching it to a partner who is flooded right now is unlikely to work.

Takeaway: When a partner is flooded, pause for at least 20 minutes of self-soothing before continuing any discussion or skill work.

Sources: Gottman, J. M., & Silver, N. (1999). The Seven Principles for Making Marriage Work. Crown, flooding and stonewalling; self-soothing breaks
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.27 Couples treatment approaches · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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89. ETH-1666 (rev 1) · Best answer: D

In a state where medical aid in dying is lawful, a hospice patient with terminal cancer asks the social worker for help understanding the request process and talking with family. The social worker objects to aid in dying on religious grounds and, after supervision, concludes they cannot discuss it without their views showing. What is the **BEST** action?

A. Tell the patient about the objection so the patient understands
B. Suggest that the patient take the questions to the physician
C. Continue with the patient while keeping personal views private
D. Arrange promptly for a colleague to help the patient

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). Sharing the objection makes the patient manage the worker's beliefs and may feel like judgment at a vulnerable time. A brief, honest word about the handoff is fine; the reasons belong in supervision, not with the patient.

B **Incorrect** (trap: Good but not best). The physician handles the medical steps of a request, but the patient asked for help understanding the process and talking with family, which is social work's role. Redirecting without a handoff leaves that need unmet.

C **Incorrect** (trap: Outside the social work role). Setting views aside is the goal when a worker can manage it. This worker has concluded, after supervision, that their views would show, so continuing risks subtly steering the patient's choice.

D **Correct.** The patient has a right to informed, supported decisions about lawful end-of-life options (NASW Code 1.02). When a worker cannot serve a client effectively, the worker refers and arranges an orderly transfer (NASW Code 1.16(a), (b)); conflicts between personal and professional values are resolved in the client's favor (Reamer, 2018). A prompt handoff to a colleague keeps the patient from being delayed or judged. It beats C because the worker has already concluded, after supervision, that their views would show, and pressing on risks steering a dying patient.

Takeaway: When personal values block competent help with a lawful choice, arrange a prompt, orderly handoff without burdening the client.

Sources: NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.16(a), (b); Reamer, F. G. (2018). Social Work Values and Ethics (5th ed.). Columbia University Press, conflicts between personal and professional values

Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.6 Law and ethics around death and dying · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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90. ASM-1569 (rev 3) · Best answer: B

A child welfare social worker has met monthly for 4 months with a parent working toward reunification, and they have a trusting working relationship. Today the parent says, 'I'm doing everything the plan asks.' Records show the parent missed 4 of the last 6 parenting classes, and a court review is scheduled next month. Earlier in this visit, the parent vented about how long the process is and said, 'At least you understand.' What should the social worker do **NEXT**?

A. Affirm the parent's commitment to getting the children back

B. Point out the gap between the parent's statement and attendance

C. Explore the parent's feelings about the case plan

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). Affirmation strengthens motivation, but affirming effort while ignoring the missed classes goes along with an inaccurate picture. It would fit if the parent's account matched the record.

B **Correct.** Confrontation points out a discrepancy between what a client says and does, and it works best within a trusting relationship and after empathic responding (Hepworth et al., confrontation). Here the alliance is solid after 4 months, the parent's frustration has just been heard and acknowledged, and the missed classes threaten reunification. Naming the gap respectfully gives the parent a chance to address it before next month's court review. It beats C because exploring feelings again, right after the parent felt understood, postpones the issue that most affects the children's return.

C **Incorrect** (trap: Good but not best). Exploring feelings is useful early on or when feelings are unclear. The parent has just aired that frustration and felt understood, so more exploration delays the discrepancy that matters most for reunification.

Takeaway: Within an established alliance and after empathic responding, respectfully point out discrepancies between a client's words and actions.

Sources: Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Additive empathy, interpretation, and confrontation

Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.2 Interviewing skills: clarifying, validating, confronting, reflecting, interpreters · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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91. ETH-1582 (rev 1) · Best answer: C

At a first session in private practice, a Black client describes tension with neighbors since moving to a mostly white suburb. The client pauses, looks at the white social worker, and says, 'I don't know if you'd get this part.' What is the social worker's **BEST** response?

A. Assure the client of the worker's training in diverse cultures

B. Note the comment and return to it once rapport is stronger

C. Name the racial difference and ask how it may affect the work

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Citing training defends the worker's competence instead of engaging the client's concern. It also implies that cultural knowledge can stand in for hearing the client as the expert on their own experience.

B **Incorrect** (trap: Right action, wrong time). Rapport matters, but the client raised the issue now. Setting it aside signals that race is off limits and leaves the doubt unanswered when the client was most open to discussing it.

C **Correct.** Broaching means the social worker openly raises race and culture, including differences between worker and client, and invites the client to discuss how they matter (Day-Vines et al., 2007). The client has signaled doubt tied to the racial difference, so naming it directly shows the topic is welcome and the worker can handle it. It beats B because waiting for more rapport leaves the doubt unanswered at the moment it was raised, and avoidance can erode trust. It also honors the client as the expert on their experience (NASW Code 1.05(c)).

Takeaway: When racial difference surfaces in the relationship, raise it openly and invite the client's view of its effect on the work.

Sources: Day-Vines, N. L., et al. (2007). Broaching the subjects of race, ethnicity, and culture during the counseling process. Journal of Counseling & Development, 85(4), 401-409, whole article; NASW Code of Ethics (2021), 1.05(c)

Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.5 Power differentials, racial privilege, internalized inferiority/superiority · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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92. INT-0106 (rev 1) · Best answer: A

Parents seek help for their teen's school refusal. The social worker meets with the parents alone, stays calm while they argue, asks each parent what they think rather than feel about the conflict, and avoids siding with either one. Which family therapy model is the social worker **MOST** likely using?

A. Bowen family systems therapy
B. Structural family therapy
C. Strategic family therapy
D. Experiential family therapy

Why each option is right or wrong

A **Correct.** Bowen family systems therapy aims to raise differentiation of self, the ability to think clearly while staying emotionally connected (Bowen, 1978; Kerr & Bowen, 1988). The therapist stays calm and detriangulated, asks thinking-focused process questions, and often works with the parents without the symptomatic child, whose problem is seen as part of the parents' anxiety. Every cue fits. It beats B because a structural therapist would join the whole family and actively restructure interactions, sometimes taking sides on purpose, rather than stay neutral and reflective.

B **Incorrect** (trap: Wrong model or concept). Structural therapy works with the whole family in the room, using enactment, boundary making and unbalancing, in which the therapist may side with one member on purpose. Neutral, thinking-focused questions with the parents alone point elsewhere.

C **Incorrect** (trap: Wrong model or concept). Strategic therapy targets the sequence that maintains the symptom, often through directives or paradoxical tasks. This social worker gives no directive and focuses on how each parent thinks.

D **Incorrect** (trap: Wrong model or concept). Experiential therapy, such as Satir's, emphasizes emotional expression, congruent communication and the therapist's warmth. Asking for thoughts rather than feelings runs opposite to that emphasis.

Takeaway: A calm, detriangulated stance with thinking-focused process questions signals Bowen therapy; active restructuring signals structural; directives signal strategic.

Sources: Bowen, M. (1978). Family Therapy in Clinical Practice. Jason Aronson, differentiation of self; triangles; Kerr, M. E., & Bowen, M. (1988). Family Evaluation. W. W. Norton, therapist's role: detriangulation
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.21 Family therapy models and interventions · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

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93. ETH-1552 (rev 3) · Best answer: A

A child welfare social worker meets with a parent whose 2 children entered foster care 4 months ago. The parent is starting substance use treatment and believes there is no deadline for reunification. What is the **MOST** appropriate way to respond to the parent's belief?

A. Explain the permanency time frames and how they shape the plan
B. Refer the parent to their attorney to explain the time frames
C. Tell the parent that rights end automatically after 15 months

Why each option is right or wrong

A **Correct.** Clients have a right to plain-language information about the limits and conditions that affect their services (NASW Code 1.03(a)). Under the Adoption and Safe Families Act, a permanency hearing is held within 12 months, and the state generally must seek termination of parental rights once a child has been in care 15 of the last 22 months, with exceptions (42 U.S.C. 675(5)). Explaining this now lets the parent use the remaining time well. It beats B because legal strategy belongs with the parent's attorney, but plain information about the case's time frames is part of the social worker's role, and handing it off leaves a costly misbelief in place now.

B **Incorrect** (trap: Hands off too soon). Parents in dependency cases usually have counsel, and questions of legal strategy belong with the attorney. Basic information about permanency time frames is part of the caseworker's job, though, and sending the parent elsewhere leaves the belief uncorrected while the federal clock runs.

C **Incorrect** (trap: Misreads law or ethics). This overstates the rule. The agency generally must file or join a petition at 15 of 22 months, but exceptions exist, such as care by a relative or a documented compelling reason, and a court decides. Accurate information, not threats, supports engagement.

Takeaway: Explain policy-driven time limits, such as federal permanency deadlines, early and accurately so clients can make informed choices.

Sources: NASW Code of Ethics (2021), 1.03(a); Adoption and Safe Families Act of 1997, 42 U.S.C. 675(5)(C), (E)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.8 How local, state, federal policy affects service delivery · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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94. ASM-1510 (rev 1) · Best answer: C

On an inpatient psychiatric unit, a patient tells the day nurse and the social worker that they are the only staff who truly understand. The patient describes the evening staff as heartless and incompetent. At the next team meeting, day and evening staff argue sharply over whether the patient's privileges should be reduced. Which defense is the patient **MOST** likely using?

A. Projection
B. Displacement
C. Splitting

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Projection places one's own unacceptable feelings onto someone else, such as accusing others of a hostility one feels. Calling the evening staff heartless fits partly, but the defining feature here is all-good versus all-bad sorting of staff.

B **Incorrect** (trap: Wrong model or concept). Displacement shifts feelings from a threatening target to a safer one, such as anger at family vented at staff. Nothing suggests the feelings belong to another target; the pattern is idealizing some staff and devaluing others.

C **Correct.** Splitting divides people into all good or all bad, because the person cannot hold positive and negative views of the same person at once (Kernberg, 1975). The patient casts day staff as uniquely understanding and evening staff as heartless, and

the team itself has divided along those lines, a classic inpatient sign of splitting. It beats A because projection attributes one's own unacceptable feelings to others, while this pattern is extreme good-versus-bad sorting of people. Recognizing splitting points the team toward a unified approach.

Takeaway: Splitting sorts people into all-good and all-bad camps and can divide treatment teams; it is common in borderline personality organization.

Sources: Kernberg, O. F. (1975). Borderline Conditions and Pathological Narcissism. Jason Aronson, splitting
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.11 Defense mechanisms and how they shape behavior and relationships · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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95. ETH-1517 (rev 1) · Best answer: A

Which approach to client testimonials is consistent with the NASW Code of Ethics?

A. Avoid soliciting testimonials from current clients
B. Request testimonials from clients who are doing well
C. Ask for reviews during a client's final session

Why each option is right or wrong

A **Correct.** The Code directs social workers to avoid soliciting testimonial endorsements, including permission to reuse past statements, from current clients or others vulnerable to undue influence (NASW Code 4.07(b)). The power and dependence in a helping relationship mean a client may feel unable to refuse. The Technology Standards apply the same caution to online testimonials (Technology Standard 2.14). It beats C because a client at a final session is still a current client, and the ending can heighten the wish to please the worker.

B **Incorrect** (trap: Misreads law or ethics). Choosing clients who are doing well does not reduce the pressure they may feel to agree. Current clients are vulnerable to undue influence regardless of progress, so the Code advises against asking them.

C **Incorrect** (trap: Misreads law or ethics). Timing the request at the last session seems to avoid affecting treatment, but the person is still a current client. Gratitude and the wish to end well can make refusal especially hard at that moment.

Takeaway: Do not solicit testimonials from current clients or others vulnerable to undue influence.

Sources: NASW Code of Ethics (2021), 4.07(b); NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 2.14
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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96. INT-1531 (rev 1) · Best answer: B

Before the annual IEP meeting for an 8-year-old autistic student, the parent tells the school social worker, 'There will be eight of them and one of me. I'll just sign whatever they put in front of me.' The parent has strong views about the child's reading support. What is the social worker's **BEST** response?

A. Offer to present the parent's reading concerns to the team
B. Help the parent write down priorities and questions to raise
C. Reassure the parent that the team wants what is best for the child

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Speaking for the parent may get the concern heard once, but it leaves the parent in a passive role at this and future meetings. Speaking on the parent's behalf fits if the parent asks for it or cannot attend.

B **Correct.** Under IDEA, parents are members of the IEP team, and schools must make sure they have a real chance to participate (34 CFR 300.321, 300.322). This parent has clear views but expects to be overwhelmed, so the most useful step is helping the parent organize priorities and questions to bring, which builds the parent's own voice on the team. It beats A because presenting the concerns for the parent may help once but keeps the parent passive; supporting the client's self-advocacy comes before advocating on their behalf (Hepworth et al.).

C **Incorrect** (trap: Good but not best). Reassurance may ease anxiety, but it does nothing to help the parent voice strong views about reading support. It can also discourage the parent from speaking up at all.

Takeaway: Help parents take part as full IEP team members by preparing their own priorities, rather than speaking for them or simply reassuring them.

Sources: IDEA Part B regulations, 34 CFR Part 300, 34 CFR 300.321(a)(1); 300.322(a); Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Advocacy and social action: case and cause advocacy
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.3 Inter- and intradisciplinary teamwork (co-therapy, care conferences, IEPs) · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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97. INT-1000 (rev 1) · Best answer: B

A retired client has been in weekly psychodynamic therapy exploring lifelong conflicts with authority figures. The client's spouse died suddenly 2 weeks ago. The client now arrives disheveled, has been skipping meals, and says they cannot think clearly enough to deal with the funeral bills. What is the **BEST** way for the social worker to adjust the approach?

A. Interpret how the loss revives early conflicts with authority
B. Shift to ego-supportive work on daily coping and tasks
C. Refer the client to a grief support group instead

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). Interpretation is a core ego-modifying technique and fits the client's earlier work. Right now, though, the client's coping is overwhelmed, and stirring up old conflicts could deepen the disorganization. Insight-oriented work can resume once daily functioning is steadier.

B **Correct.** Ego psychology distinguishes ego-supportive from ego-modifying work. When crisis weakens a client's ego functioning, the social worker shifts to supportive methods: bolstering coping, helping with concrete tasks, and lending structure and judgment (Goldstein, 1995). Disorganization, skipped meals, and trouble handling bills after a sudden death show that the client's coping is overwhelmed. It beats A because interpreting old conflicts asks a strained ego to tolerate more anxiety now; insight work can resume once the client stabilizes.

C **Incorrect** (trap: Hands off too soon). A grief group may help later as an added support. Replacing individual therapy with a group now removes the established relationship at the moment the client most needs steady, individualized help with basic functioning.

Takeaway: When crisis overwhelms ego functioning, psychodynamic work shifts from insight-oriented interpretation to ego-supportive help with coping and daily tasks.

Sources: Goldstein, E. G. (1995). Ego Psychology and Social Work Practice (2nd ed.). Free Press, ego-supportive and ego-modifying intervention
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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98. ETH-1676 (rev 2) · Best answer: C

A white clinical supervisor notices that a Black supervisee agrees with every plan in supervision but later handles several family cases differently, with good results. When the supervisor asks about it, the supervisee says, 'I didn't think disagreeing with you was an option here.' What is the supervisor's **BEST** response?

A. Remind the supervisee to voice disagreement instead of bypassing plans
B. Offer to arrange supervision with a Black colleague instead
C. Name the power and racial difference and invite open disagreement

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Revisiting plans rather than bypassing them is the right norm. Said now, though, it answers the supervisee's disclosure with a rule and puts the whole burden on the supervisee. It fits after the supervisor has made disagreement clearly safe.

B **Incorrect** (trap: Hands off too soon). Moving the supervisee to a Black supervisor treats a dynamic the supervisor helps create as the supervisee's problem to escape. It also assumes the supervisee wants a race-matched supervisor, which the supervisee has not said.

C **Correct.** Supervisors hold evaluative power, and a racial difference can make that power harder for a supervisee to challenge. Naming both openly and inviting disagreement sets clear, culturally sensitive boundaries and makes room for honest clinical judgment (NASW Code 3.01(b); 1.05(c)). The comment shows the problem lies in the relationship, not in clinical skill, since the departures worked. It beats A because a reminder, though fair, leaves the supervisor's role in the silence unspoken and keeps the burden on the supervisee.

Takeaway: Supervisors address how their power and racial differences can silence supervisees by naming them and inviting disagreement, rather than enforcing compliance.

Sources: NASW Code of Ethics (2021), 3.01(b); NASW Code of Ethics (2021), 1.05(c)
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.5 Power differentials, racial privilege, internalized inferiority/superiority · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 1 of 3

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99. INT-1182 (rev 2) · Best answer: A

A social worker in a state prison meets with a client whose parent died last week. The facility denied the client's request to attend the funeral. In their first meeting since the death, the client sits quietly for a while, then says, 'It doesn't feel real. I never got to say goodbye.' What should the social worker do **FIRST**?

A. Invite the client to talk about the relationship and the loss
B. Request a psychiatric evaluation for depression after the loss
C. Help the client plan a private memorial ritual on site

Why each option is right or wrong

A **Correct.** Early grief work starts by making room for the loss: inviting the client to talk about the parent, their relationship and what being kept from the funeral means. Talking about the person who died helps a mourner begin to accept the reality of the death, Worden's first task of mourning (Worden, 2018), and shows the worker what the client needs before anything is proposed. It beats C because a substitute ritual can help the client say goodbye, but planning one in the first conversation, before the client has been heard, moves to a task before the grief has been received; it fits once the client's wishes are known.

B **Incorrect** (trap: Skips assessment). One week after a death, disbelief, anger and guilt are expected grief reactions, not signs of a disorder. A psychiatric evaluation would fit if the client showed pervasive hopelessness, suicidal thinking or an inability to function.

C **Incorrect** (trap: Right action, wrong time). Doka describes therapeutic rituals as a way to recognize grief that circumstances have blocked (Doka, 2002), and a memorial inside the facility may well help this client say goodbye. Proposed in the first meeting, though, before the client has talked about the parent or the loss, it skips hearing what the client needs and wants.

Takeaway: In a first contact after a death, invite the bereaved to talk about the person and the loss before proposing rituals or referrals.

Sources: Worden, J. W. (2018). Grief Counseling and Grief Therapy (5th ed.). Springer Publishing, Task I: accept the reality of the loss; funerals and rituals; Doka, K. J. (Ed.). (2002). Disenfranchised Grief: New Directions, Challenges, and Strategies for Practice. Research Press, Ch. 9: The role of ritual in the treatment of disenfranchised grief

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.10 Working with loss, separation, and grief · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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100. INT-0002 (rev 1) · Best answer: C

A client in psychodynamic treatment arrives late and apologizes repeatedly, saying the social worker must be angry. The social worker has shown no displeasure. The client adds that a parent used to explode over small mistakes. What is the **BEST** response within a psychodynamic approach?

A. Reassure the client that being late is fine
B. Assign a thought record to test the belief about anger
C. Explore how this fear may echo the client's past relationships

Why each option is right or wrong

- **A** **Incorrect** (trap: Good but not best). Reassurance is kind and may lower anxiety, but it bypasses the transference. In psychodynamic work, the client's expectation of anger is information to explore rather than something to smooth over.
- **B** **Incorrect** (trap: Wrong model or concept). A thought record is a cognitive behavioral technique for testing beliefs. It may help in CBT, but it does not fit the psychodynamic approach named in the stem.
- **C** **Correct.** The client expects the social worker to react the way an explosive parent did, though nothing in the session supports that. This is transference: feelings and expectations from early relationships placed onto the social worker. A psychodynamic approach uses it by gently exploring the link, which builds insight into a pattern that likely appears in other relationships (Greenson, 1967). It beats A because reassurance may calm the moment but closes off the material the approach is designed to work with.

Takeaway: When a client reacts to the social worker as to an early caregiver (transference), psychodynamic work explores the link to build insight.

Sources: Greenson, R. R. (1967). The Technique and Practice of Psychoanalysis. International Universities Press, transference

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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101. INT-1664 (rev 3) · Best answer: A

A social worker reviews the service plan of a 79-year-old client who lives alone and receives home-delivered meals to address weight loss. After 2 months, the client has lost another 8 pounds. The client says they eat every meal, and the home aide's log confirms the meals are finished. What should the social worker do **NEXT**?

- **A.** Ask the client's doctor to check promptly for a medical cause
- **B.** Ask a dietitian to review the calorie content of the meals
- **C.** Continue the current plan and reweigh next month

Why each option is right or wrong

- **A** **Correct.** Evaluating a service plan means asking whether the services worked, and why not when they did not (NASW, 2013, Standard 6). Unintentional weight loss in an older adult warrants medical evaluation, because causes range from cancer to medication effects (Gaddey & Holder, 2021). The meals are confirmed eaten, so the plan's assumption that intake was the problem no longer explains the loss. It beats B because reviewing the meals' calories still treats intake as the problem, which the data now make unlikely, and could delay finding a medical cause.
- **B** **Incorrect** (trap: Skips assessment). A dietitian's review is sensible when intake might be too low, and it may help later. The meals are confirmed eaten, though, so continued loss points beyond the menu to a possible medical cause, and reviewing calories first could delay that evaluation.

C **Incorrect** (trap: Ignores risk). Waiting another month can be reasonable when a plan is new and the trend unclear. Two months of continued loss despite confirmed eating is a clear signal, and delay risks missing a treatable illness.

Takeaway: When monitoring shows a fully delivered service plan is not working, reassess the cause, including medical evaluation, rather than adding more service.

Sources: Gaddey, H. L., & Holder, K. K. (2021). Unintentional weight loss in older adults. American Family Physician, 104(1), 34-40, whole article; NASW Standards for Social Work Case Management (2013), Standard 6: Service Planning, Implementation, and Monitoring

Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.1 Evaluating progress and effectiveness of intervention and service plans · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 2 of 3

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102. ETH-0118 (rev 1) · Best answer: C

At intake, a social worker asked a client with a Spanish surname whether they needed an interpreter. The client, who grew up in the United States and speaks only English, said no and was quiet for the rest of the session. In supervision afterward, the social worker identifies the question as an assumption based on the client's name. What is the social worker's **BEST** action?

A. Offer the client a transfer to a bilingual colleague at the agency

B. Wait to see if the client brings up the question

C. Acknowledge the question's impact with the client next session

Why each option is right or wrong

A **Incorrect** (trap: Hands off too soon). Offering a transfer assumes the client wants a worker who shares a presumed identity, repeating the original assumption. It also avoids the repair the relationship needs. A transfer fits if the client asks for one.

B **Incorrect** (trap: Good but not best). Waiting leaves the client to raise a painful moment with the person who caused it, which many clients will not do. The rupture may lead to dropout before the client ever brings it up.

C **Correct.** A microaggression can harm the alliance whether or not it was intended, and the social worker, not the client, is responsible for repair (Sue et al., 2007). The client's withdrawal after the question signals a rupture. Raising it directly, owning the assumption and inviting the client's reaction practices cultural humility and self-correction (NASW Code 1.05(c)). It beats B because waiting puts the burden on the client, who may simply not return; open repair of ruptures supports a stronger alliance (Safran & Muran, 2000).

Takeaway: After a microaggression, the social worker initiates repair: acknowledge the impact, take responsibility and invite the client's response.

Sources: Sue, D. W., Capodilupo, C. M., Torino, G. C., Bucceri, J. M., Holder, A. M. B., Nadal, K. L., & Esquilin, M. (2007). Racial microaggressions in everyday life: Implications for clinical practice. American Psychologist, 62(4), 271-286, whole article; NASW Code of Ethics (2021), 1.05(c); Safran, J. D., & Muran, J. C. (2000). Negotiating the Therapeutic Alliance: A Relational Treatment Guide. Guilford Press, rupture and repair

Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.6 Privilege, implicit bias, microaggressions and their client impact · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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103. ASM-1006 (rev 2) · Best answer: C

A patient is admitted to an inpatient psychiatric unit during a first manic episode. On the unit, the patient demands special privileges, insults staff and boasts of being destined for fame. A psychiatry resident suggests adding narcissistic personality disorder to the discharge diagnoses. What is the **BEST** response by the social worker?

A. Support adding it as a provisional diagnosis for follow-up
B. Administer a personality inventory before discharge to confirm it
C. Recommend deferring personality diagnosis until mood stabilizes

Why each option is right or wrong

A **Incorrect** (trap: Skips assessment). A provisional label sounds cautious, but it still records a personality disorder based only on behavior seen during a manic episode, when grandiosity and entitlement are expected symptoms. Even provisional diagnoses travel with the record and can bias later care. A personality diagnosis can be considered once the mood is stable.

B **Incorrect** (trap: Right action, wrong time). Standardized personality testing can help, but scores taken during acute mania reflect the episode rather than stable traits. Testing belongs after the mood has stabilized.

C **Correct.** A personality disorder is an enduring pattern that begins by early adulthood and is not better explained by another mental disorder (DSM-5-TR). Grandiosity, entitlement and irritability are core features of mania, so traits seen only during a first manic episode cannot show the person's long-term pattern. Personality is best assessed after the mood episode resolves, with history from the patient and others. It beats B because an inventory given during mania would measure the episode, not stable traits; it may be useful later.

Takeaway: Do not diagnose a personality disorder from behavior seen only during an acute mood or psychotic episode; reassess after it resolves.

Sources: DSM-5-TR (APA, 2022), Personality Disorders: General Personality Disorder
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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104. ASM-1160 (rev 1) · Best answer: C

PRETEST — unscored on this form; do not count it

At an outpatient intake, a client describes recurring urges to shout insults during religious services. The client calls them 'my own brain being cruel,' finds them upsetting, and tries to push them away with silent prayer. The client has never acted on them. Which mental status finding is this **MOST** likely to represent?

A. Delusion
B. Thought insertion
C. Obsession

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). A delusion is a fixed false belief held despite clear evidence against it. The client holds no false belief here; they see the urges as products of their own mind and find them unwelcome.

B **Incorrect** (trap: Wrong model or concept). Thought insertion is a delusion that someone or something outside has put thoughts into one's head. Calling the urges 'my own brain' shows the client knows they are self-generated.

C **Correct.** An obsession is a recurring thought, urge or image that feels unwanted and distressing, which the person tries to ignore or neutralize with another thought or act (DSM-5-TR). The client recognizes the urges as coming from their own mind and uses prayer to push them away. It beats B because in thought insertion the person believes an outside force has placed thoughts in their mind; this client calls the urges the work of their own brain.

Takeaway: Unwanted thoughts the client recognizes as their own and resists are obsessions; thoughts felt as placed by an outside force are thought insertion.

Sources: DSM-5-TR (APA, 2022), Obsessive-Compulsive and Related Disorders: Obsessive-Compulsive Disorder; DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Key Features That Define the Psychotic Disorders (Delusions)
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.10 Mental status exam: components and purpose · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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105. INT-1191 (rev 1) · Best answer: B

A client in outpatient treatment for alcohol use disorder wants peer support between sessions. After attending two 12-step meetings, the client says, 'I'm an atheist. All the talk about a higher power made me feel like I didn't belong.' What is the **MOST** appropriate response?

A. Encourage the client to keep attending and define a higher power freely
B. Share secular mutual-help options, such as SMART Recovery
C. Suggest relying on counseling sessions alone for now

Why each option is right or wrong

A **Incorrect** (trap: Overrides the client's choice). Many nonreligious members do find a personal meaning for a higher power, and the idea can be mentioned. Pushing continued attendance after the client reported feeling excluded overrides their stated values and risks losing peer support altogether.

B **Correct.** Recovery occurs through many pathways, and supports should fit the person's values and culture (SAMHSA, 2012). Secular mutual-help groups such as SMART Recovery offer peer support without spiritual language. The client wants support between sessions but felt excluded by the spiritual framing, so offering alternatives keeps that goal alive. It beats A because reframing a higher power works for some atheists, but pressing it after the client said it made them feel they did not belong risks dropout from peer support entirely.

C **Incorrect** (trap: Good but not best). Counseling alone is an acceptable choice if the client prefers it, but this client asked for support between sessions. Dropping that goal discards a resource the client wants when other options exist.

Takeaway: Match mutual-help referrals to the client's values; recovery has many pathways, including secular peer support groups.

Sources: SAMHSA (2012) Working Definition of Recovery, Guiding principles: recovery occurs via many pathways
Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.17 Intervening with addiction · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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106. ASM-1107 (rev 2) · Best answer: C

A 15-year-old in a group home returns from a weekend away with new clothes, a second phone and cash. The youth says a 24-year-old 'partner' pays for hotel rooms and sometimes asks the youth to spend time with the partner's friends, who then give the youth money. The youth insists no one forces anything. What should the social worker do **FIRST**?

A. Ask the youth whether threats or force were ever used
B. Make a safety plan with the youth for the next outing
C. Make a child abuse report for suspected sex trafficking

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Asking about force fits an adult case, because force, fraud or coercion defines adult sex trafficking. For a 15-year-old, sex exchanged for money is enough, so this question delays a required report.

B **Incorrect** (trap: Right action, wrong time). Safety planning with the youth is part of the response and should follow quickly. Planning for the next outing before reporting treats ongoing exploitation of a minor as something to manage rather than report.

C **Correct.** Under federal law, anyone under 18 induced into a commercial sex act is a sex trafficking victim; force, fraud or coercion need not be shown (22 U.S.C. 7102(11)(A)), and CAPTA treats an identified child trafficking victim as a victim of child abuse (42 U.S.C. 5106g(b)(1)). Money from a much older partner's friends gives reasonable suspicion, the threshold for a mandated report. It beats A because the youth's denial of force does not change the legal picture, and seeking proof delays protection.

Takeaway: A minor in any commercial sex exchange is a trafficking victim; report on reasonable suspicion without proof of force.

Sources: Trafficking Victims Protection Act, 22 U.S.C. 7102(3), (7), (11); Child Abuse Prevention and Treatment Act, 42 U.S.C. 5106g(b)(1)
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.15 Exploitation across the lifespan: financial, immigration-related, trafficking · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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107. ASM-1065 (rev 1) · Best answer: B

PRETEST — unscored on this form; do not count it

A hospital social worker completes a mental status exam with a patient admitted after a fall. The patient looks down at the floor throughout, answers fully and warmly, and describes a good mood. The patient explains that in their family, looking an elder or professional in the eye is disrespectful. What is the **MOST** appropriate way to record the patient's eye contact?

A. Note the patient's poor eye contact as a sign of low mood
B. Describe the downward gaze and the patient's stated reason
C. Omit eye contact from the record to avoid cultural bias

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Reduced eye contact can accompany depression or anxiety, but this patient reports good mood, engages warmly and gives a cultural reason. Treating the gaze as a symptom pathologizes a norm.

B **Correct.** MSE observations should be described accurately and interpreted in cultural context. Recording the downward gaze together with the patient's explanation gives an accurate observation without pathologizing a culturally rooted norm, and it fits the warm engagement and good mood the patient reports. It beats A because labeling the gaze a sign of low mood reads a cultural norm as a symptom, against the patient's own account (NASW Cultural Competence Standards, Standard 4; DSM-5-TR).

C **Incorrect** (trap: Misuses the technique). Leaving out an observed behavior makes the mental status exam incomplete. Accurate documentation with context, not omission, is how bias is kept out of the record.

Takeaway: Record MSE observations accurately along with the client's cultural context, rather than reading a cultural norm as a symptom.

Sources: NASW Standards and Indicators for Cultural Competence in Social Work Practice (2015), Standard 4: Cross-Cultural Skills; DSM-5-TR (APA, 2022), Section III: Culture and Psychiatric Diagnosis
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.10 Mental status exam: components and purpose · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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108. ASM-1558 (rev 1) · Best answer: A

A social worker on a mobile outreach team serves a client with schizophrenia who takes an antipsychotic and lives alone in a top-floor apartment without air conditioning. A week of extreme heat is forecast. The client declines the city cooling center, saying crowds make the voices worse, but agrees the apartment 'gets like an oven.' What is the **BEST** plan?

A. Arrange a quieter cool place and daily check-ins during the heat
B. Ask the prescriber to hold the antipsychotic during the heat wave
C. Respect the refusal and review heat safety tips with the client

Why each option is right or wrong

A **Correct.** Extreme heat is an environmental hazard that falls hardest on people with serious mental illness. Antipsychotics can impair sweating and temperature regulation, and CDC advises clinicians to help patients reach a cool environment and name someone to check on them (CDC, Heat and Medications). This client lives alone in a hot apartment and turned down one option for a clinical reason, not all help. A quieter cooled space the client accepts, with daily contact, respects that choice while reducing risk. It beats C because tips alone leave a high-risk client in dangerous heat.

B **Incorrect** (trap: Ignores risk). Stopping an antipsychotic abruptly risks relapse, and CDC guidance cautions against stopping medications without a plan. Asking the prescriber about heat precautions is reasonable, but holding the medication is not the answer.

C **Incorrect** (trap: Good but not best). Respecting the refusal of the cooling center is right, but stopping there accepts a serious risk. The client has not refused all help, and impaired heat regulation makes advice alone insufficient.

Takeaway: During extreme heat, clients taking antipsychotics need an active plan for cooling and check-ins, built around what the client will accept.

Sources: CDC (Centers for Disease Control and Prevention) (2025). Heat and Medications - Guidance for Clinicians, Antipsychotics: interference with central thermoregulation; patient planning
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.22 Urbanization, globalization, environmental hazards, climate change impacts · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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109. ASM-1060 (rev 1) · Best answer: D

A 7-year-old has lived with a relative caregiver since infancy. Child welfare records show the birth parent drank heavily throughout the pregnancy. The 7-year-old repeats the same mistakes despite consequences, forgets rules learned the day before and has a small head size noted by the pediatrician. The school asks the social worker to arrange an ADHD evaluation. What is the social worker's **MOST** appropriate recommendation?

A. Arrange the ADHD evaluation that the school requested
B. Start a home reward chart to teach rule following
C. Refer for trauma-focused therapy about early separation
D. Request a fetal alcohol spectrum disorder evaluation

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). ADHD is common in children with prenatal alcohol exposure, so attention should be assessed. An ADHD-only evaluation, though, can miss the wider memory and learning deficits that the exposure history points to.

B **Incorrect** (trap: Skips assessment). Reward charts rely on learning from consequences, the very skill this child struggles with. Home strategies should follow an evaluation that clarifies the cause of the difficulties.

C **Incorrect** (trap: Near-miss diagnosis). Early separation can matter, but the child has had one stable caregiver since infancy, and the memory and learning pattern with small head size points to a neurodevelopmental cause.

D **Correct.** Prenatal alcohol exposure can cause lifelong problems with memory, learning from consequences and attention, sometimes with physical signs such as small head

size. With documented heavy drinking in pregnancy plus these features, a specialized fetal alcohol spectrum disorder (FASD) evaluation is the most complete assessment, and it also examines attention. It beats A because an ADHD-only evaluation may label the attention problems while missing the broader cause that shapes school supports and caregiver strategies (Hoyme et al., 2016; DSM-5-TR).

Takeaway: Documented prenatal alcohol exposure with learning, memory and physical signs warrants a specialized FASD evaluation, not an ADHD-only workup.

Sources: Hoyme, H. E., et al. (2016). Updated clinical guidelines for diagnosing fetal alcohol spectrum disorders. Pediatrics, 138(2), e20154256, whole article; DSM-5-TR (APA, 2022), Conditions for Further Study: Neurobehavioral Disorder Associated With Prenatal Alcohol Exposure

Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.8 Addiction and substance use: signs and impact on systems · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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110. INT-1125 (rev 1) · Best answer: B

In the fourth month of psychodynamic therapy, a client makes a joke each time the conversation turns to a parent's recent cancer diagnosis, then changes the subject. The client has never remarked on this pattern. What should the social worker do **FIRST**?

A. Suggest that the joking guards against fear of losing the parent

B. Point out that the jokes appear whenever the illness comes up

C. Ask the client to stop the jokes and stay with the feeling

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). This interpretation may well be accurate, and it is where the work is heading. Offered now, before the client has noticed the joking at all, it is likely to feel like an accusation and be dismissed. Interpret once the client can see the pattern.

B **Correct.** In psychodynamic technique, a defense is first brought to the client's attention (confrontation and clarification) before its unconscious meaning is interpreted (Greenson, 1967). The client has never remarked on the joking, so the pattern is not yet something the client can observe. Pointing out when it happens invites the client to notice it and strengthens the observing part of the ego. It beats A because an interpretation offered before the client sees the defense tends to be rejected or intellectualized.

C **Incorrect** (trap: Too directive). Telling the client to drop the humor attacks the defense head-on instead of understanding it. Psychodynamic work treats a defense as meaningful material to explore, not a behavior to stop on command, and pressing can increase resistance.

Takeaway: Bring a defense to the client's attention before interpreting its unconscious meaning; interpretation works best once the client can observe the pattern.

Sources: Greenson, R. R. (1967). The Technique and Practice of Psychoanalysis. International Universities Press, analysis of resistance: confrontation, clarification, interpretation

Area III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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111. INT-1501 (rev 2) · Best answer: A

In weekly telehealth sessions, a client with panic attacks tells the social worker, 'You really get me.' Their plan centers on the client's wish to finish a grocery trip without fleeing the store in panic. Each week, though, the client skips the breathing practice, saying, 'I don't see how that connects to anything.' Which part of the working alliance is **MOST** likely weakest?

A. Agreement on tasks
B. Emotional bond
C. Agreement on goals

Why each option is right or wrong

A **Correct.** Bordin (1979) described the working alliance as three linked parts: agreement on goals, agreement on tasks, and the bond. Here the bond is strong ('You really get me') and the plan is built on the client's own wish, so the aim is shared. The break is the client's doubt that breathing practice leads anywhere, which is disagreement about the methods used to reach the goal. It beats C because the client accepts the goal itself and questions only the route. The repair is to discuss how each exercise links to that goal.

B **Incorrect** (trap: Wrong model or concept). The bond is the trust and attachment between client and worker. The client's warm comment shows a strong bond, so it is not the weak part here. Bond problems show up as mistrust, guardedness, or ruptures.

C **Incorrect** (trap: Wrong model or concept). Goal agreement concerns what the work is trying to achieve. The plan is built on the client's own wish to finish a grocery trip, so the aim is shared. Weak goal agreement would look like the client wanting a different outcome altogether.

Takeaway: Bordin's alliance has three parts: goals, tasks, and bond. A client who accepts the goal but doubts the methods signals weak task agreement.

Sources: Bordin, E. S. (1979). The generalizability of the psychoanalytic concept of the working alliance. Psychotherapy: Theory, Research & Practice, 16(3), 252-260, three components of the working alliance
Area III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.2 Building and sustaining the helping relationship (empathy, rapport, acceptance) · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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112. ETH-1166 (rev 1) · Best answer: C

Under the NASW Code of Ethics, what must social workers tell research participants about withdrawing from a study?

A. They may withdraw before data collection begins
B. They may withdraw if the review board approves
C. They may withdraw at any time without penalty

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). Withdrawal is not limited to the start of a study. Participants keep the right throughout, including after data collection begins, because consent continues for as long as they take part.

B **Incorrect** (trap: Misreads law or ethics). Review boards approve a study's design, not each participant's choice to leave. Requiring approval to withdraw would pressure people to stay, which conflicts with voluntary participation (5.02(e)).

C **Correct.** The Code directs those doing evaluation or research to inform participants of their right to withdraw at any time without penalty (NASW Code 5.02(i)). Consent is ongoing, not a one-time signature, so a participant can stop midway, after data collection has started, without losing services. It beats A because limiting withdrawal to the period before data collection would turn consent into a binding contract. Consent must also be free of coercion and of penalty for declining (5.02(e)).

Takeaway: Research consent is ongoing: participants may stop at any time, without penalty, and must be told so.

Sources: NASW Code of Ethics (2021), 5.02(i); NASW Code of Ethics (2021), 5.02(e)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.11 Research ethics: IRBs, human subjects protections, consent · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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113. ETH-1005 (rev 1) · Best answer: C

A client at an outpatient behavioral health clinic has signed the clinic's standard consent for sharing information with other treating providers. The client's new psychiatrist at another practice asks the social worker for the process notes that the social worker keeps separately from the clinic record. What should the social worker do?

A. Send the notes, since the request is for treatment
B. Give the psychiatrist a verbal summary of the notes
C. Get the client's specific written authorization for them

Why each option is right or wrong

A **Incorrect** (trap: Misreads law or ethics). HIPAA lets providers share most of the record for treatment without authorization, which makes this tempting. Psychotherapy notes are the exception: they need a specific authorization, even when another treating provider asks.

B **Incorrect** (trap: Misuses the technique). Summarizing the notes aloud still discloses their content. Changing the format from paper to speech does not remove the need for the client's specific authorization.

C **Correct.** Notes that analyze session conversation and are kept apart from the medical record are psychotherapy notes under HIPAA. Unlike the rest of the record, they need the client's specific authorization even for disclosure to another treating provider, and a general consent for sharing records does not cover them (45 CFR 164.508(a)(2)). The Code also requires valid consent for disclosure (NASW Code 1.07(b)). C beats A because the usual treatment exception applies to the clinic record, not to separately kept psychotherapy notes.

Takeaway: Separately kept psychotherapy notes need the client's specific authorization, even when another treating provider requests them.

Sources: HIPAA Privacy Rule, 45 CFR 164.508(a)(2); NASW Code of Ethics (2021), 1.07(b)
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** none · **Cognitive** application · **Difficulty** 2 of 3

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114. ETH-1695 (rev 1) · Best answer: B

Which term names the process in which oppressed people come to see how social and economic forces shape their lives, and then act together to change them?

A. Empowerment
B. Critical consciousness
C. Cultural humility

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Empowerment is the broader process of gaining personal, interpersonal and political power so people can improve their situations. Critical consciousness is one part of it: seeing how social structures shape personal problems.

B **Correct.** Critical consciousness, Freire's conscientizacao, is learning to perceive social, political and economic contradictions and to take action against oppressive conditions (Freire, 1970). It pairs awareness of structural forces with collective action, a foundation of anti-oppressive practice. It beats A because empowerment is the broader process of gaining personal, interpersonal and political power, and developing critical consciousness is one of its components (Gutierrez, 1990).

C **Incorrect** (trap: Wrong model or concept). Cultural humility describes a practitioner's stance of self-reflection and learning from clients. It concerns how the worker approaches difference, not how oppressed people come to analyze their own conditions.

Takeaway: Critical consciousness links personal problems to social structures and leads to collective action; it is a core process in empowerment and anti-oppressive practice.

Sources: Freire, P. (1970). Pedagogy of the Oppressed. Herder and Herder, conscientizacao (critical consciousness); Gutierrez, L. M. (1990). Working with women of color: An empowerment perspective. Social Work, 35(2), 149-153, empowerment process: critical consciousness
Area I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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115. ETH-1110 (rev 2) · Best answer: C

A child protective services social worker arrives with a court order and a police officer to remove a 6-year-old after substantiated severe neglect. The parent is crying but calm and cooperative and asks for a few minutes to say goodbye and to pack the child's stuffed animal and pajamas. What is the **BEST** response?

A. Complete the removal quickly to limit the child's distress
B. Let the officer decide whether a goodbye is safe
C. Permit a brief supervised farewell and the comfort items

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). A quick exit makes sense when a parent is escalating or the child faces danger in the moment. Here the parent is calm and cooperative, so rushing takes away comforts that ease the transition.

B **Incorrect** (trap: Good but not best). Officers keep the scene safe, and their view matters if the situation turns tense. Here the parent is calm and cooperative, and

shaping a removal that limits trauma is the social worker's responsibility (NCTSN, 2013); handing the decision to police gives that role away.

C **Correct.** The Dignity and Worth principle calls for treating parents and children with care and respect, even during protective action (NASW Code, Ethical Principles). With a court order in hand and a calm, cooperative parent, a brief supervised goodbye and familiar items cost little and help the child keep connection and comfort through the transition (NCTSN, 2013). C beats A because speed for its own sake does not reduce distress when no safety threat is present; an abrupt removal without a goodbye or comfort objects can add to the child's trauma.

Takeaway: Protective action does not suspend respect: when safe, allow goodbyes and comfort items, and give children honest explanations.

Sources: NASW Code of Ethics (2021), Ethical Principles: Dignity and Worth of the Person; National Child Traumatic Stress Network (2013). Child Welfare Trauma Training Toolkit (2nd ed.), Essential Element 1: Maximize physical and psychological safety

Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.5 Core values: competence, social justice, integrity, dignity and worth · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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116. ASM-1075 (rev 2) · Best answer: C

In an integrated primary care clinic, a client with depression and chronic back pain reported passive thoughts of death 3 weeks ago. At intake, the client agreed to the clinic's team-based care, in which providers share clinical information. The physician has just started amitriptyline and, to save on copays, prescribed a 90-day supply. The client asks the social worker not to 'make a big deal' of the earlier thoughts. What is the social worker's **BEST** response?

A. Coach the client to keep the pills locked away at home

B. Respect the request and track mood at follow-up

C. Alert the client's doctor to the overdose danger

Why each option is right or wrong

A **Incorrect** (trap: Good but not best). Locked storage is sound lethal-means advice and can be part of the plan. By itself, though, it leaves the client in control of 90 days of a drug that can be fatal in overdose. Only the prescriber can shorten the supply or choose another drug.

B **Incorrect** (trap: Ignores risk). The client's wish deserves a respectful hearing, and mood should be tracked. Waiting, however, leaves a large supply of a highly toxic medicine with someone who recently had thoughts of death, a risk the prescriber is not yet weighing.

C **Correct.** Tricyclic antidepressants such as amitriptyline are far more dangerous in overdose than newer antidepressants, mainly because of effects on heart rhythm (Kaplan & Sadock). For a client with recent thoughts of death, a 90-day supply matters. The prescriber can limit the quantity or change the drug; the social worker cannot. The client agreed to team-based care, and HIPAA permits sharing with a treating provider (45 CFR 164.506(c)); the worker still tells the client openly what will be shared and why. It beats A because locked storage still leaves a potentially fatal quantity in the client's hands.

Takeaway: When a client with recent suicidal thoughts receives a large supply of a drug that is lethal in overdose, alert the prescriber.

Sources: Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Tricyclic and tetracyclic antidepressants: overdose and cardiac toxicity; HIPAA Privacy Rule, 45 CFR 164.506(c)
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.5 Prescription, OTC, alternative medications: uses and side effects · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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117. INT-1666 (rev 1) · Best answer: B

An agency can offer a new caregiver support group to only 20 people at a time, and 60 eligible caregivers are waiting. Burden scores are in the moderate range for nearly all of them, and caregivers in crisis are routed to urgent services. The director wants to learn whether the group reduces burden and proposes starting with the 20 highest scorers. What should the social worker **BEST** recommend?

A. Start with the 20 highest scorers and compare their pre and post scores
B. Set start order by lottery and compare early and later groups
C. Start in list order and measure each group before and after

Why each option is right or wrong

A **Incorrect** (trap: Wrong model or concept). Starting with the highest scorers looks need-based, but extreme scorers tend to score closer to average when retested even without help, and they differ from later groups. Both make the group look more effective than it is. Ranking makes sense when needs differ sharply, which they do not here.

B **Correct.** When a program cannot serve everyone at once, setting start order by lottery is fair, because each person has an equal chance, and it creates a randomized comparison: caregivers still waiting serve as the control for those in the group (Shadish, Cook & Campbell, 2002). With burden similar across the list and crises handled elsewhere, little is lost by not ranking. It beats C because list order is also fair, but measuring participants only before and after cannot separate the group's effect from time, outside events, or other help caregivers receive.

C **Incorrect** (trap: Good but not best). Serving people in list order is fair, but a before-and-after measure of each group has no concurrent comparison. Any improvement could reflect time passing or outside events, so the agency would not know whether the group itself helped.

Takeaway: When demand exceeds capacity, a lottery for start order is fair and creates a randomized comparison group for evaluation.

Sources: Shadish, W. R., Cook, T. D., & Campbell, D. T. (2002). Experimental and Quasi-Experimental Designs for Generalized Causal Inference. Houghton Mifflin, randomized experiments: random assignment when demand exceeds supply
Area III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.5 Research design, data collection/analysis, reliability, validity · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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118. ASM-1026 (rev 1) · Best answer: D

At an overnight shelter, staff ask the social worker to see a resident with years of heavy drinking who has not had alcohol for 2 days. Since this morning, the resident has been confused about where they are, walks with a wide, staggering gait and has jerky eye movements. Staff think the resident just needs to sleep it off. What should the social worker do **FIRST**?

A. Complete a brief cognitive screen such as the MoCA
B. Arrange admission to a withdrawal management program
C. Let the resident rest and check on them hourly
D. Send the resident to the hospital right away

Why each option is right or wrong

A **Incorrect** (trap: Right action, wrong time). A cognitive screen adds little when sudden confusion and new neurological signs already call for medical care. Screening can follow once the resident is medically stable.

B **Incorrect** (trap: Good but not best). Withdrawal management is a sound plan for heavy drinking, but these new neurological signs need hospital evaluation and treatment first. Many programs cannot treat a possible thiamine-deficiency emergency.

C **Incorrect** (trap: Ignores risk). Assuming intoxication delays care for a possible brain emergency. The resident has not been drinking for 2 days, so sleeping it off does not explain new confusion, gait problems and eye movements.

D **Correct.** Confusion, an unsteady wide-based gait and abnormal eye movements in a person with heavy alcohol use are the classic signs of Wernicke encephalopathy, a thiamine-deficiency brain emergency that can leave lasting memory damage (Korsakoff syndrome) without prompt treatment (Kaplan & Sadock). Alcohol withdrawal around 48 hours can also cause dangerous confusion (DSM-5-TR). Either way, safety comes first and the resident needs medical care now. It beats B because a withdrawal program may not give the immediate hospital treatment these signs require.

Takeaway: Sudden confusion, unsteady gait or abnormal eye movements in heavy alcohol use signal a medical emergency; get medical care before screening or referral.

Sources: Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Alcohol-related disorders: Wernicke encephalopathy and Korsakoff syndrome; DSM-5-TR (APA, 2022), Substance-Related and Addictive Disorders: Alcohol Withdrawal
Area II Assessment and Planning · **Sub-area** II.B Assessment Methods and Techniques · **Topic** II.B.9 DSM use in assessment; common mental and brain-related condition indicators · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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119. ASM-1056 (rev 2) · Best answer: A

Staff at a youth drop-in center ask a social worker to see a 13-year-old who arrived dizzy and unsteady. The youth's speech is slurred, the eyes make jerky side-to-side movements, and there is silver paint around the nose and mouth. The youth's clothes smell strongly of chemicals. Which condition is **MOST** likely?

A. Inhalant intoxication
B. Alcohol intoxication
C. Sedative-hypnotic intoxication

Why each option is right or wrong

A **Correct.** Inhalant intoxication brings dizziness, slurred speech, unsteady gait and nystagmus (jerky eye movements), and it is most common in young adolescents because household products are easy to get. Paint residue around the nose and mouth and a chemical odor on clothing are telltale signs of breathing in vapors from paints or solvents. It beats B, alcohol intoxication, which shares the slurring, unsteadiness and nystagmus but would not explain paint on the face or a solvent smell (DSM-5-TR). Inhalants can cause sudden death, so medical assessment is warranted.

B **Incorrect** (trap: Near-miss diagnosis). Alcohol intoxication produces slurred speech, poor coordination and nystagmus, so it fits much of the picture. It does not account for the paint residue around the nose and mouth or the chemical odor.

C **Incorrect** (trap: Near-miss diagnosis). Sedative-hypnotic intoxication, as from benzodiazepines, also brings slurred speech, unsteadiness and nystagmus, so it fits the physical signs. It does not explain the paint residue around the nose and mouth or the chemical odor on the clothing.

Takeaway: Dizziness, slurring and nystagmus with paint or solvent residue and chemical odor indicate inhalant intoxication, most common in young adolescents.

Sources: DSM-5-TR (APA, 2022), Substance-Related and Addictive Disorders: Inhalant Intoxication
Area II Assessment and Planning · **Sub-area** II.A Assessment Concepts · **Topic** II.A.8 Addiction and substance use: signs and impact on systems · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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120. ETH-0002 (rev 1) · Best answer: D

A hospital social worker is asked to help with an older adult patient who was just diagnosed with advanced cancer. Before the physician meets with the patient, the patient's adult children ask the team not to share the diagnosis. They explain that in their family's culture, elders are protected from bad news. What should the social worker do **FIRST**?

A. Tell the adult children that the patient has a legal right to know
B. Agree to the family's request out of respect for their cultural values
C. Refer the disagreement to the hospital ethics committee for a decision
D. Ask the patient how much they want to know and who should be told

Why each option is right or wrong

A **Incorrect** (trap: Too directive). The patient does hold a right to the information, but announcing it to the family first is confrontational. It also skips the deciding question, because the patient may choose to have family receive the news. Rights are clarified once the patient's preference is known.

B **Incorrect** (trap: Overrides the client's choice). Agreeing lets the family decide for a competent adult. Respecting culture means asking the patient how they want their culture honored, rather than assuming the family speaks for them (NASW Code 1.05(c)).

C **Incorrect** (trap: Hands off too soon). An ethics consultation can help if the conflict continues after the patient's wishes are known. Referring now hands off a question that a direct conversation with the patient may settle.

D **Correct.** When values conflict and no one is in immediate danger, the first step is to clarify the facts. The key fact here is the patient's own wishes. Asking how much the patient wants to know, and who should be told, respects self-determination (NASW Code 1.02) and leaves room for a culturally grounded choice to defer to family (1.05). It beats A because asserting a right to know before learning the patient's wishes could force unwanted information on someone who would choose family-centered disclosure. Reamer's decision-making framework likewise starts by identifying the issues and the people affected.

Takeaway: In a values conflict without imminent danger, first clarify the facts, starting with the competent client's own wishes.

Sources: NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.05(c); Reamer, F. G. (2018). Social Work Values and Ethics (5th ed.). Columbia University Press, ethical decision-making framework
Area I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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121. ETH-1668 (rev 1) · Best answer: C

PRETEST — unscored on this form; do not count it

A nursing home resident with no living family has worked with the facility social worker for 3 years. The resident, who has full decision-making capacity, asks the social worker to serve as health care agent, saying, 'You know what I want better than anyone.' What is the **BEST** response?

A. Agree, since no family is available to serve
B. Suggest that a court appoint a guardian for the resident
C. Decline, and help the resident consider other possible agents
D. Ask the administrator whether staff may serve as agents

Why each option is right or wrong

A **Incorrect** (trap: Boundary slip or collusion). The request is sincere and the resident trusts the worker, which makes agreeing tempting. As agent, though, the worker could face decisions that conflict with the facility's interests or their own job, so the dual role puts the resident at risk.

B **Incorrect** (trap: Misreads law or ethics). Guardianship is for people who cannot make their own decisions. This resident has full capacity and can name an agent, so seeking a guardian would remove rights the resident is able to exercise.

C **Correct.** Serving as agent would add a second, powerful role with a current client, the kind of dual relationship the Code says to avoid when it risks exploitation or harm (NASW Code 1.06(c)). An agent may face decisions where the worker's job, the facility's interests and the resident's wishes collide (1.06(a)). Declining while helping the resident find another agent, such as a trusted friend, faith community member or professional fiduciary, respects the purpose of the request. It beats A because a lack of family is a reason to search widely, not to merge the roles.

D **Incorrect** (trap: Good but not best). Facility rules matter, but checking them first frames the issue as one of permission. Even if staff may serve, the conflict between the agent role and the worker's job remains, so the worker should decline either way.

Takeaway: Decline to take on a client's personal legal roles, such as health care agent, and help find someone outside the professional relationship.

Sources: NASW Code of Ethics (2021), 1.06(c); NASW Code of Ethics (2021), 1.06(a)
Area I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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122. ASM-1078 (rev 1) · Best answer: D

A psychiatric inpatient with schizophrenia had a haloperidol dose increase 5 days ago. Today the patient is confused and sweating heavily, with a temperature of 103°F. Nursing staff report that the patient's arms are stiff and hard to bend and that the blood pressure keeps swinging up and down. What is the **MOST** likely explanation?

A. Serotonin syndrome
B. Acute dystonic reaction
C. Worsening psychotic symptoms
D. Neuroleptic malignant syndrome

Why each option is right or wrong

A **Incorrect** (trap: Near-miss diagnosis). Serotonin syndrome also brings fever, sweating and confusion. It follows serotonergic drugs, usually starts within hours, and features jerking muscles and overactive reflexes rather than stiff, hard-to-bend limbs after an antipsychotic increase.

B **Incorrect** (trap: Near-miss diagnosis). Acute dystonia is a sudden muscle spasm, such as a twisted neck or upward-rolling eyes, soon after starting an antipsychotic. It does not cause high fever, confusion or unstable blood pressure.

C **Incorrect** (trap: Near-miss diagnosis). Psychosis can worsen, but it does not explain fever, heavy sweating, rigid limbs and swinging blood pressure. Those physical signs point to a medication reaction that is a medical emergency.

D **Correct.** Neuroleptic malignant syndrome is a rare, potentially fatal reaction to dopamine-blocking drugs, often after a new medicine or a dose increase. Its core features are fever, severe muscle rigidity, changed mental status and unstable blood pressure or pulse, developing over days (DSM-5-TR). This patient shows all four after a haloperidol increase. It beats A because serotonin syndrome follows serotonergic drugs, usually begins within hours and features muscle jerking and brisk reflexes rather than rigidity. Either condition needs emergency medical care (Kaplan & Sadock).

Takeaway: Fever, rigidity, confusion and unstable vital signs days after starting or raising an antipsychotic suggest neuroleptic malignant syndrome, a medical emergency.

Sources: DSM-5-TR (APA, 2022), Medication-Induced Movement Disorders and Other Adverse Effects of Medication: Neuroleptic Malignant Syndrome; Boland, Verduin & Ruiz (Eds.). Kaplan & Sadock's Synopsis of Psychiatry (12th ed.), Medication-induced movement disorders: neuroleptic malignant syndrome
Area II Assessment and Planning · **Sub-area** II.C Assessment Practices · **Topic** II.C.5 Prescription, OTC, alternative medications: uses and side effects · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3
